Occupational English Test 2.0

Reading Subtest

(All authentic materials)

Table of Contents

1. Test 1 (Practice book 1): Page 3
2. Test 2 (Practice book 2): Page 28
3. Test 3 (Practice book 3): Page 54
4. Test 4 (OET sample 1): Page 79
5. Test 5 (OET sample 2): Page 105
6. Test 6 (E2language 1): Page 125
7. Test 7 (E2language 2): Page 150
8. Test 8 (E2language 3): Page 168
9. Test 9 (Premium OETonline): Page 186
10. Test 10 (Kaplan 1): Page 204
11. Test 11 (Kaplan 2): Page 227
12. Test 12: Page 253

OCCUPATIONAL ENGLISH TEST

READING SUB-TEST – TEXT BOOKLET: PART A

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

MIDDLE NAMES:

PROFESSION:

VENUE:

TEST DATE:

Passport Photo

CANDIDATE DECLARATION

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INSTRUCTIONS TO CANDIDATES

You must **NOT** remove OET material from the test room.

SAMPLE

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Text A

Paediatric nasogastric tube use

Nasogastric is the most common route for enteral feeding. It is particularly useful in the short term, and when it is necessary to avoid a surgical procedure to insert a gastrostomy device. However, in the long term, gastrostomy feeding may be more suitable.

Issues associated with paediatric nasogastric tube feeding include:

- The procedure for inserting the tube is traumatic for the majority of children.
- The tube is very noticeable.
- Patients are likely to pull out the tube making regular re-insertion necessary.
- Aspiration, if the tube is incorrectly placed.
- Increased risk of gastro-esophageal reflux with prolonged use.
- Damage to the skin on the face.

Text B

Inserting the nasogastric tube

All tubes must be radio opaque throughout their length and have externally visible markings.

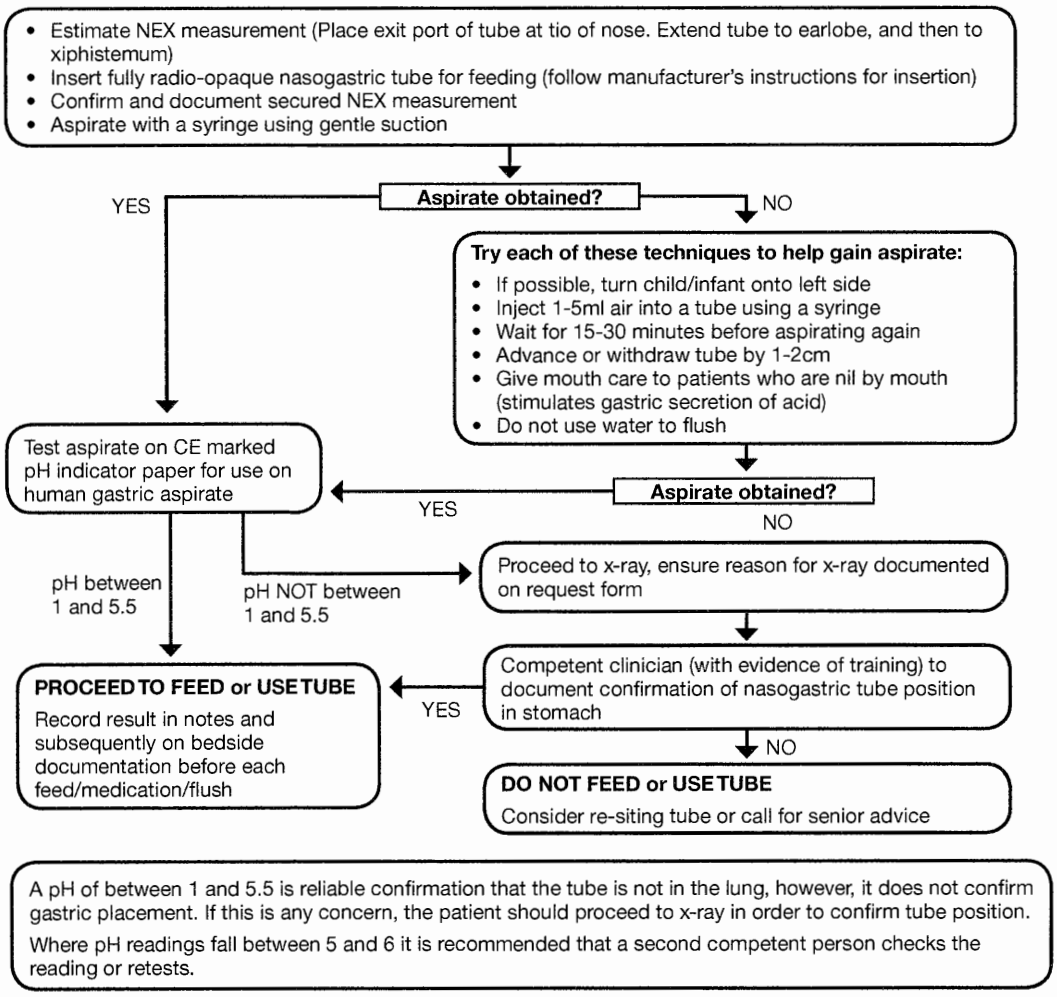
1. Wide bore:
   - for short-term use only.
   - should be changed every seven days.
   - range of sizes for paediatric use is 6 Fr to 10 Fr.
2. Fine bore:
   - for long-term use.
   - should be changed every 30 days.

In general, tube sizes of 6 Fr are used for standard feeds, and 7-10 Fr for higher density and fibre feeds. Tubes come in a range of lengths, usually 55cm, 75cm or 85cm.

Wash and dry hands thoroughly. Place all the equipment needed on a clean tray.

- Find the most appropriate position for the child, depending on age and/or ability to co-operate. Older children may be able to sit upright with head support. Younger children may sit on a parent's lap. Infants may be wrapped in a sheet or blanket.
- Check the tube is intact then stretch it to remove any shape retained from being packaged.
- Measure from the tip of the nose to the bottom of the ear lobe, then from the ear lobe to xiphisternum. The length of tube can be marked with indelible pen or a note taken of the measurement marks on the tube (for neonates: measure from the nose to ear and then to the halfway point between xiphisternum and umbilicus).
- Lubricate the end of the tube using a water-based lubricant.
- Gently pass the tube into the child's nostril, advancing it along the floor of the nasopharynx to the oropharynx. Ask the child to swallow a little water, or offer a younger child their soother, to assist passage of the tube down the oesophagus. Never advance the tube against resistance.
- If the child shows signs of breathlessness or severe coughing, remove the tube immediately.
- Lightly secure the tube with tape until the position has been checked.

Text C



Text D

Administering feeds/fluid via a feeding tube

Feeds are ordered through a referral to the dietitian.

When feeding directly into the small bowel, feeds must be delivered continuously via a feeding pump. The small bowel cannot hold large volumes of feed.

Feed bottles must be changed every six hours, or every four hours for expressed breast milk.

Under no circumstances should the feed be decanted from the container in which it is sent up from the special feeds unit.

All feeds should be monitored and recorded hourly using a fluid balance chart.

If oral feeding is appropriate, this must also be recorded.

The child should be measured and weighed before feeding commences and then twice weekly.

The use of this feeding method should be re-assessed, evaluated and recorded daily.

END OF PART A
THIS TEXT BOOKLET WILL BE COLLECTED

OCCUPATIONAL ENGLISH TEST

READING SUB-TEST – QUESTION PAPER: PART A

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

MIDDLE NAMES:

PROFESSION:

VENUE:

TEST DATE:

Passport Photo

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CANDIDATE SIGNATURE: _____

TIME: 15 MINUTES

INSTRUCTIONS TO CANDIDATES

DO NOT open this **Question Paper** or the **Text Booklet** until you are told to do so.

Write your answers on the spaces provided on this **Question Paper.**

You must answer the questions within the 15-minute time limit.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the 15 minutes, hand in this **Question Paper** and the **Text Booklet.**

DO NOT remove OET material from the test room.

SAMPLE

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Part A

TIME: 15 minutes

- Look at the four texts, **A-D**, in the separate **Text Booklet**.
- For each question, **1-20**, look through the texts, **A-D**, to find the relevant information.
- Write your answers on the spaces provided in this **Question Paper**.
- Answer all the questions within the 15-minute time limit.
- Your answers should be correctly spelt.

The use of feeding tubes in paediatrics: Questions

Questions 1-7

For each question, **1-7**, decide which text (**A**, **B**, **C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about

1 the risks of feeding a child via a nasogastric tube? _____

2 calculating the length of tube that will be required for a patient? _____

3 when alternative forms of feeding may be more appropriate than nasogastric? _____

4 who to consult over a patient's liquid food requirements? _____

5 the outward appearance of the tubes? _____

6 knowing when it is safe to go ahead with the use of a tube for feeding? _____

7 how regularly different kinds of tubes need replacing? _____

Questions 8-15

Answer each of the questions, **8-15**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

8 What type of tube should you use for patients who need nasogastric feeding for an extended period?

_____

9 What should you apply to a feeding tube to make it easier to insert?

_____

10 What should you use to keep the tube in place temporarily?

_____

11 What equipment should you use initially to aspirate a feeding tube?

_____

12 If initial aspiration of the feeding tube is unsuccessful, how long should you wait before trying again?

_____

13 How should you position a patient during a second attempt to obtain aspirate?

_____

14 If aspirate exceeds pH 5.5, where should you take the patient to confirm the position of the tube?

_____

15 What device allows for the delivery of feeds via the small bowel?

_____

Questions 16-20

Complete each of the sentences, **16-20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

16 If a feeding tube isn't straight when you unwrap it, you should

_____ it.

17 Patients are more likely to experience _____ if they need long-term feeding via a tube.

18 If you need to give the patient a standard liquid feed, the tube to use is

_____ in size.

19 You must take out the feeding tube at once if the patient is coughing badly or is

experiencing _____ .

20 If a child is receiving _____ via a feeding tube, you should replace the feed bottle after four hours.

END OF PART A

THIS QUESTION PAPER WILL BE COLLECTED

OCCUPATIONAL ENGLISH TEST

READING SUB-TEST – QUESTION PAPER: PARTS B & C

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

MIDDLE NAMES:

PROFESSION:

VENUE:

TEST DATE:

Passport Photo

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By signing this, you agree not to disclose or use in any way (other than to take the test) or assist any other person to disclose or use any OET test or sub-test content. If you cheat or assist in any cheating, use any unfair practice, break any of the rules or regulations, or ignore any advice or information, you may be disqualified and your results may not be issued at the sole discretion of CBLA. CBLA also reserves its right to take further disciplinary action against you and to pursue any other remedies permitted by law. If a candidate is suspected of and investigated for malpractice, their personal details and details of the investigation may be passed to a third party where required.

CANDIDATE SIGNATURE: _____

TIME: 45 MINUTES

INSTRUCTIONS TO CANDIDATES

DO NOT open this **Question Paper** until you are told to do so.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the test, hand in this **Question Paper**.

HOW TO ANSWER THE QUESTIONS:

Mark your answers on this **Question Paper** by filling in the circle using a 2B pencil. **Example:** Ⓐ Ⓑ Ⓒ

SAMPLE

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Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For **questions 1-6**, choose answer (**A**, **B** or **C**) which you think fits best according to the text.

1. If vaccines have been stored incorrectly,

(A) this should be reported.

(B) staff should dispose of them securely.

(C) they should be sent back to the supplier.

Manual extract: effective cold chain
The cold chain is the system of transporting and storing vaccines within the temperature range of +2°C to +8°C from the place of manufacture to the point of administration. Maintenance of the cold chain is essential for maintaining vaccine potency and, in turn, vaccine effectiveness. Purpose-built vaccine refrigerators (PBVR) are the preferred means of storage for vaccines. Domestic refrigerators are not designed for the special temperature needs of vaccine storage. Despite best practices, cold chain breaches sometimes occur. Do not discard or use any vaccines exposed to temperatures below +2°C or above +8°C without obtaining further advice. Isolate vaccines and contact the state or territory public health bodies for advice on the National Immunisation Program vaccines and the manufacturer for privately purchased vaccines.

2. According to the extract, prior to making a home visit, nurses must

(A) record the time they leave the practice.

(B) refill their bag with necessary items.

(C) communicate their intentions to others.

Nurse home visit guidelines

When the nurse is ready to depart, he/she must advise a minimum of two staff members that he/she is commencing home visits, with one staff member responsible for logging the nurse's movements. More than one person must be made aware of the nurse's movements; failure to do so could result in the breakdown of communication and increased risk to the nurse and/or practice.

On return to the practice, the nurse will immediately advise staff members of his/her return. This time will be documented on the patient visit list, and then scanned and filed by administration staff. The nurse will then attend to any specimens, cold chain requirements, restocking of the nurse kit and biohazardous waste.

3. What is being described in this section of the guidelines?

(A) changes in procedures

(B) best practice procedures

(C) exceptions to the procedures

Guidelines for dealing with hospital waste

All biological waste must be carefully stored and disposed of safely. Contaminated materials such as blood bags, dirty dressings and disposable needles are also potentially hazardous and must be treated accordingly. If biological waste and contaminated materials are not disposed of properly, staff and members of the community could be exposed to infectious material and become infected. It is essential for the hospital to have protocols for dealing with biological waste and contaminated materials. All staff must be familiar with them and follow them.

The disposal of biohazardous materials is time-consuming and expensive, so it is important to separate out non-contaminated waste such as paper, packaging and non-sterile materials. Make separate disposal containers available where waste is created so that staff can sort the waste as it is being discarded.

4. When is it acceptable for a health professional to pass on confidential information given by a patient?

(A) if non-disclosure could adversely affect those involved

(B) if the patient's treatment might otherwise be compromised

(C) if the health professional would otherwise be breaking the law

Extract from guidelines: Patient Confidentiality

Where a patient objects to information being shared with other health professionals involved in their care, you should explain how disclosure would benefit the continuity and quality of care. If their decision has implications for the proposed treatment, it will be necessary to inform the patient of this. Ultimately if they refuse, you must respect their decision, even if it means that for reasons of safety you must limit your treatment options. You should record their decision within their clinical notes.

It may be in the public interest to disclose information received in confidence without consent, for example, information about a serious crime. It is important that confidentiality may only be broken in this way in exceptional circumstances and then only after careful consideration. This means you can justify your actions and point out the possible harm to the patient or other interested parties if you hadn't disclosed the information. Theft, fraud or damage to property would generally not warrant a breach of confidence.

5. The purpose of the email to practitioners about infection control obligations is to

(A) act as a reminder of their obligations.

(B) respond to a specific query they have raised.

(C) announce a change in regulations affecting them.

Email from Dental Board of Australia

Dear Practitioner,

You may be aware of the recent media and public interest in standards of infection control in dental practice. As regulators of the profession, we are concerned that there has been doubt among registered dental practitioners about these essential standards.

Registered dental practitioners must comply with the National Board's Guidelines on infection control. The guidelines list the reference material that you must have access to and comply with, including the National Health and Medical Research Council's (NHMRC) Guidelines for the prevention and control of infection in healthcare.

We believe that most dental practitioners consistently comply with these guidelines and implement appropriate infection control protocols. However, the consequences for non-compliance with appropriate infection control measures will be significant for you and also for your patients and the community.

6. The results of the study described in the memo may explain why

(A) superior communication skills may protect women from dementia.

(B) female dementia sufferers have better verbal skills.

(C) mild dementia in women can remain undiagnosed.

Memo to staff: Women and Dementia

Please read this extract from a recent research paper

Women's superior verbal skills could work against them when it comes to recognizing Alzheimer's disease. A new study looked at more than 1300 men and women divided into three groups: one group comprised patients with amnestic mild cognitive impairment; the second group included patients with Alzheimer's dementia; and the final group included healthy controls. The researchers measured glucose metabolic rates with PET scans. Participants were then given immediate and delayed verbal recall tests.

Women with either no, mild or moderate problems performed better than men on the verbal memory tests. There was no difference in those with advanced Alzheimer's.

Because verbal memory scores are used for diagnosing Alzheimer's, some women may be further along in their disease before they are diagnosed. This suggests the need to have an increased index of suspicion when evaluating women with memory problems.

Part C

In this part of the test, there are two texts about different aspects of healthcare. For **questions 7-22**, choose the answer (**A**, **B**, **C** or **D**) which you think fits best according to the text.

Text 1: Asbestosis

Asbestos is a naturally occurring mineral that has been linked to human lung disease. It has been used in a huge number of products due to its high tensile strength, relative resistance to acid and temperature, and its varying textures and degrees of flexibility. It does not evaporate, dissolve, burn or undergo significant reactions with other chemicals. Because of the widespread use of asbestos, its fibres are **ubiquitous** in the environment. Building insulation materials manufactured since 1975 should no longer contain asbestos; however, products made or stockpiled before this time remain in many homes. Indoor air may become contaminated with fibres released from building materials, especially if they are damaged or crumbling.

One of the three types of asbestos-related diseases is asbestosis, a process of lung tissue scarring caused by asbestos fibres. The symptoms of asbestosis usually include slowly progressing shortness of breath and cough, often 20 to 40 years after exposure. Breathlessness advances throughout the disease, even without further asbestos inhalation. This fact is highlighted in the case of a 67-year-old retired plumber. He was on ramipril to treat his hypertension and developed a persistent dry cough, which his doctor presumed to be an ACE inhibitor induced cough. The ramipril was changed to losartan. The patient had never smoked and did not have a history of asthma or COPD. His cough worsened and he complained of breathlessness on exertion. In view of this history and the fact that he was a non-smoker, he was referred for a chest X-ray and to the local respiratory physician. His doctor was surprised to learn that the patient had asbestosis, diagnosed by a high-resolution CT scan. The patient then began legal proceedings to claim compensation as he had worked in a dockyard 25 years previously, during which time he was exposed to asbestos.

There are two major groups of asbestos fibres, the amphibole and chrysotile fibres. The amphiboles are much more likely to cause cancer of the lining of the lung (mesothelioma) and scarring of the lining of the lung (pleural fibrosis). Either group of fibres can cause disease of the lung, such as asbestosis. The risk of developing asbestos-related lung cancer varies between fibre types. Studies of groups of patients exposed to chrysotile fibres show only a moderate increase in risk. On the other hand, exposure to amphibole fibres or to both types of fibres increases the risk of lung cancer two-fold. Although the Occupational Safety and Health Administration (OSHA) has a standard for workplace exposure to asbestos (0.2 fibres/millilitre of air), there is debate over what constitutes a safe level of exposure. While some believe asbestos-related disease is a 'threshold phenomenon', which requires a certain level of exposure for disease to occur, others believe there is no safe level of asbestos.

Depending on their shape and size, asbestos fibres deposit in different areas of the lung. Fibres less than 3mm easily move into the lung tissue and the lining surrounding the lung. Long fibres, greater than 5mm cannot be completely broken down by scavenger cells (macrophages) and become lodged in the lung tissue, causing inflammation. Substances damaging to the lungs are then released by cells that are responding to the foreign asbestos material. The persistence of these long fibres in the lung tissue and the resulting inflammation seem to initiate the process of cancer formation. As inflammation and damage to tissue around the asbestos fibres continues, the resulting scarring can extend from the small airways to the larger airways and the tiny air sacs (alveoli) at the end of the airways.

There is no cure for asbestosis. Treatments focus on a patient's ability to breathe. Medications like bronchodilators, aspirin and antibiotics are often prescribed and such treatments as oxygen therapy and postural drainage may be recommended. If symptoms are so severe that medications don't work, surgery may be recommended to remove scar tissue. Patients with asbestosis, like others with chronic lung disease, are at a higher risk of serious infections that take advantage of diseased or scarred lung tissue, so prevention and rapid treatment is vital. Flu and pneumococcal vaccinations are a part of routine care for these patients. Patients with progressive disease may be given corticosteroids and cyclophosphamide with limited improvement.

Chrysotile is the only form of asbestos that is currently in production today. Despite their association with lung cancer, chrysotile products are still used in 60 countries, according to the industry-sponsored Asbestos Institute. Although the asbestos industry proclaims the 'safety' of chrysotile fibres, which are now imbedded in less friable and 'dusty' products, little is known about the long term effects of these products because of the long delay in the development of disease. In spite of their potential health risks, the durability and cheapness of these products continue to attract commercial applications. Asbestosis remains a significant clinical problem even after marked reductions in on-the-job exposure to asbestos. Again, **this** is due to the long period of time between exposure and the onset of disease.

Text 1: Questions 7-14

7. The writer suggests that the potential for harm from asbestos is increased by

(A) a change in the method of manufacture.

(B) the way it reacts with other substances.

(C) the fact that it is used so extensively.

(D) its presence in recently constructed buildings.

8. The word '**ubiquitous**' in paragraph one suggests that asbestos fibres

(A) can be found everywhere.

(B) may last for a long time.

(C) have an unchanging nature.

(D) are a natural substance.

9. The case study of the 67-year-old man is given to show that

(A) smoking is unrelated to a diagnosis of asbestosis.

(B) doctors should be able to diagnose asbestosis earlier.

(C) the time from exposure to disease may cause delayed diagnosis.

(D) patients must provide full employment history details to their doctors.

10. In the third paragraph, the writer highlights the disagreement about

(A) the relative safety of the two types of asbestos fibres.

(B) the impact of types of fibres on disease development.

(C) the results of studies into the levels of risk of fibre types.

(D) the degree of contact with asbestos fibres considered harmful.

11. In the fourth paragraph, the writer points out that longer asbestos fibres

(A) can travel as far as the alveoli.

(B) tend to remain in the pulmonary tissue.

(C) release substances causing inflammation.

(D) mount a defence against the body's macrophages.

12. What is highlighted as an important component of patient management?

(A) the use of corticosteroids

(B) infection control

(C) early intervention

(D) excision of scarred tissue

13. The writer states that products made from chrysotile

(A) have restricted application.

(B) may pose a future health threat.

(C) enjoy approval by the regulatory bodies.

(D) are safer than earlier asbestos-containing products.

14. In the final paragraph, the word 'this' refers to

(A) the interval from asbestos exposure to disease.

(B) the decreased use of asbestos in workplaces.

(C) asbestosis as an ongoing medical issue.

(D) occupational exposure to asbestos.

Text 2: Medication non-compliance

A US doctor gives his views on a new program

An important component of a patient's history and physical examination is the question of 'medication compliance,' the term used by physicians to designate whether, or not, a patient is taking his or her medications. Many a hospital chart bears the notorious comment 'Patient has a history of non-compliance.' Now, under a new experimental program in Philadelphia, USA, patients are being paid to take their medications. The concept makes sense in theory - failure to comply is one of the most common reasons that patients are readmitted to hospital shortly after being discharged.

Compliant patients take their medications because they want to live as long as possible; some simply do so because they're responsible, conscientious individuals by nature. But the hustle and bustle of daily life and employment often get in the way of taking medications, especially those that are timed inconveniently or in frequent doses, even for such well-intentioned patients. For the elderly and the mentally or physically impaired, US insurance companies will often pay for a daily visit by a nurse, to ensure a patient gets at least one set of the most vital pills. But other patients are left to fend for themselves, and it is not uncommon these days for patients to be taking a considerable number of vital pills daily.

Some patients have not been properly educated about the importance of their medications in layman's terms. They have told me, for instance, that they don't have high blood pressure because they were once prescribed a high blood pressure pill – in essence, they view an antihypertensive as an antibiotic that can be used as short-term treatment for a short-term problem. Others have told me that they never had a heart attack because they were taken to the cardiac catheterization lab and 'fixed.' As physicians we are responsible for making sure patients understand their own medical history and their own medications.

Not uncommonly patients will say, 'I googled it the other day, and there was a long list of side effects.' But a simple conversation with the patient at this juncture can easily change their perspective. As with many things in medicine, it's all about risks versus benefits – that's what we as physicians are trained to analyse. And patients can rest assured that we'll monitor them closely for side effects and address any that are unpleasant, either by treating them or by trying a different medication.

But to return to the program in Philadelphia, my firm belief is that if patients don't have strong enough incentives to take their medications so they can live longer, healthier lives, then the long-term benefits of providing a financial incentive are likely to be minimal. At the outset, the rewards may be substantial enough to elicit a response. But one isolated system or patient study is not an accurate depiction of the real-life scenario: patients will have to be taking these medications for decades.

Although a simple financial incentives program has its appeal, its complications abound. What's worse, it seems to be saying to society: as physicians, we tell our patients that not only do we work to care for them, but we'll now pay them to take better care of themselves. And by the way, for all you medication-compliant patients out there, you can have the inherent reward of a longer, healthier life, but we're not going to bother sending you money. This seems like some sort of implied punishment.

But more generally, what advice can be given to doctors with non-compliant patients? Dr John Steiner has written a paper on the matter: 'Be compassionate,' he urges doctors. 'Understand what a complicated balancing act it is for patients.' He's surely right **on that score**. Doctors and patients need to work together to figure out what is reasonable and realistic, prioritizing which measures are most important. For one patient, taking the diabetes pills might be more crucial than trying to quit smoking. For another, treating depression is more critical than treating cholesterol. 'Improving compliance is a team sport,' Dr Steiner adds. 'Input from nurses, care managers, social workers and pharmacists is critical.'

When discussing the complicated nuances of compliance with my students, I give the example of my grandmother. A thrifty, no-nonsense woman, she routinely sliced all the cholesterol and heart disease pills her doctor prescribed in half, taking only half the dose. If I questioned this, she'd wave me off with, 'What do those doctors know, anyway?' Sadly, she died suddenly, aged 87, most likely of a massive heart attack. Had she taken her medicines at the appropriate doses, she might have survived it. But then maybe she'd have died a more painful death from some other ailment. Her biggest fear had always been ending up dependent in a nursing home, and by luck or design, she was able to avoid that. Perhaps there was some wisdom in her 'non-compliance.'

Text 2: Questions 15-22

15. In the first paragraph, what is the writer's attitude towards the new programme?

(A) He doubts that it is correctly named.

(B) He appreciates the reasons behind it.

(C) He is sceptical about whether it can work.

(D) He is more enthusiastic than some other doctors.

16. In the second paragraph, the writer suggests that one category of non-compliance is

(A) elderly patients who are given occasional assistance.

(B) patients who are over-prescribed with a certain drug.

(C) busy working people who mean to be compliant.

(D) people who are by nature wary of taking pills.

17. What problem with some patients is described in the third paragraph?

(A) They forget which prescribed medication is for which of their conditions.

(B) They fail to recognise that some medical conditions require ongoing treatment.

(C) They don't understand their treatment even when it's explained in simple terms.

(D) They believe that taking some prescribed pills means they don't need to take others.

18. What does the writer say about side effects to medication?

(A) Doctors need to have better plans in place if they develop.

(B) There is too much misleading information about them online.

(C) Fear of them can waste a lot of unnecessary consultation time.

(D) Patients need to be informed about the likelihood of them occurring.

19. In the fifth paragraph, what is the writer's reservation about the Philadelphia program?

(A) the long-term feasibility of the central idea

(B) the size of the financial incentives offered

(C) the types of medication that were targeted

(D) the particular sample chosen to participate

20. What objection to the program does the writer make in the sixth paragraph?

(A) It will be counter-productive.

(B) It will place heavy demands on doctors.

(C) It sends the wrong message to patients.

(D) It is a simplistic idea that falls down on its details.

21. The expression '**on that score**' in the seventh paragraph refers to

(A) a complex solution to patients' problems.

(B) a co-operative attitude amongst medical staff.

(C) a realistic assessment of why something happens.

(D) a recommended response to the concerns of patients.

22. The writer suggests that his grandmother

(A) may ultimately have benefited from her non-compliance.

(B) would have appreciated closer medical supervision.

(C) might have underestimated how ill she was.

(D) should have followed her doctor's advice.

END OF READING TEST

THIS BOOKLET WILL BE COLLECTED

READING SUB-TEST – ANSWER KEY

PART A: QUESTIONS 1-20

1	A
2	B
3	A
4	D
5	B
6	C
7	B
8	fine bore
9	water-based lubricant
10	tape
11	(a) syringe
12	15-30 minutes/mins OR fifteen-thirty minutes/mins
13	(turn) on(to) left side
14	(to) x-ray (department) OR (to) radiology
15	(a) feeding pump
16	stretch
17	gastroesophageal reflux
18	6/six Fr/French
19	breathlessness
20	(expressed) breast milk

Reading sub-test

Answer Key – Parts B & C

PART B: QUESTIONS 1-6

1	A	this should be reported.
2	C	communicate their intentions to others.
3	B	best practice procedures
4	A	if non-disclosure could adversely affect those involved
5	A	act as a reminder of their obligations.
6	C	mild dementia in women can remain undiagnosed.

PART C: QUESTIONS 7-14

7	C	the fact that it is used so extensively.
8	A	can be found everywhere.
9	C	the time from exposure to disease may cause delayed diagnosis.
10	D	the degree of contact with asbestos fibres considered harmful.
11	B	tend to remain in the pulmonary tissue.
12	B	infection control
13	B	may pose a future health threat.
14	C	asbestosis as an ongoing medical issue.

PART C: QUESTIONS 15-22

15	B	He appreciates the reasons behind it.
16	C	busy working people who mean to be compliant.
17	B	They fail to recognise that some medical conditions require ongoing treatment.
18	D	Patients need to be informed about the likelihood of them occurring.
19	A	the long-term feasibility of the central idea
20	C	It sends the wrong message to patients.
21	D	a recommended response to the concerns of patients.
22	A	may ultimately have benefited from her non-compliance.

Tetanus: Texts

Text A

Tetanus is a severe disease that can result in serious illness and death. Tetanus vaccination protects against the disease.

Tetanus (sometimes called lock-jaw) is a disease caused by the bacteria Clostridium tetani. Toxins made by the bacteria attack a person's nervous system. Although the disease is fairly uncommon, it can be fatal.

Early symptoms of tetanus include:

- Painful muscle contractions that begin in the jaw (lock jaw)
- Rigidity in neck, shoulder and back muscles
- Difficulty swallowing
- Violent generalized muscle spasms
- Convulsions
- Breathing difficulties

A person may have a fever and sometimes develop abnormal heart rhythms. Complications include pneumonia, broken bones (from the muscle spasms), respiratory failure and cardiac arrest.

There is no specific diagnostic laboratory test; diagnosis is made clinically. The spatula test is useful: touching the back of the pharynx with a spatula elicits a bite reflex in tetanus, instead of a gag reflex.

Text B

Tetanus Risk

Tetanus is an acute disease induced by the toxin tetanus bacilli, the spores of which are present in soil.

A TETANUS-PRONE WOUND IS:

- any wound or burn that requires surgical intervention that is delayed for > 6 hours
- any wound or burn at any interval after injury that shows one or more of the following characteristics:
  - a significant degree of tissue damage
  - puncture-type wound particularly where there has been contact with soil or organic matter which is likely to harbour tetanus organisms
- any wound from compound fractures
- any wound containing foreign bodies
- any wound or burn in patients who have systemic sepsis
- any bite wound
- any wound from tooth re-implantation

Intravenous drug users are at greater risk of tetanus. Every opportunity should be taken to ensure that they are fully protected against tetanus. Booster doses should be given if there is any doubt about their immunisation status.

Immunosuppressed patients may not be adequately protected against tetanus, despite having been fully immunised. They should be managed as if they were incompletely immunised.

Text C

Tetanus Immunisation following injuries

Thorough cleaning of the wound is essential irrespective of the immunisation history of the patient, and appropriate antibiotics should be prescribed.

Immunisation Status	**Clean Wound**	**Tetanus-prone wound**	
Vaccine		**Vaccine**	**Human Tetanus Immunoglobulin (HTIG)**
Fully immunised[1]	Not required	Not required	Only if high risk[2]
Primary immunisation complete, boosters incomplete but up to date	Not required	Not required	Only if high risk[2]
Primary immunisation incomplete or boosters not up to date	Reinforcing dose and further doses to complete recommended schedule	Reinforcing dose and further doses to complete recommended schedule	Yes (opposite limb to vaccine)
Not immunised or immunisation status not known/uncertain[3]	Immediate dose of vaccine followed by completion of full 5-dose course	Immediate dose of vaccine followed by completion of full 5-dose course	Yes (opposite limb to vaccine)

Notes

1. has received total of 5 doses of vaccine at appropriate intervals
2. heavy contamination with material likely to contain tetanus spores and/or extensive devitalised tissue
3. immunosuppressed patients presenting with a tetanus-prone wound should always be managed as if they were incompletely immunised

Text D

Human Tetanus Immunoglobulin (HTIG)

Indications

- treatment of clinically suspected cases of tetanus
- prevention of tetanus in high-risk, tetanus-prone wounds

Dose

Available in 1ml ampoules containing 250IU

Prevention Dose	Treatment Dose
250 IU by IM injection[1] Or 500 IU by IM injection[1] if >24 hours since injury/risk of heavy contamination/burns	5,000 – 10,000 IU by IV infusion Or 150 IU/kg by IM injection[1] (given in multiple sites) if IV preparation unavailable

[1] Due to its viscosity, HTIG should be administered slowly, using a 23 gauge needle

Contraindications

- Confirmed anaphylactic reaction to tetanus containing vaccine
- Confirmed anaphylactic reaction to neomycin, streptomycin or polymyxin B

Adverse reactions

Local – pain, erythema, induration (Arthus-type reaction)

General – pyrexia, hypotonic-hyporesponsive episode, persistent crying

END OF PART A

THIS TEXT BOOKLET WILL BE COLLECTED

OCCUPATIONAL ENGLISH TEST

READING SUB-TEST – QUESTION PAPER: PART A

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

MIDDLE NAMES:

PROFESSION:

VENUE:

TEST DATE:

Passport Photo

CANDIDATE DECLARATION

By signing this, you agree not to disclose or use in any way (other than to take the test) or assist any other person to disclose or use any OET test or sub-test content. If you cheat or assist in any cheating, use any unfair practice, break any of the rules or regulations, or ignore any advice or information, you may be disqualified and your results may not be issued at the sole discretion of CBLA. CBLA also reserves its right to take further disciplinary action against you and to pursue any other remedies permitted by law. If a candidate is suspected of and investigated for malpractice, their personal details and details of the investigation may be passed to a third party where required.

CANDIDATE SIGNATURE: _____

TIME: 15 MINUTES

INSTRUCTIONS TO CANDIDATES

DO NOT open this **Question Paper** or the **Text Booklet** until you are told to do so.

Write your answers on the spaces provided on this **Question Paper.**

You must answer the questions within the 15-minute time limit.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the 15 minutes, hand in this **Question Paper** and the **Text Booklet.**

DO NOT remove OET material from the test room.

SAMPLE

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Part A

TIME: 15 minutes

- Look at the four texts, **A-D**, in the separate **Text Booklet**.
- For each question, **1-20**, look through the texts, **A-D**, to find the relevant information.
- Write your answers on the spaces provided in this **Question Paper**.
- Answer all the questions within the 15-minute time limit.
- Your answers should be correctly spelt.

Questions 1-6

For each question, **1-6**, decide which text (**A**, **B**, **C** or **D**) the information comes from.

You may use any letter more than once.

In which text can you find information about

1 the type of injuries that may lead to tetanus? _____

2 signs that a patient may have tetanus? _____

3 how to decide whether a tetanus vaccine is necessary? _____

4 an alternative name for tetanus? _____

5 possible side-effects of a particular tetanus medication? _____

6 other conditions which are associated with tetanus? _____

Questions 7-13

Complete each of the sentences, **7-13**, with a word or short phrase from one of the texts.
Each answer may include words, numbers or both.

Patients at increased risk of tetanus:

7 If a patient has been touching _____ or earth, they are more susceptible to tetanus.

8 Any _____ lodged in the site of an injury will increase the likelihood of tetanus.

9 Patients with _____ fractures are prone to tetanus.

10 Delaying surgery on an injury or burn by more than _____ increases the probability of tetanus.

11 If a burns patient has been diagnosed with _____ they are more liable to contract tetanus.

12 A patient who is _____ or a regular recreational drug user will be at greater risk of tetanus.

Management of tetanus-prone injuries:

13 Clean the wound thoroughly and prescribe _____ if necessary, followed by tetanus vaccine and HTIG as appropriate.

Questions 14-20

Answer each of the questions, **14-20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

14 Where will a patient suffering from tetanus first experience muscle contractions?

_____

15 What can muscle spasms in tetanus patients sometimes lead to?

_____

16 If you test for tetanus using a spatula, what type of reaction will confirm the condition?

_____

17 How many times will you have to vaccinate a patient who needs a full course of tetanus vaccine?

_____

18 What should you give a drug user if you're uncertain of their vaccination history?

_____

19 What size of needle should you use to inject HTIG?

_____

20 What might a patient who experienced an adverse reaction to HTIG be unable to stop doing?

_____

END OF PART A
THIS QUESTION PAPER WILL BE COLLECTED

OCCUPATIONAL ENGLISH TEST

READING SUB-TEST – QUESTION PAPER: PARTS B & C

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

MIDDLE NAMES:

PROFESSION:

VENUE:

TEST DATE:

Passport Photo

CANDIDATE DECLARATION

By signing this, you agree not to disclose or use in any way (other than to take the test) or assist any other person to disclose or use any OET test or sub-test content. If you cheat or assist in any cheating, use any unfair practice, break any of the rules or regulations, or ignore any advice or information, you may be disqualified and your results may not be issued at the sole discretion of CBLA. CBLA also reserves its right to take further disciplinary action against you and to pursue any other remedies permitted by law. If a candidate is suspected of and investigated for malpractice, their personal details and details of the investigation may be passed to a third party where required.

CANDIDATE SIGNATURE: _____

TIME: 45 MINUTES

INSTRUCTIONS TO CANDIDATES

DO NOT open this **Question Paper** until you are told to do so.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the test, hand in this **Question Paper**.

HOW TO ANSWER THE QUESTIONS:

Mark your answers on this **Question Paper** by filling in the circle using a 2B pencil. **Example:** Ⓐ Ⓑ Ⓒ

SAMPLE

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Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For **questions 1-6**, choose answer (**A**, **B** or **C**) which you think fits best according to the text.

1. Nursing staff can remove a dressing if

(A) a member of the surgical team is present.

(B) there is severe leakage from the wound.

(C) they believe that the wound has healed.

Post-operative dressings

Dressings are an important component of post-operative wound management. Any dressings applied during surgery have been done in sterile conditions and should ideally be left in place, as stipulated by the surgical team. It is acceptable for initial dressings to be removed prematurely in order to have the wound reviewed and, in certain situations, apply a new dressing. These situations include when the dressing is no longer serving its purpose (i.e. dressing falling off, excessive exudate soaking through the dressing and resulting in a suboptimal wound healing environment) or when a wound complication is suspected.

2. As explained in the protocol, the position of the RUM container will ideally

(A) encourage participation in the scheme.

(B) emphasise the value of recycling.

(C) facilitate public access to it.

Unwanted medicine: pharmacy collection protocol

A Returned Unwanted Medicine (RUM) Project approved container will be delivered by the wholesaler to the participating pharmacy.

The container is to be kept in a section of the dispensary or in a room or enclosure in the pharmacy to which the public does not have access. The container may be placed in a visible position, but out of reach of the public, as this will reinforce the message that unwanted prescription drugs can be returned to the pharmacy and that the returned medicines will not be recycled.

Needles, other sharps and liquid cytotoxic products should not be placed in the container, but in one specifically designed for such waste.

3. The report mentioned in the memo suggests that

(A) data about patient errors may be incomplete.

(B) errors by hospital staff can often go unreported.

(C) errors in prescriptions pose the greatest threat to patients.

Memo: Report on oral anti-cancer medications

Nurse Unit Managers are directed to review their systems for the administration of oral anti-cancer drugs, and the reporting of drug errors. Serious concerns have been raised in a recent report drawing on a national survey of pharmacists.

Please note the following paragraph quoted from the report:

> Incorrect doses of oral anti-cancer medicines can have fatal consequences. Over the previous four years, there were three deaths and 400 patient safety issues involving oral anti-cancer medicines. Half of the reports concerned the wrong dosage, frequency, quantity or duration of oral anti-cancer treatment. Of further concern is that errors on the part of patients may be under-reported. In light of these reports, there is clearly a need for improved systems covering the management of patients receiving oral therapies.

4. What point does the training manual make about anaesthesia workstations?

(A) Parts of the equipment have been shown to be vulnerable to failure.

(B) There are several ways of ensuring that the ventilator is working effectively.

(C) Monitoring by health professionals is a reliable way to maintain patient safety.

Anaesthesia Workstations

Studies on safety in anaesthesia have documented that human vigilance alone is inadequate to ensure patient safety and have underscored the importance of monitoring devices. These findings are reflected in improved standards for equipment design, guidelines for patient monitoring and reduced malpractice premiums for the use of capnography and pulse oximetry during anaesthesia. Anaesthesia workstations integrate ventilator technology with patient monitors and alarms to help prevent patient injury in the unlikely event of a ventilator failure. Furthermore, since the reservoir bag is part of the circuit during mechanical ventilation, the visible movement of the reservoir bag is confirmation that the ventilator is functioning.

5. In cases of snakebite, the flying doctor should be aware of

(A) where to access specific antivenoms.

(B) the appropriate method for wound cleaning.

(C) the patients most likely to suffer complications.

Memo to Flying Doctor staff: Antivenoms for snakebite

Before starting treatment:

- Do not wash the snakebite site.
- If possible, determine the type of snake by using a 'snake-venom detection kit' to test a bite site swab or, in systemic envenoming, the person's urine. If venom detection is not available or has proved negative, seek advice from a poisons information centre.
- Testing blood for venom is not reliable.
- Assess the degree of envenoming; not all confirmed snakebites will result in systemic envenoming; risk varies with the species of snake.
- People with pre-existing renal, hepatic, cardiac or respiratory impairment and those taking anticoagulant or antiplatelet drugs may have an increased risk of serious outcome from snakebite. Children are also especially at increased risk of severe envenoming because of smaller body mass and the likelihood of physical activity immediately after a bite.

6. What was the purpose of the BMTEC forum?

(A) to propose a new way of carrying out cleaning audits

(B) to draw conclusions from the results of cleaning audits

(C) to encourage more groups to undertake cleaning audits

Cleaning Audits

Three rounds of environmental cleaning audits were completed in 2013-2014. Key personnel in each facility were surveyed to assess the understanding of environmental cleaning from the perspective of the nurse unit manager, environmental services manager and the director of clinical governance. Each facility received a report about their environmental cleaning audits and lessons learned from the surveys. Data from the 15 units were also provided to each facility for comparison purposes.

The knowledge and experiences from the audits were shared at the BMTEC Forum in August 2014. This forum allowed environmental services managers, cleaners, nurses and clinical governance to discuss the application of the standards and promote new and improved cleaning practice. The second day of the forum focused on auditor training and technique with the view of enhancing internal environmental cleaning auditing by the participating groups.

Part C

In this part of the test, there are two texts about different aspects of healthcare. For **questions 7-22**, choose the answer (**A**, **B**, **C** or **D**) which you think fits best according to the text.

Text 1: Does homeopathy 'work'?

For many, homeopathy is simply unscientific, but regular users hold a very different view.

Homeopathy works by giving patients very dilute substances that, in larger doses, would cause the very symptoms that need curing. Taking small doses of these substances – derived from plants, animals or minerals – strengthens the body's ability to heal and increases resistance to illness or infection. Or that is the theory. The debate about its effectiveness is nothing new. Recently, Australia's National Health and Medical Research Council (NHMRC) released a paper which found there were 'no health conditions for which there was reliable evidence that homeopathy was effective'. This echoed a report from the UK House of Commons which said that the evidence failed to show a 'credible physiological mode of action' for homeopathic products, and that what data were available showed homeopathic products to be no better than placebo. Yet Australians spend at least $11 million per year on homeopathy.

So what's going on? If Australians – and citizens of many other nations around the world – are voting with their wallets, does this mean homeopathy must be doing something right? 'For me, the crux of the debate is a disconnect between how the scientific and medical community view homeopathy, and what many in the wider community are getting out of it,' says Professor Alex Broom of the University of Queensland. 'The really interesting question is how can we possibly have something that people think works, when to all intents and purposes, from a scientific perspective, it doesn't?'

Part of homeopathy's appeal may lie in the nature of the patient-practitioner consultation. In contrast to a typical 15-minute GP consultation, a first homeopathy consultation might take an hour and a half. 'We don't just look at an individual symptom in isolation. For us, that symptom is part of someone's overall health condition,' says Greg Cope, spokesman for the Australian Homeopathic Association. 'Often we'll have a consultation with someone and find details their GP simply didn't have time to.' Writer Johanna Ashmore is a case in point. She sees her homeopath for a one-hour monthly consultation. 'I feel, if I go and say I've got this health concern, she's going to treat my body to fight it rather than just treat the symptom.'

Most people visit a homeopath after having received a diagnosis from a 'mainstream' practitioner, often because they want an alternative choice to medication, says Greg Cope. 'Generally speaking, for a homeopath, their preference is if someone has a diagnosis from a medical practitioner before starting homeopathic treatment, so it's rare for someone to come and see us with an undiagnosed condition and certainly if they do come undiagnosed, we'd want to refer them on and get that medical evaluation before starting a course of treatment,' he says.

Given that homeopathic medicines are by their very nature incredibly dilute – and, some might argue, diluted beyond all hope of efficacy – they are unlikely to cause any adverse effects, so where's the harm? Professor Paul Glasziou, chair of the NHMRC's Homeopathy Working Committee, says that while financial cost is one harm, potentially more harmful are the non-financial costs associated with missing out on effective treatments. 'If it's just a cold, I'm not too worried. But if it's for a serious illness, you may not be taking disease-modifying treatments, and most worrying is things like HIV which affect not only you, but people around you,' says Glasziou. This is a particular concern with homeopathic vaccines, he says, which jeopardise the 'herd immunity' – the immunity of a significant proportion of the population – which is crucial in containing outbreaks of vaccine-preventable diseases.

The question of a placebo effect inevitably arises, as studies repeatedly seem to suggest that whatever benefits are being derived from homeopathy are more a product of patient faith rather than of any active ingredient of the medications. However, Greg Cope dismisses this argument, pointing out that homeopathy appears to benefit even the sceptics: 'We might see kids first, then perhaps Mum and after a couple of years, Dad will follow and, even though he's only there reluctantly, we get **wonderful outcomes**. This cannot be explained simply by the placebo effect.' As a patient, Johanna Ashmore is aware scientific research does little to support homeopathy but can still see its benefits. 'If seeing my homeopath each month improves my health, I'm happy. I don't care how it works, even if it's all in the mind – I just know that it does.'

But if so many people around the world are placing their faith in homeopathy, despite the evidence against it, Broom questions why homeopathy seeks scientific validation. The problem, as he sees it, lies in the fact that 'if you're going to dance with conventional medicine and say "we want to be proven to be effective in dealing with discrete physiological conditions", then you indeed do have to show efficacy. In my view **this** is not about broader credibility per se, it's about scientific and medical credibility – there's actually quite a lot of cultural credibility surrounding homeopathy within the community but that's not replicated in the scientific literature.'

Text 1: Questions 7-14

7. The two reports mentioned in the first paragraph both concluded that homeopathy

(A) could be harmful if not used appropriately.

(B) merely works on the same basis as the placebo effect.

(C) lacks any form of convincing proof of its value as a treatment.

(D) would require further investigation before it was fully understood.

8. When commenting on the popularity of homeopathy, Professor Broom shows his

(A) surprise at people's willingness to put their trust in it.

(B) frustration at scientists' inability to explain their views on it.

(C) acceptance of the view that the subject may merit further study.

(D) concern over the risks people face when receiving such treatment.

9. Johanna Ashmore's views on homeopathy highlight

(A) how practitioners put their patients at ease.

(B) the key attraction of the approach for patients.

(C) how it suits patients with a range of health problems.

(D) the opportunities to improve patient care which GPs miss.

10. In the fourth paragraph, it is suggested that visits to homeopaths

(A) occasionally depend on a referral from a mainstream doctor.

(B) frequently result from a patient's treatment preferences.

(C) should be preceded by a visit to a relevant specialist.

(D) often reveal previously overlooked medical problems.

11. What particularly concerns Professor Glasziou?

(A) the risks to patients of relying on homeopathic vaccinations

(B) the mistaken view that homeopathic treatments can only do good

(C) the way that homeopathic remedies endanger more than just the user

(D) the ineffectiveness of homeopathic remedies against even minor illnesses

12. Greg Cope uses the expression '**wonderful outcomes**' to underline

(A) the ability of homeopathy to defy its scientific critics.

(B) the value of his patients' belief in the whole process.

(C) the claim that he has solid proof that homeopathy works.

(D) the way positive results can be achieved despite people's doubts.

13. From the comments quoted in the sixth paragraph, it is clear that Johanna Ashmore is

(A) prepared to accept that homeopathy may depend on psychological factors.

(B) happy to admit that she was uncertain at first about proceeding.

(C) sceptical about the evidence against homeopathic remedies.

(D) confident that research will eventually validate homeopathy.

14. What does the word '**this**' in the final paragraph refer to?

(A) the continuing inability of homeopathy to gain scientific credibility

(B) the suggestion that the scientific credibility of homeopathy is in doubt

(C) the idea that there is no need to pursue scientific acceptance for homeopathy

(D) the motivation behind the desire for homeopathy to gain scientific acceptance

Text 2: Brain-controlled prosthetics

Paralysed from the neck down by a stroke, Cathy Hutchinson stared fixedly at a drinking straw in a bottle on the table in front of her. A cable rose from the top of her head, connecting her to a robot arm, but her gaze never wavered as she mentally guided the robot arm, which was opposite her, to close its grippers around the bottle, then slowly lift the vessel towards her mouth. Only when she finally managed to take a sip did her face relax. This example illustrates the strides being taken in brain-controlled prosthetics. But Hutchinson's focused stare also illustrates the one crucial feature still missing from prosthetics. Her eyes could tell her where the arm was, but she couldn't feel what it was doing.

Prosthetics researchers are now trying to create prosthetics that can 'feel'. It's a **daunting** task: the researchers have managed to read signals from the brain; now they must write information into the nervous system. Touch encompasses a complicated mix of information – everything from the soft prickliness of wool to the slipping of a sweaty soft-drink can. The sensations arise from a host of receptors in the skin, which detect texture, vibration, pain, temperature and shape, as well as from receptors in the muscles, joints and tendons that contribute to 'proprioception' – the sense of where a limb is in space. Prosthetics are being outfitted with sensors that can gather many of these sensations, but the challenge is to get the resulting signals flowing to the correct part of the brain.

For people who have had limbs amputated, the obvious way to achieve that is to route the signals into the remaining nerves in the stump, the part of the limb left after amputation. Ken Horch, a neuroprosthetics researcher, has done just that by threading electrodes into the nerves in stumps then stimulating them with a tiny current, so that patients felt like their fingers were moving or being touched. The technique can even allow patients to distinguish basic features of objects: a man who had lost his lower arms was able to determine the difference between blocks made of wood or foam rubber by using a sensor-equipped prosthetic hand. He correctly identified the objects' size and softness more than twice as often as would have been expected by chance. Information about force and finger position was delivered from the prosthetic to a computer, which prompted stimulation of electrodes implanted in his upper-arm nerves.

As promising as this result was, researchers will probably need to stimulate hundreds or thousands of nerve fibres to create complex sensations, and they'll need to keep the devices working for many years if they are to minimise the number of surgeries required to replace them as they wear out. To get around this, some researchers are instead trying to give patients sensory feedback by touching their skin. The technique was discovered by accident by researcher Todd Kuiken. The idea was to rewire arm nerves that used to serve the hand, for example, to muscles in other parts of the body. When the patient thought about closing his or her hand, the newly targeted muscle would contract and generate an electric signal, driving movement of the prosthetic.

However, this technique won't work for stroke patients like Cathy Hutchinson. So some researchers are skipping directly to the brain. In principle, this should be straightforward. Because signals from specific parts of the body go to specific parts of the brain, scientists should be able to create sensations of touch or proprioception in the limb by directly activating the neurons that normally receive those signals. However, with electrical stimulation, all neurons close to the electrode's tip are activated indiscriminately, so 'even if I had the sharpest needle in the Universe, that could create unintended effects', says Arto Nurmikko, a neuroengineer. For example, an attempt to create sensation in one finger might produce sensation in other parts of the hand as well, he says.

Nurmikko and other researchers are therefore using light, in place of electricity, to activate highly specific groups of neurons and recreate a sense of touch. They trained a monkey to remove its hand from a pad when it vibrated. When the team then stimulated the part of its brain that receives tactile information from the hand with a light source implanted in its skull, the monkey lifted its hand off the pad about 90% of the time. The use of such techniques in humans is still probably 10–20 years away, but it is a promising strategy.

Even if such techniques can be made to work, it's unclear how closely they will approximate natural sensations. Tingles, pokes and vibrations are still **a far cry from** the complicated sensations that we feel when closing a hand over an apple, or running a finger along a table's edge. But patients don't need a perfect sense of touch, says Douglas Weber, a bioengineer. Simply having enough feedback to improve their control of grasp could help people to perform tasks such as picking up a glass of water, he explains. He goes on to say that patients who wear cochlear implants, for example, are often happy to regain enough hearing to hold a phone conversation, even if they're still unable to distinguish musical subtleties.

Text 2: Questions 15-22

15. What do we learn about the experiment Cathy Hutchinson took part in?

(A) It required intense concentration.

(B) It failed to achieve what it had set out to do.

(C) It could be done more quickly given practice.

(D) It was the first time that it had been attempted.

16. The task facing researchers is described as '**daunting**' because

(A) signals from the brain can be misunderstood.

(B) it is hard to link muscle receptors with each other.

(C) some aspects of touch are too difficult to reproduce.

(D) the connections between sensors and the brain need to be exact.

17. What is said about the experiment done on the patient in the third paragraph?

(A) There was statistical evidence that it was successful.

(B) It enabled the patient to have a wide range of feeling.

(C) Its success depended on when amputation had taken place.

(D) It required the use of a specially developed computer program.

18. What drawback does the writer mention in the fourth paragraph?

(A) The devices have a high failure rate.

(B) Patients might have to undergo too many operations.

(C) It would only be possible to create rather simple sensations.

(D) The research into the new technique hasn't been rigorous enough.

19. What point is made in the fifth paragraph?

(A) Severed nerves may be able to be reconnected.

(B) More research needs to be done on stroke victims.

(C) Scientists' previous ideas about the brain have been overturned.

(D) It is difficult for scientists to pinpoint precise areas with an electrode.

20. What do we learn about the experiment that made use of light?

(A) It can easily be replicated in humans.

(B) It worked as well as could be expected.

(C) It may have more potential than electrical stimulation.

(D) It required more complex surgery than previous experiments.

21. In the final paragraph, the writer uses the phrase '**a far cry from**' to underline

(A) how much more there is to achieve.

(B) how complex experiments have become.

(C) the need to reduce people's expectations.

(D) the differences between types of artificial sensation.

22. Why does Weber give the example of a cochlear implant?

(A) to underline the need for a similar breakthrough in prosthetics

(B) to illustrate the fact that some sensation is better than none

(C) to highlight the advances made in other areas of medicine

(D) to demonstrate the ability of the body to relearn skills

END OF READING TEST
THIS BOOKLET WILL BE COLLECTED

Reading sub-test

Answer Key – Part A

READING SUB-TEST – ANSWER KEY

PART A: QUESTIONS 1-20

1 B

2 A

3 C

4 A

5 D

6 A

7 organic matter

8 foreign bodies

9 compound

10 6/six hours

11 systemic sepsis

12 immuno(-)suppressed

13 antibiotics

14 (in) (the) jaw

15 broken bones

16 (a) bite reflex

17 5/five (times)

18 (a) booster dose OR booster doses

19 twenty-three/23 gauge

20 crying

Reading sub-test

Answer Key – Parts B & C

PART B: QUESTIONS 1-6

1	B	there is severe leakage from the wound.
2	A	encourage participation in the scheme.
3	A	data about patient errors may be incomplete.
4	B	There are several ways of ensuring that the ventilator is working effectively.
5	C	the patients most likely to suffer complications.
6	B	to draw conclusions from the results of cleaning audits

PART C: QUESTIONS 7-14

7	C	lacks any form of convincing proof of its value as a treatment.
8	A	surprise at people's willingness to put their trust in it.
9	B	the key attraction of the approach for patients.
10	B	frequently result from a patient's treatment preferences.
11	C	the way that homeopathic remedies endanger more than just the user
12	D	the way positive results can be achieved despite people's doubts.
13	A	prepared to accept that homeopathy may depend on psychological factors.
14	D	the motivation behind the desire for homeopathy to gain scientific acceptance

PART C: QUESTIONS 15-22

15	A	It required intense concentration.
16	D	the connections between sensors and the brain need to be exact.
17	A	There was statistical evidence that it was successful.
18	B	Patients might have to undergo too many operations.
19	D	It is difficult for scientists to pinpoint precise areas with an electrode.
20	C	It may have more potential than electrical stimulation.
21	A	how much more there is to achieve.
22	B	to illustrate the fact that some sensation is better than none

OCCUPATIONAL ENGLISH TEST

Test 3

READING SUB-TEST – TEXT BOOKLET: PART A

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

MIDDLE NAMES:

PROFESSION:

VENUE:

TEST DATE:

Passport Photo

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INSTRUCTIONS TO CANDIDATES

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Text A

Necrotizing fasciitis (NF) is a severe, rare, potentially lethal soft tissue infection that develops in the scrotum and perineum, the abdominal wall, or the extremities. The infection progresses rapidly, and septic shock may ensue; hence, the mortality rate is high (median mortality 32.2%). NF is classified into four types, depending on microbiological findings.

Table 1

Classification of responsible pathogens according to type of infection

Microbiological type	Pathogens	Site of infection	Co-morbidities
Type 1 (polymicrobial)	Obligate and facultative anaerobes	Trunk and perineum	Diabetes mellitus
Type 2 (monomicrobial)	Beta-hemolytic streptococcus A	Limbs	
Type 3	*Clostridium* species Gram-negative bacteria Vibrios spp. Aeromonas hydrophila	Limbs, trunk and perineum	Trauma Seafood consumption (for *Aeromonas*)
Type 4	*Candida* spp. Zygomycetes	Limbs, trunk, perineum	Immuno-suppression

Text B

Antibiotic treatment for NF

Type 1

- Initial treatment includes ampicillin or ampicillin–sulbactam combined with metronidazole or clindamycin.
- Broad gram-negative coverage is necessary as an initial empirical therapy for patients who have recently been treated with antibiotics, or been hospitalized. In such cases, antibiotics such as ampicillin–sulbactam, piperacillin–tazobactam, ticarcillin–clavulanate acid, third or fourth generation cephalosporins, or carbapenems are used, and at a higher dosage.

Type 2

- First or second generation of cephalosporins are used for the coverage of methicillin-sensitive Staphylococcus aureus (MSSA).
- MRSA tends to be covered by vancomycin, or daptomycin and linezolid in cases where S. aureus is resistant to vancomycin.

Type 3

- NF should be managed with clindamycin and penicillin, which kill the Clostridium species.
- If Vibrio infection is suspected, the early use of tetracyclines (including doxycycline and minocycline) and third-generation cephalosporins is crucial for the survival of the patient, since these antibiotics have been shown to reduce the mortality rate drastically.

Type 4

- Can be treated with amphotericin B or fluoroconazoles, but the results of this treatment are generally disappointing.

Antibiotics should be administered for up to 5 days after local signs and symptoms have resolved. The mean duration of antibiotic therapy for NF is 4–6 weeks.

Text C

Supportive care in an ICU is critical to NF survival. This involves fluid resuscitation, cardiac monitoring, aggressive wound care, and adequate nutritional support. Patients with NF are in a catabolic state and require increased caloric intake to combat infection. This can be delivered orally or via nasogastric tube, peg tube, or intravenous hyperalimentation. This should begin immediately (within the first 24 hours of hospitalization). Prompt and aggressive support has been shown to lower complication rates. Baseline and repeated monitoring of albumin, prealbumin, transferrin, blood urea nitrogen, and triglycerides should be performed to ensure the patient is receiving adequate nutrition.

Wound care is also an important concern. Advanced wound dressings have replaced wet-to-dry dressings. These dressings promote granulation tissue formation and speed healing. Advanced wound dressings may lend to healing or prepare the wound bed for grafting. A healthy wound bed increases the chances of split-thickness skin graft take. Vacuum-assisted closure (VAC) was recently reported to be effective in a patient whose cardiac status was too precarious to undergo a long surgical reconstruction operation. With the VAC., the patient's wound decreased in size, and the VAC was thought to aid in local management of infection and improve granulation tissue.

Text D

Advice to give the patient before discharge

- Help arrange the patient's aftercare, including home health care and instruction regarding wound management, social services to promote adjustment to lifestyle changes and financial concerns, and physical therapy sessions to help rebuild strength and promote the return to optimal physical health.
- The life-threatening nature of NF, scarring caused by the disease, and in some cases the need for limb amputation can alter the patient's attitude and viewpoint, so be sure to take a holistic approach when dealing with the patient and family.

Remind the diabetic patient to

- control blood glucose levels, keeping the glycated haemoglobin (HbAlc) level to 7% or less.
- keep needles capped until use and not to reuse needles.
- clean the skin thoroughly before blood glucose testing or insulin in¬jection, and to use alcohol pads to clean the area afterward.

END OF PART A

THIS TEXT BOOKLET WILL BE COLLECTED

OCCUPATIONAL ENGLISH TEST

READING SUB-TEST – QUESTION PAPER: PART A

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

MIDDLE NAMES:

PROFESSION:

VENUE:

TEST DATE:

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CANDIDATE SIGNATURE: _____

TIME: 15 MINUTES

INSTRUCTIONS TO CANDIDATES

DO NOT open this **Question Paper** or the **Text Booklet** until you are told to do so.

Write your answers on the spaces provided on this **Question Paper.**

You must answer the questions within the 15-minute time limit.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the 15 minutes, hand in this **Question Paper** and the **Text Booklet.**

DO NOT remove OET material from the test room.

SAMPLE

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Part A

TIME: 15 minutes

- Look at the four texts, **A-D**, in the separate **Text Booklet**.
- For each question, **1-20**, look through the texts, **A-D**, to find the relevant information.
- Write your answers on the spaces provided in this **Question Paper**.
- Answer all the questions within the 15-minute time limit.
- Your answers should be correctly spelt.

Questions 1-7

For each question, **1-7**, decide which text (**A**, **B**, **C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about

1 the drug treatment required? _____

2 which parts of the body can be affected? _____

3 the various ways calories can be introduced? _____

4 who to contact to help the patient after they leave hospital? _____

5 what kind of dressing to use? _____

6 how long to give drug therapy to the patient? _____

7 what advice to give the patient regarding needle use? _____

Questions 8-14

Complete each of the sentences, **8-14**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

Patients at increased risk of tetanus:

8 Which two drugs can you use to treat the clostridium species of pathogen?

_____

9 Which common metabolic condition may occur with NF?

_____

10 What complication can a patient suffer from if NF isn't treated quickly enough?

_____

11 What procedure can you use with a wound if the patient can't be operated on?

_____

12 What should the patient be told to use to clean an injection site?

_____

13 Which two drugs can be used if you can't use vancomycin?

_____

14 What kind of infection should you use tetracyclines for?

_____

Questions 15-20

Answer each of the questions, **15-20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

15 The average proportion of patients who die as a result of contracting NF is

_____.

16 Patients who have eaten _____ may be infected with Aeromonas hydrophilia.

17 Patients with Type 2 infection usually present with infected

_____.

18 Type 1 NF is also known as _____.

19 The patient needs to be aware of the need to keep glycated haemoglobin levels lower than _____.

20 The patient will need a course of _____ to regain fitness levels after returning home.

END OF PART A
THIS QUESTION PAPER WILL BE COLLECTED

OCCUPATIONAL ENGLISH TEST

READING SUB-TEST – QUESTION PAPER: PARTS B & C

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

MIDDLE NAMES:

PROFESSION:

VENUE:

TEST DATE:

Passport Photo

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CANDIDATE SIGNATURE: _____

TIME: 45 MINUTES

INSTRUCTIONS TO CANDIDATES

DO NOT open this **Question Paper** until you are told to do so.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the test, hand in this **Question Paper**.

HOW TO ANSWER THE QUESTIONS:

Mark your answers on this **Question Paper** by filling in the circle using a 2B pencil. **Example:** Ⓐ Ⓑ Ⓒ

SAMPLE

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Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For **questions 1-6**, choose answer (**A**, **B** or **C**) which you think fits best according to the text.

1. The policy document tells us that

(A) stop dates aren't relevant in all circumstances.

(B) anyone using EPMA can disregard the request for a stop date.

(C) prescribers must know in advance of prescribing what the stop date should be.

Prescribing stop dates

Prescribers should write a review date or a stop date on the electronic prescribing system EPMA or the medicine chart for each antimicrobial agent prescribed. On the EPMA, there is a forced entry for stop dates on oral antimicrobials. There is not a forced stop date on EPMA for IV antimicrobial treatment – if the prescriber knows how long the course of IV should be, then the stop date can be filled in. If not known, then a review should be added to the additional information, e.g. 'review after 48 hrs'. If the prescriber decides treatment needs to continue beyond the stop date or course length indicated, then it is their responsibility to amend the chart. In critical care, it has been agreed that the routine use of review/stop dates on the charts is not always appropriate.

2. The guidelines inform us that personalised equipment for radiotherapy

(A) is advisable for all patients.

(B) improves precision during radiation.

(C) needs to be tested at the first consultation.

Guidelines: Radiotherapy Simulation Planning Appointment

The initial appointment may also be referred to as the Simulation Appointment. During this appointment you will discuss your patient's medical history and treatment options, and agree on a radiotherapy treatment plan. The first step is usually to take a CT scan of the area requiring treatment. The patient will meet the radiation oncologist, their registrar and radiation therapists. A decision will be made regarding the best and most comfortable position for treatment, and this will be replicated daily for the duration of the treatment. Depending on the area of the body to be treated, personalised equipment such as a face mask may be used to stabilise the patient's position. This equipment helps keep the patient comfortable and still during the treatment and makes the treatment more accurate.

3. The purpose of these instructions is to explain how to

(A) monitor an ECG reading.

(B) position electrodes correctly.

(C) handle an animal during an ECG procedure.

CT200CV Veterinarian Electrocardiograph User Manual

Animal connections

Good electrode connection is the most important factor in recording a high quality ECG. By following a few basic steps, consistent, clean recordings can be achieved.

1. Shave a patch on each forelimb of the animal at the contact site.
2. Clean the electrode sites with an alcohol swab or sterilising agent.
3. Attach clips to the ECG leads.
4. Place a small amount of ECG electrode gel on the metal electrode of the limb strap or adapter clip.
5. Pinch skin on animal and place clips on the shaved skin area of the animal being tested. The animal must be kept still.
6. Check the LCD display for a constant heart reading.
7. If there is no heart reading, you have a contact problem with one or more of the leads.
8. Recheck the leads and reapply the clips to the shaven skin of the animal.

4. The group known as 'impatient patients' are more likely to continue with a course of prescribed medication if

(A) their treatment can be completed over a reduced period of time.

(B) it is possible to link their treatment with a financial advantage.

(C) its short-term benefits are explained to them.

Medication adherence and impatient patients

A recent article addressed the behaviour of people who have a 'taste for the present rather than the future'. It proposed that these so-called 'impatient patients' are unlikely to adhere to medications that require use over an extended period. The article proposes that, an 'impatience genotype' exists and that assessing these patients' view of the future while stressing the immediate advantages of adherence may improve adherence rates more than emphasizing potentially distant complications. The authors suggest that rather than attempting to change the character of those who are 'impatient', it may be wise to ascertain the patient's individual priorities, particularly as they relate to immediate gains. For example, while advising an 'impatient' patient with diabetes, stressing improvement in visual acuity rather than avoidance of retinopathy may result in greater medication adherence rates. Additionally, linking the cost of frequently changing prescription lenses when visual acuity fluctuates with glycemic levels may sometimes provide the patient with an immediate financial motivation for improving adherence.

5. The memo reminds nursing staff to avoid

(A) x-raying a patient unless pH readings exceed 5.5.

(B) the use of a particular method of testing pH levels.

(C) reliance on pH testing in patients taking acid-inhibiting medication.

Checking the position of a nasogastric tube

It is essential to confirm the position of the tube in the stomach by one of the following:

- Testing pH of aspirate: gastric placement is indicated by a pH of less than 4, but may increase to between pH 4-6 if the patient is receiving acid-inhibiting drugs. Blue litmus paper is insufficiently sensitive to adequately distinguish between levels of acidity of aspirate.
- X-rays: will only confirm position at the time the X-ray is carried out. The tube may have moved by the time the patient has returned to the ward. In the absence of a positive aspirate test, where pH readings are more than 5.5, or in a patient who is unconscious or on a ventilator, an X-ray must be obtained to confirm the initial position of the nasogastric tube.

6. This extract informs us that

(A) the amount of oxytocin given will depend on how the patient reacts.

(B) the patient will go into labour as soon as oxytocin is administered.

(C) the staff should inspect the oxytocin pump before use.

Extract from guidelines: Oxytocin

1 Oxytocin Dosage and Administration

Parenteral drug products should be inspected visually for particulate matter and discoloration prior to administration, whenever solution and container permit. Dosage of Oxytocin is determined by the uterine response. The dosage information below is based upon various regimens and indications in general use.

1.1 Induction or Stimulation of Labour

Intravenous infusion (drip method) is the only acceptable method of administration for the induction or stimulation of labour. Accurate control of the rate of infusion flow is essential. An infusion pump or other such device and frequent monitoring of strength of contractions and foetal heart rate are necessary for the safe administration of Oxytocin for the induction or stimulation of labour. If uterine contractions become too powerful, the infusion can be abruptly stopped, and oxytocic stimulation of the uterine musculature will soon wane.

Part C

In this part of the test, there are two texts about different aspects of healthcare. For **questions 7-22**, choose the answer (**A**, **B**, **C** or **D**) which you think fits best according to the text.

Text 1: Phobia pills

An irrational fear, or phobia, can cause the heart to pound and the pulse to race. It can lead to a full-blown panic attack – and yet the sufferer is not in any real peril. All it takes is a glimpse of, for example, a spider's web for the mind and body to race into panicked overdrive. These fears are difficult to conquer, largely because, although there are no treatment guidelines specifically about phobias, the traditional way of helping the sufferer is to expose them to the fear numerous times. Through the cumulative effect of these experiences, sufferers should eventually feel an increasing sense of control over their phobia. For some people, the process is too protracted, but there may be a short cut. Drugs that work to boost learning may help someone with a phobia to 'detrain' their brain, losing the fearful associations that fuel the panic.

The brain's extraordinary ability to store new memories and forge associations is so well celebrated that its **dark side** is often disregarded. A feeling of contentment is easily evoked when we see a photo of loved ones, though the memory may sometimes be more idealised than exact. In the case of a phobia, however, a nasty experience with, say, spiders, that once triggered a panicked reaction, leads the feelings to resurge whenever the relevant cue is seen again. The current approach is exposure therapy, which uses a process called extinction learning. This involves people being gradually exposed to whatever triggers their phobia until they feel at ease with it. As the individual becomes more comfortable with each situation, the brain automatically creates a new memory – one that links the cue with reduced feelings of anxiety, rather than the sensations that mark the onset of a panic attack.

Unfortunately, while it is relatively easy to create a fear-based memory, expunging that fear is more complicated. Each exposure trial will involve a certain degree of distress in the patient, and although the process is carefully managed throughout to limit this, some psychotherapists have concluded that the treatment is unethical. Neuroscientists have been looking for new ways to speed up extinction learning **for that same reason**.

One such avenue is the use of 'cognitive enhancers' such as a drug called D-cycloserine or DCS. DCS slots into part of the brain's 'NMDA receptor' and seems to modulate the neurons' ability to adjust their signalling in response to events. This tuning of a neuron's firing is thought to be one of the key ways the brain stores memories, and, at very low doses, DCS appears to boost that process, improving our ability to learn. In 2004, a team from Emory University in Atlanta, USA, tested whether DCS could also help people with phobias. A pilot trial was conducted on 28 people undergoing specific exposure therapy for acrophobia – a fear of heights. Results showed that those given a small amount of DCS alongside their regular therapy were able to reduce their phobia to a greater extent than those given a placebo. Since then, other groups have replicated the finding in further trials.

For people undergoing exposure therapy, achieving just one of the steps on the long journey to overcoming their fears requires considerable perseverance, says Cristian Sirbu, a behavioural scientist and psychologist. Thanks to improvement being so slow, patients – often already anxious – tend to feel they have failed. But Sirbu thinks that DCS may make it possible to tackle the problem in a single 3-hour session, which is enough for the patient to make real headway and to leave with a feeling of satisfaction. However, some people have misgivings about this approach, claiming that as it doesn't directly undo the fearful response which is deep-seated in the memory, there is a very real risk of relapse.

Rather than simply attempting to overlay the fearful associations with new ones, Merel Kindt at the University of Amsterdam is instead trying to alter the associations at source. Kindt's studies into anxiety disorders are based on the idea that memories are not only vulnerable to alteration when they're first laid down, but, of key importance, also at later retrieval. This allows for memories to be 'updated', and these amended memories are re-consolidated by the effect of proteins which alter synaptic responses, thereby maintaining the strength of feeling associated with the original memory. Kindt's team has produced encouraging results with arachnophobic patients by giving them propranolol, a well-known and well-tolerated beta-blocker drug, while they looked at spiders. This blocked the effects of norepinephrine in the brain, disrupting the way the memory was put back into storage after being retrieved, as part of the process of reconsolidation. Participants reported that while they still don't like spiders, they were able to approach them. Kindt reports that the benefit was still there three months after the test ended.

Text 1: Questions 7-14

7. In the first paragraph, the writer says that conventional management of phobias can be problematic because of

(A) the lasting psychological effects of the treatment.

(B) the time required to identify the cause of the phobia.

(C) the limited choice of therapies available to professionals.

(D) the need for the phobia to be confronted repeatedly over time.

8. In the second paragraph, the writer uses the phrase '**dark side**' to reinforce the idea that

(A) memories of agreeable events tend to be inaccurate.

(B) positive memories can be negatively distorted over time.

(C) unhappy memories are often more detailed than happy ones.

(D) unpleasant memories are aroused in response to certain prompts.

9. In the second paragraph, extinction learning is explained as a process which

(A) makes use of an innate function of the brain.

(B) encourages patients to analyse their particular fears.

(C) shows patients how to react when having a panic attack.

(D) focuses on a previously little-understood part of the brain.

10. What does the phrase '**for that same reason**' refer to?

(A) the anxiety that patients feel during therapy

(B) complaints from patients who feel unsupported

(C) the conflicting ethical concerns of neuroscientists

(D) psychotherapists who take on unsuitable patients

11. In the fourth paragraph, we learn that the drug called DCS

(A) is unsafe to use except in small quantities.

(B) helps to control only certain types of phobias.

(C) affects how neurons in the brain react to stimuli.

(D) increases the emotional impact of certain events.

12. In the fifth paragraph, some critics believe that one drawback of using DCS is that

(A) its benefits are likely to be of limited duration.

(B) it is only helpful for certain types of personality.

(C) few patients are likely to complete the course of treatment.

(D) patients feel discouraged by their apparent lack of progress.

13. In the final paragraph, we learn that Kindt's studies into anxiety disorders focused on how

(A) proteins can affect memory retrieval.

(B) memories are superimposed on each other.

(C) negative memories can be reduced in frequency.

(D) the emotional force of a memory is naturally retained.

14. The writer suggests that propranolol may

(A) not offer a permanent solution for patients' phobias.

(B) increase patients' tolerance of key triggers.

(C) produce some beneficial side-effects.

(D) be inappropriate for certain phobias.

Text 2: Challenging medical thinking on placebos

Dr Damien Finniss, Associate Professor at Sydney University's Pain Management and Research Institute, was previously a physiotherapist. He regularly treated football players during training sessions using therapeutic ultrasound. 'One particular session', Finniss explains, 'I treated five or six athletes. I'd treat them for five or ten minutes and they'd say, "I feel much better" and run back onto the field. But at the end of the session, I realised the ultrasound wasn't on.' It was a light bulb moment that set Finniss on the path to becoming a leading researcher on the placebo effect.

Used to treat depression, psoriasis and Parkinson's, to name but a few, placebos have an image problem among medics. For years, the thinking has been that a placebo is useless unless the doctor convinces the patient that it's a genuine treatment – problematic for a profession that promotes informed consent. However, a new study casts doubt on this assumption and, along with a swathe of research showing some remarkable results with placebos, raises questions about whether they should now enter the mainstream as legitimate prescription items. The study examined five trials in which participants were told they were getting a placebo, and the conclusion was that doing so honestly can work.

'If the evidence is there, I don't see the harm in openly administering a placebo,' says Ben Colagiuri, a researcher at the University of Sydney. Colagiuri recently published a meta-analysis of thirteen studies which concluded that placebo sleeping pills, whose genuine counterparts **notch up** nearly three million prescriptions in Australia annually, significantly improve sleep quality. The use of placebos could therefore reduce medical costs and the burden of disease in terms of adverse reactions.

But the placebo effect isn't just about fake treatments. It's about raising patients' expectations of a positive result; something which also occurs with real drugs. Finniss cites the 'open-hidden' effect, whereby an analgesic can be twice as effective if the patient knows they're getting it, compared to receiving it unknowingly. 'Treatment is always part medical and part ritual,' says Finniss. This includes the austere consulting room and even the doctor's clothing. But behind the performance of healing is some strong science. Simply believing an analgesic will work activates the same brain regions as the genuine drug. 'Part of the outcome of what we do is the way we interact with patients,' says Finniss.

That interaction is also the focus of Colagiuri's research. He's looking into the 'nocebo' effect, when a patient's pessimism about a treatment becomes self-fulfilling. 'If you give a placebo, and warn only 50% of the patients about side effects, those you warn report more side effects,' says Colagiuri. He's aiming to reverse that by exploiting the psychology of food packaging. Products are labelled '98% fat-free' rather than '2% fat' because positive reference to the word 'fat' puts consumers off. Colagiuri is deploying similar tactics. A drug with a 30% chance of causing a side effect can be reframed as having a 70% chance of not causing it. 'You're giving the same information, but framing it a way that minimises negative expectations,' says Colagiuri.

There is also a body of research showing that a placebo can produce a genuine biological response that could affect the disease process itself. It can be traced back to a study from the 1970s, when psychologist Robert Ader was trying to condition taste-aversion in rats. He gave them a saccharine drink whilst simultaneously injecting Cytoxan, an immune-suppressant which causes nausea. The rats learned to hate the drink due to the nausea. But as Ader continued giving it to them, without Cytoxan, they began to die from infection. Their immune system had 'learned' to fail by repeated pairing of the drink with Cytoxan. Professor Andrea Evers of Leiden University is running a study that capitalises on this conditioning effect and may benefit patients with rheumatoid arthritis, which causes the immune system to attack the joints. Evers' patients are given the immunosuppressant methotrexate, but instead of always receiving the same dose, they get a higher dose followed by a lower one. The theory is that the higher dose will cause the body to link the medication with a damped-down immune system. The lower dose will then work because the body has 'learned' to curb immunity as a placebo response to taking the drug. Evers hopes it will mean effective drug regimes that use lower doses with fewer side effects.

The medical profession, however, remains less than enthusiastic about placebos. 'I'm one of two researchers in the country who speak on placebos, and I've been invited to lecture at just one university,' says Finniss. According to Charlotte Blease, a philosopher of science, this antipathy may go to the core of what it means to be a doctor. 'Medical education is largely about biomedical facts. 'Softer' sciences, such as psychology, get marginalised because it's the hard stuff that's associated with what it means to be a doctor.' The result, says Blease, is a large, placebo-shaped hole in the medical curriculum. 'There's a great deal of medical illiteracy about the placebo effect ... it's the science behind the art of medicine. Doctors need training in that.'

Text 2: Questions 15-22

15. A football training session sparked Dr Finniss' interest in the placebo effect because

(A) he saw for himself how it could work in practice.

(B) he took the opportunity to try out a theory about it.

(C) he made a discovery about how it works with groups.

(D) he realised he was more interested in research than treatment.

16. The writer suggests that doctors should be more willing to prescribe placebos now because

(A) research indicates that they are effective even without deceit.

(B) recent studies are more reliable than those conducted in the past.

(C) they have been accepted as a treatment by many in the profession.

(D) they have been shown to relieve symptoms in a wide range of conditions.

17. What is suggested about sleeping pills by the use of the verb '**notch up**'?

(A) they may have negative results

(B) they could easily be replaced

(C) they are extremely effective

(D) they are very widely used

18. What point does the writer make in the fourth paragraph?

(A) The way a treatment is presented is significant even if it is a placebo.

(B) The method by which a drug is administered is more important than its content.

(C) The theatrical side of medicine should not be allowed to detract from the science.

(D) The outcome of a placebo treatment is affected by whether the doctor believes in it.

19. In researching side effects, Colagiuri aims to

(A) discover whether placebos can cause them.

(B) reduce the number of people who experience them.

(C) make information about them more accessible to patients.

(D) investigate whether pessimistic patients are more likely to suffer from them.

20. What does the word '**it**' in the sixth paragraph refer to?

(A) a placebo treatment

(B) the disease process itself

(C) a growing body of research

(D) a genuine biological response

21. What does the writer tell us about Ader's and Evers' studies?

(A) Both involve gradually reducing the dosage of a drug.

(B) Evers is exploiting a response which Ader discovered by chance.

(C) Both examine the side effects caused by immunosuppressant drugs.

(D) Evers is investigating whether the human immune system reacts to placebos as Ader's rats did.

22. According to Charlotte Blease, placebos are omitted from medical training because

(A) there are so many practical subjects which need to be covered.

(B) those who train doctors do not believe that they work.

(C) they can be administered without specialist training.

(D) their effect is more psychological than physical.

END OF READING TEST
THIS BOOKLET WILL BE COLLECTED

READING SUB-TEST – ANSWER KEY

PART A: QUESTIONS 1-20

1	B
2	A
3	C
4	D
5	C
6	B
7	D
8	clindamycin (and) penicillin
9	diabetes mellitus
10	septic shock
11	VAC/ vacuum-assisted closure
12	alcohol pads
13	daptomycin (and) linezolid
14	vibrio (infection)
15	32.2%
16	seafood
17	limbs
18	polymicrobial
19	7%
20	physical therapy

Reading sub-test

Answer Key – Parts B & C

READING SUB-TEST – ANSWER KEY

PART B: QUESTIONS 1-6

1	A	stop dates aren't relevant in all circumstances.
2	B	improves precision during radiation.
3	B	position electrodes correctly.
4	C	its short-term benefits are explained to them.
5	B	the use of a particular method of testing pH levels.
6	A	the amount of oxytocin given will depend on how the patient reacts.

PART C: QUESTIONS 7-14

7	D	the need for the phobia to be confronted repeatedly over time.
8	D	unpleasant memories are aroused in response to certain prompts.
9	A	makes use of an innate function of the brain.
10	A	the anxiety that patients feel during therapy
11	C	affects how neurons in the brain react to stimuli.
12	A	its benefits are likely to be of limited duration.
13	D	the emotional force of a memory is naturally retained.
14	B	increase patients' tolerance of key triggers.

PART C: QUESTIONS 15-22

15	A	he saw for himself how it could work in practice.
16	A	research indicates that they are effective even without deceit.
17	D	they are very widely used
18	A	The way a treatment is presented is significant even if it is a placebo.
19	B	reduce the number of people who experience them.
20	C	a growing body of research
21	B	Evers is exploiting a response which Ader discovered by chance.
22	D	their effect is more psychological than physical.

OCCUPATIONAL ENGLISH TEST

Test 4

READING SUB-TEST – TEXT BOOKLET: PART A

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

OTHER NAMES:

PROFESSION:

VENUE:

TEST DATE:

Your details and photo will be printed here.

Passport Photo

CANDIDATE SIGNATURE: _____

SAMPLE

SAMPLE

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Text A

Fractures (buckle or break in the bone) often occur following direct or indirect injury, e.g. twisting, violence to bones. Clinically, fractures are either:

- closed, where the skin is intact, or
- compound, where there is a break in the overlying skin

Dislocation is where a bone is completely displaced from the joint. It often results from injuries away from the affected joint, e.g. elbow dislocation after falling on an outstretched hand.

Sprain is a partial disruption of a ligament or capsule of a joint.

Text B

Simple Fracture of Limbs

Immediate management:

- Halt any external haemorrhage by pressure bandage or direct pressure
- Immobilise the affected area
- Provide pain relief

Clinical assessment:

- Obtain complete patient history, including circumstances and method of injury
  - medication history – enquire about anticoagulant use, e.g. warfarin
- Perform standard clinical observations. Examine and record:
  - colour, warmth, movement, and sensation in hands and feet of injured limb(s)
- Perform physical examination

  Examine:
  - all places where it is painful
  - any wounds or swelling
  - colour of the whole limb (especially paleness or blue colour)
  - the skin over the fracture
  - range of movement
  - joint function above and below the injury site

  Check whether:
  - the limb is out of shape – compare one side with the other
  - the limb is warm
  - the limb (if swollen) is throbbing or getting bigger
  - peripheral pulses are palpable

Management:

- Splint the site of the fracture/dislocation using a plaster backslab to reduce pain
- Elevate the limb – a sling for arm injuries, a pillow for leg injuries
- If in doubt over an injury, treat as a fracture
- Administer analgesia to patients in severe pain. If not allergic, give morphine (preferable); if allergic to morphine, use fentanyl
- Consider compartment syndrome where pain is severe and unrelieved by splinting and elevation or two doses of analgesia
- X-ray if available

SAMPLE

Text C

Drug Therapy Protocol:
Authorised Indigenous Health Worker (IHW) must consult Medical Officer (MO) or Nurse Practitioner (NP).
Scheduled Medicines Rural & Isolated Practice Registered Nurse may proceed.

Drug	Form	Strength	Route of administration	Recommended dosage	Duration
Morphine	Ampoule	10 mg/mL	IM/SC	**Adult only:** 0.1-0.2 mg/kg to a max. of 10 mg	Stat
			IV (IHW may not administer IV)	**Adult only:** Initial dose of 2 mg then 0.5-1 mg increments slowly, repeated every 3-5 minutes if required to a max. of 10 mg	Further doses on MO/NP order

Use the lower end of dose range in patients ≥70 years.
Provide Consumer Medicine Information: advise can cause nausea and vomiting, drowsiness.
Respiratory depression is rare – if it should occur, give naloxone.

Text D

Technique for plaster backslab for arm fractures – use same principle for leg fractures

1. Measure a length of non-compression cotton stockinette from half way up the middle finger to just below the elbow. Width should be 2–3 cm more than the width of the distal forearm.
2. Wrap cotton padding over top for the full length of the stockinette — 2 layers, 50% overlap.
3. Measure a length of plaster of Paris 1 cm shorter than the padding/stockinette at each end. Fold the roll in about ten layers to the same length.
4. Immerse the layered plaster in a bowl of room temperature water, holding on to each end. Gently squeeze out the excess water.
5. Ensure any jewellery is removed from the injured limb.
6. Lightly mould the slab to the contours of the arm and hand in a neutral position.
7. Do not apply pressure over bony prominences. Extra padding can be placed over bony prominences if applicable.
8. Wrap crepe bandage firmly around plaster backslab.

END OF PART A
THIS TEXT BOOKLET WILL BE COLLECTED

SAMPLE

OCCUPATIONAL ENGLISH TEST

Sample Test 1

READING SUB-TEST – QUESTION PAPER: PART A

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

OTHER NAMES:

PROFESSION:

VENUE:

TEST DATE:

Your details and photo will be printed here.

Passport Photo

CANDIDATE SIGNATURE: _____

TIME: **15 MINUTES**

INSTRUCTIONS TO CANDIDATES:

DO NOT open this **Question Paper** or the **Text Booklet** until you are told to do so.

Write your answers on the spaces provided on this **Question Paper.**

You must answer the questions within the 15-minute time limit.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the 15 minutes, hand in this **Question Paper** and the **Text Booklet.**

DO NOT remove OET material from the test room.

SAMPLE

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Part A

TIME: 15 minutes

• Look at the four texts, **A-D**, in the separate **Text Booklet.**

• For each question, **1-20**, look through the texts, **A-D**, to find the relevant information.

• Write your answers on the spaces provided in this **Question Paper**.

• Answer all the questions within the 15-minute time limit.

• Your answers should be correctly spelt.

Fractures, dislocations and sprains: Questions

Questions 1-7

For each question, **1-7**, decide which text (**A**, **B**, **C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about

1 procedures for delivering pain relief? _____

2 the procedure to follow when splinting a fractured limb? _____

3 what to record when assessing a patient? _____

4 the terms used to describe different types of fractures? _____

5 the practitioners who administer analgesia? _____

6 what to look for when checking an injury? _____

7 how fractures can be caused? _____

Questions 8-14

Answer each of the questions, **8-14**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

8 What should be used to elevate a patient's fractured leg?

_____

9 What is the maximum dose of morphine per kilo of a patient's weight that can be given using the intra-muscular (IM) route?

_____

10 Which parts of a limb may need extra padding?

_____

SAMPLE

11 What should be used to treat a patient who suffers respiratory depression?

_____

12 What should be used to cover a freshly applied plaster backslab?

_____

13 What analgesic should be given to a patient who is allergic to morphine?

_____

14 What condition might a patient have if severe pain persists after splinting, elevation and repeated analgesia?

_____

Questions 15-20

Complete each of the sentences, **15**-**20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

15 Falling on an outstretched hand is a typical cause of a _____ of the elbow.

16 Upper limb fractures should be elevated by means of a _____ .

17 Make sure the patient isn't wearing any _____ on the part of the body where the plaster backslab is going to be placed.

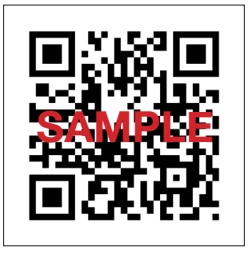
18 Check to see whether swollen limbs are _____ or increasing in size.

19 In a plaster backslab, there is a layer of _____ closest to the skin.

20 Patients aged _____ and over shouldn't be given the higher dosages of pain relief.

END OF PART A
THIS QUESTION PAPER WILL BE COLLECTED

SAMPLE



READING SUB-TEST – ANSWER KEY

PART A: QUESTIONS 1-20

1 C

2 D

3 B

4 A

5 C

6 B

7 A

8 (a) pillow / pillows

9 0.2 mg (/kg)

10 bony prominences

11 naloxone

12 crêpe/crepe bandage

13 fentanyl

14 compartment syndrome

15 dislocation

16 sling

17 jewellery

18 throbbing

19 (cotton / non-compression) stockinette

20 70 / seventy (years / yrs)

SAMPLE

OCCUPATIONAL ENGLISH TEST

READING SUB-TEST – QUESTION PAPER: PARTS B & C

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

OTHER NAMES:

Your details and photo will be printed here.

PROFESSION:

VENUE:

TEST DATE:

CANDIDATE SIGNATURE: _____

Passport Photo

SAMPLE

TIME: 45 MINUTES

INSTRUCTIONS TO CANDIDATES:

DO NOT open this **Question Paper** until you are told to do so.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the test, hand in this **Question Paper**.

DO NOT remove OET material from the test room.

HOW TO ANSWER THE QUESTIONS:

Mark your answers on this **Question Paper** by filling in the circle using a 2B pencil.

Example:

(A)

(B)

(C)

SAMPLE

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Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For **questions 1-6**, choose the answer (**A**, **B** or **C**) which you think fits best according to the text.

---

1. The manual informs us that the Blood Pressure Monitor

(A) is likely to interfere with the operation of other medical equipment.

(B) may not work correctly in close proximity to some other devices.

(C) should be considered safe to use in all hospital environments.

Instruction Manual: Digital Automatic Blood Pressure Monitor

Electromagnetic Compatibility (EMC)

With the increased use of portable electronic devices, medical equipment may be susceptible to electromagnetic interference. This may result in incorrect operation of the medical device and create a potentially unsafe situation. In order to regulate the requirements for EMC, with the aim of preventing unsafe product situations, the EN60601-1-2 standard defines the levels of immunity to electromagnetic interferences as well as maximum levels of electromagnetic emissions for medical devices. This medical device conforms to EN60601-1-2:2001 for both immunity and emissions. Nevertheless, care should be taken to avoid the use of the monitor within 7 metres of cellphones or other devices generating strong electrical or electromagnetic fields.

SAMPLE

2. The notice is giving information about

(A) ways of checking that an NG tube has been placed correctly.

(B) how the use of NG feeding tubes is authorised.

(C) which staff should perform NG tube placement.

NG feeding tubes

Displacement of nasogastric (NG) feeding tubes can have serious implications if undetected. Incorrectly positioned tubes leave patients vulnerable to the risks of regurgitation and respiratory aspiration. It is crucial to differentiate between gastric and respiratory placement on initial insertion to prevent potentially fatal pulmonary complications. Insertion and care of an NG tube should therefore only be carried out by a registered doctor or nurse who has undergone theoretical and practical training and is deemed competent or is supervised by someone competent. Assistant practitioners and other unregistered staff must never insert NG tubes or be involved in the initial confirmation of safe NG tube position.

SAMPLE

3. What must all staff involved in the transfusion process do?

(A) check that their existing training is still valid

(B) attend a course to learn about new procedures

(C) read a document that explains changes in policy

'Right Patient, Right Blood' Assessments

The administration of blood can have significant morbidity and mortality. Following the introduction of the 'Right Patient, Right Blood' safety policy, all staff involved in the transfusion process must be competency assessed. To ensure the safe administration of blood components to the intended patient, all staff must be aware of their responsibilities in line with professional standards.

Staff must ensure that if they take any part in the transfusion process, their competency assessment is updated every three years. All staff are responsible for ensuring that they attend the mandatory training identified for their roles. Relevant training courses are clearly identified in Appendix 1 of the Mandatory Training Matrix.

SAMPLE

SAMPLE

4. The guidelines establish that the healthcare professional should

Ⓐ aim to make patients fully aware of their right to a chaperone.

Ⓑ evaluate the need for a chaperone on a case-by-case basis.

Ⓒ respect the wishes of the patient above all else.

Extract from 'Chaperones: Guidelines for Good Practice'
A patient may specifically request a chaperone or in certain circumstances may nominate one, but it will not always be the case that a chaperone is required. It is often a question of using professional judgement to assess an individual situation. If a chaperone is offered and declined, this must be clearly documented in the patient's record, along with any relevant discussion. The chaperone should only be present for the physical examination and should be in a position to see what the healthcare professional undertaking the examination/investigation is doing. The healthcare professional should wait until the chaperone has left the room/cubicle before discussion takes place on any aspect of the patient's care, unless the patient specifically requests the chaperone to remain.

SAMPLE

5. The guidelines require those undertaking a clinical medication review to

(A) involve the patient in their decisions.

(B) consider the cost of any change in treatments.

(C) recommend other services as an alternative to medication.

Annual medication review

To give all patients an annual medication review is an ideal to strive for. In the meantime there is an argument for targeting all clinical medication reviews to those patients likely to benefit most.

Our guidelines state that 'at least a level 2 medication review will occur', i.e. the minimum standard is a treatment review of medicines with the full notes but not necessarily with the patient present. However, the guidelines go on to say that 'all patients should have the chance to raise questions and highlight problems about their medicines' and that 'any changes resulting from the review are agreed with the patient'.

It also states that GP practices are expected to

- minimise waste in prescribing and avoid ineffective treatments.
- engage effectively in the prevention of ill health.
- avoid the need for costly treatments by proactively managing patients to recovery through the whole care pathway.

SAMPLE

SAMPLE

6. The purpose of this email is to

(A) report on a rise in post-surgical complications.

(B) explain the background to a change in patient care.

(C) remind staff about procedures for administrating drugs.

To: All staff
Subject: Advisory Email: Safe use of opioids
In August, an alert was issued on the safe use of opioids in hospitals. This reported the incidence of respiratory depression among post-surgical patients to an average 0.5% – thus for every 5,000 surgical patients, 25 will experience respiratory depression. Failure to recognise respiratory depression and institute timely intervention can lead to cardiopulmonary arrest, resulting in brain injury or death. A retrospective multi-centre study of 14,720 cardiopulmonary arrest cases showed that 44% were respiratory related and more than 35% occurred on the general care floor. It is therefore recommended that post-operative patients now have continuous monitoring, instead of spot checks, of both oxygenation and ventilation.

SAMPLE

SAMPLE

Part C

In this part of the test, there are two texts about different aspects of healthcare. For **questions 7-22**, choose the answer (**A**, **B**, **C** or **D**) which you think fits best according to the text.

Text 1: Sleep deprivation

Millions of people who suffer sleep problems also suffer myriad health burdens. In addition to emotional distress and cognitive impairments, these can include high blood pressure, obesity, and metabolic syndrome. 'In the studies we've done, almost every variable we measured was affected. There's not a system in the body that's not affected by sleep,' says University of Chicago sleep researcher Eve Van Cauter. 'Every time we sleep-deprive ourselves, things go wrong.'

A common refrain among sleep scientists about two decades ago was that sleep was performed by the brain in the interest of the brain. That wasn't a fully elaborated theory, but it wasn't wrong. Numerous recent studies have hinted at the purpose of sleep by confirming that neurological function and cognition are messed up during sleep loss, with the patient's reaction time, mood, and judgement all suffering if they are kept awake too long.

In 1997, Bob McCarley and colleagues at Harvard Medical School found that when they kept cats awake by playing with them, a compound known as adenosine increased in the basal forebrain as the sleepy felines stayed up longer, and slowly returned to normal levels when they were later allowed to sleep. McCarley's team also found that administering adenosine to the basal forebrain acted as a sedative, putting animals to sleep. It should come as no surprise then that caffeine, which blocks adenosine's receptor, keeps us awake. Teaming up with Basheer and others, McCarley later discovered that, as adenosine levels rise during sleep deprivation, so do concentrations of adenosine receptors, magnifying the molecule's sleep-inducing effect. 'The brain has cleverly designed a two-stage defence against the consequences of sleep loss,' McCarley says. Adenosine may underlie some of the cognitive deficits that result from sleep loss. McCarley and colleagues found that infusing adenosine into rats' basal forebrain impaired their performance on an attention test, similar to that seen in sleep-deprived humans. But adenosine levels are **by no means the be-all and end-all** of sleep deprivation's effects on the brain or the body.

Over a century of sleep research has revealed numerous undesirable outcomes from staying awake too long. In 1999, Van Cauter and colleagues had eleven men sleep in the university lab. For three nights, they spent eight hours in bed, then for six nights they were allowed only four hours (accruing what Van Cauter calls a sleep debt), and then for six nights they could sleep for up to twelve hours (sleep recovery). During sleep debt and recovery, researchers gave the participants a glucose tolerance test and found striking differences. While sleep deprived, the men's glucose metabolism resembled a pre-diabetic state. 'We knew it would be affected,' says Van Cauter. 'The big surprise was the effect being much greater than we thought.'

SAMPLE

Subsequent studies also found insulin resistance increased during bouts of sleep restriction, and in 2012, Van Cauter's team observed impairments in insulin signalling in subjects' fat cells. Another recent study showed that sleep-restricted people will add 300 calories to their daily diet. Echoing Van Cauter's results, Basheer has found evidence that enforced lack of sleep sends the brain into a catabolic, or energy-consuming, state. This is because **it** degrades the energy molecule adenosine triphosphate (ATP) to produce adenosine monophosphate and this results in the activation of AMP kinase, an enzyme that boosts fatty acid synthesis and glucose utilization. 'The system sends a message that there's a need for more energy,' Basheer says. Whether this is indeed the mechanism underlying late-night binge-eating is still speculative.

Within the brain, scientists have glimpsed signs of physical damage from sleep loss, and the time-line for recovery, if any occurs, is unknown. Chiara Cirelli's team at the Madison School of Medicine in the USA found structural changes in the cortical neurons of mice when the animals are kept awake for long periods. Specifically, Cirelli and colleagues saw signs of mitochondrial activation – which makes sense, as 'neurons need more energy to stay awake,' she says – as well as unexpected changes, such as undigested cellular debris, signs of cellular aging that are unusual in the neurons of young, healthy mice. 'The number [of debris granules] was small, but it's worrisome because it's only four to five days' of sleep deprivation,' says Cirelli. After thirty-six hours of sleep recovery, a period during which she expected normalcy to resume, those changes remained.

Further insights could be drawn from the study of shift workers and insomniacs, who serve as natural experiments on how the human body reacts to losing out on such a basic life need for chronic periods. But with so much of our physiology affected, an effective therapy − other than sleep itself – is hard to imagine. 'People like to define a clear pathway of action for health conditions,' says Van Cauter. 'With sleep deprivation, everything you measure is affected and interacts synergistically to produce the effect.'

SAMPLE

Text 1: Questions 7-14

7. In the first paragraph, the writer uses Eve Van Cauter's words to

(A) explain the main causes of sleep deprivation.

(B) reinforce a view about the impact of sleep deprivation.

(C) question some research findings about sleep deprivation.

(D) describe the challenges involved in sleep deprivation research.

8. What do we learn about sleep in the second paragraph?

(A) Scientific opinion about its function has changed in recent years.

(B) There is now more controversy about it than there was in the past.

(C) Researchers have tended to confirm earlier ideas about its purpose.

(D) Studies undertaken in the past have formed the basis of current research.

9. What particularly impressed Bob McCarley of Harvard Medical School?

(A) the effectiveness of adenosine as a sedative

(B) the influence of caffeine on adenosine receptors

(C) the simultaneous production of adenosine and adenosine receptors

(D) the extent to which adenosine levels fall when subjects are allowed to sleep

10. In the third paragraph, what idea is emphasised by the phrase '**by no means the be-all and end-all**'?

(A) Sleep deprivation has consequences beyond its impact on adenosine levels.

(B) Adenosine levels are a significant factor in situations other than sleep deprivation.

(C) The role of adenosine as a response to sleep deprivation is not yet fully understood.

(D) The importance of the link between sleep deprivation and adenosine should not be underestimated.

SAMPLE

11. What was significant about the findings in Van Cauter's experiment?

(A) the rate at which the sleep-deprived men entered a pre-diabetic state

(B) the fact that sleep deprivation had an influence on the men's glucose levels

(C) the differences between individual men with regard to their glucose tolerance

(D) the extent of the contrast in the men's metabolic states between sleep debt and recovery

12. In the fifth paragraph, what does the word '**it**' refer to?

(A) an enzyme

(B) new evidence

(C) a catabolic state

(D) enforced lack of sleep

13. What aspect of her findings surprised Chiara Cirelli?

(A) There was no reversal of a certain effect of sleep deprivation.

(B) The cortical neurons of the mice underwent structural changes.

(C) There was evidence of an increased need for energy in the brains of the mice.

(D) The neurological response to sleep deprivation only took a few hours to become apparent.

14. In the final paragraph, the quote from Van Cauter is used to suggest that

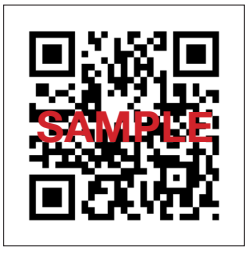
(A) the goals of sleep deprivation research are sometimes unclear.

(B) it could be difficult to develop any treatment for sleep deprivation.

(C) opinions about the best way to deal with sleep deprivation are divided.

(D) there is still a great deal to be learnt about the effects of sleep deprivation.

SAMPLE



Text 2: ADHD

The American Psychiatric Association (APA) recognised Attention Deficit Hyperactivity Disorder (ADHD) as a childhood disorder in the 1960s, but it wasn't until 1978 that the condition was formally recognised as afflicting adults. In recent years, the USA has seen a 40% rise in diagnoses of ADHD in children. It could be that the disorder is becoming more prevalent, or, as seems more plausible, doctors are making the diagnosis more frequently. The issue is complicated by the lack of any recognised neurological markers for ADHD. The APA relies instead on a set of behavioural patterns for diagnosis. It specifies that patients under 17 must display at least six symptoms of inattention and/or hyperactivity; adults need only display five.

ADHD can be a controversial condition. Dr Russell Barkley, Professor of Psychiatry at the University of Massachusetts insists; 'the science is overwhelming: it's a real disorder, which can be managed, in many cases, by using stimulant medication in combination with other treatments'. Dr Richard Saul, a behavioural neurologist with five decades of experience, disagrees; 'Many of us have difficulty with organization or details, a tendency to lose things, or to be forgetful or distracted. Under such subjective criteria, the entire population could potentially qualify. Although some patients might need stimulants to function well in daily life, the lumping together of many vague and subjective symptoms could be causing a national phenomenon of misdiagnosis and over-prescription of stimulants.'

A recent study found children in foster care three times more likely than others to be diagnosed with ADHD. Researchers also found that children with ADHD in foster care were more likely to have another disorder, such as depression or anxiety. This finding certainly reveals the need for medical and behavioural services for these children, but it could also prove the non-specific nature of the symptoms of ADHD: anxiety and depression, or an altered state, can easily be mistaken for manifestations of ADHD.

ADHD, the thinking goes, begins in childhood. In fact, in order to be diagnosed with it as an adult, a patient must demonstrate that they had traits of the condition in childhood. However, studies from the UK and Brazil, published in JAMA Psychiatry, are fuelling questions about the origins and trajectory of ADHD, suggesting not only that it can begin in adulthood, but that there may be two distinct syndromes: adult-onset ADHD and childhood ADHD. **They** echo earlier research from New Zealand. However, an editorial by Dr Stephen Faraone in JAMA Psychiatry highlights potential flaws in the findings. Among them, underestimating the persistence of ADHD into adulthood and overestimating the prevalence of adult-onset ADHD. In Dr Faraone's words, 'the researchers found a group of people who had sub-threshold ADHD in their youth. There may have been signs that things weren't right, but not enough to go to a doctor. Perhaps these were smart kids with particularly supportive parents or teachers who helped them cope with attention problems. Such intellectual and social scaffolding would help in early life, but when the scaffolding is removed, full ADHD could develop'.

SAMPLE

Until this century, adult ADHD was a seldom-diagnosed disorder. Nowadays however, it's common in mainstream medicine in the USA, a paradigm shift apparently driven by two factors: reworked – many say less stringent – diagnostic criteria, introduced by the APA in 2013, and marketing by manufacturers of ADHD medications. Some have suggested that this new, broader definition of ADHD was fuelled, at least in part, to broaden the market for medication. In many instances, the evidence proffered to expand the definitions came from studies funded in whole or part by manufacturers. And as the criteria for the condition loosened, reports emerged about clinicians involved in diagnosing ADHD receiving money from drug-makers.

This brings us to the issue of the addictive nature of ADHD medication. As Dr Saul asserts, 'addiction to stimulant medication isn't rare; it's common. Just observe the many patients periodically seeking an increased dosage as their powers of concentration diminish. This is because the body stops producing the appropriate levels of neurotransmitters that ADHD drugs replace − **a trademark** of addictive substances.' Much has been written about the staggering increase in opioid overdoses and abuse of prescription painkillers in the USA, but the abuse of drugs used to treat ADHD is no less a threat. While opioids are more lethal than prescription stimulants, there are parallels between the opioid epidemic and the increase in problems tied to stimulants. In the former, users switch from prescription narcotics to heroin and illicit fentanyl. With ADHD drugs, patients are switching from legally prescribed stimulants to illicit ones such as methamphetamine and cocaine. The medication is particularly prone to abuse because people feel it improves their lives. These drugs are antidepressants, aid weight-loss and improve confidence, and can be abused by students seeking to improve their focus or academic performance. So, more work needs to be done before we can settle the questions surrounding the diagnosis and treatment of ADHD.

SAMPLE

Text 2: Questions 15-22

15. In the first paragraph, the writer questions whether

(A) adult ADHD should have been recognised as a disorder at an earlier date.

(B) ADHD should be diagnosed in the same way for children and adults.

(C) ADHD can actually be indicated by neurological markers.

(D) cases of ADHD have genuinely increased in the USA.

16. What does Dr Saul object to?

(A) the suggestion that people need stimulants to cope with everyday life

(B) the implication that everyone has some symptoms of ADHD

(C) the grouping of imprecise symptoms into a mental disorder

(D) the treatment for ADHD suggested by Dr Barkley

17. The writer regards the study of children in foster care as significant because it

(A) highlights the difficulty of distinguishing ADHD from other conditions.

(B) focuses on children known to have complex mental disorders.

(C) suggests a link between ADHD and a child's upbringing.

(D) draws attention to the poor care given to such children.

18. In the fourth paragraph, the word '**They**' refers to

(A) syndromes.

(B) questions.

(C) studies.

(D) origins.

SAMPLE

19. Dr Faraone suggests that the group of patients diagnosed with adult-onset ADHD

(A) had teachers or parents who recognised the symptoms of ADHD.

(B) should have consulted a doctor at a younger age.

(C) had mild undiagnosed ADHD in childhood.

(D) were specially chosen by the researchers.

20. In the fifth paragraph, it is suggested that drug companies have

(A) been overly aggressive in their marketing of ADHD medication.

(B) influenced research that led to the reworking of ADHD diagnostic criteria.

(C) attempted to change the rules about incentives for doctors who diagnose ADHD.

(D) encouraged the APA to rush through changes to the criteria for diagnosing ADHD.

21. In the final paragraph, the word '**trademark**' refers to

(A) a physiological reaction.

(B) a substitute medication.

(C) a need for research.

(D) a common request.

22. In the final paragraph, what does the writer imply about addiction to ADHD medication?

(A) It is unlikely to turn into a problem on the scale of that caused by opioid abuse.

(B) The effects are more marked in certain sectors of the population.

(C) Insufficient attention seems to have been paid to it.

(D) The reasons for it are not yet fully understood.

END OF READING TEST
THIS BOOKLET WILL BE COLLECTED

SAMPLE

READING SUB-TEST - ANSWER KEY

PART B: QUESTIONS 1-6

1	B	may not work correctly in close proximity to some other devices.
2	C	which staff should perform NG tube placement.
3	A	check that their existing training is still valid
4	B	evaluate the need for a chaperone on a case-by-case basis.
5	A	involve the patient in their decisions.
6	B	explain the background to a change in patient care.

PART C: QUESTIONS 7-14

7	B	reinforce a view about the impact of sleep deprivation.
8	C	Researchers have tended to confirm earlier ideas about its purpose.
9	C	the simultaneous production of adenosine and adenosine receptors
10	A	Sleep deprivation has consequences beyond its impact on adenosine levels.
11	D	the extent of the contrast in the men's metabolic states between sleep debt and recovery
12	D	enforced lack of sleep
13	A	There was no reversal of a certain effect of sleep deprivation.
14	B	it could be difficult to develop any treatment for sleep deprivation.

PART C: QUESTIONS 15-22

15	D	cases of ADHD have genuinely increased in the USA.
16	C	the grouping of imprecise symptoms into a mental disorder
17	A	highlights the difficulty of distinguishing ADHD from other conditions.
18	C	studies.
19	C	had mild undiagnosed ADHD in childhood.
20	B	influenced research that led to the reworking of ADHD diagnostic criteria.
21	A	a physiological reaction.
22	C	Insufficient attention seems to have been paid to it.

SAMPLE

OCCUPATIONAL ENGLISH TEST

Test 5

READING SUB-TEST – TEXT BOOKLET: PART A

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

OTHER NAMES:

Your details and photo will be printed here.

PROFESSION:

VENUE:

TEST DATE:

CANDIDATE SIGNATURE: _____

Passport Photo

SAMPLE

SAMPLE

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Text A

Paracetamol: contraindications and interactions

4.4 Special warnings and precautions for use

Where analgesics are used long-term (>3 months) with administration every two days or more frequently, headache may develop or increase. Headache induced by overuse of analgesics (MOH medication-overuse headache) should not be treated by dose increase. In such cases, the use of analgesics should be discontinued in consultation with the doctor.

Care is advised in the administration of paracetamol to patients with alcohol dependency, severe renal or severe hepatic impairment. Other contraindications are: shock and acute inflammation of liver due to hepatitis C virus. The hazards of overdose are greater in those with non-cirrhotic alcoholic liver disease.

4.5 Interaction with other medicinal products and other forms of interaction

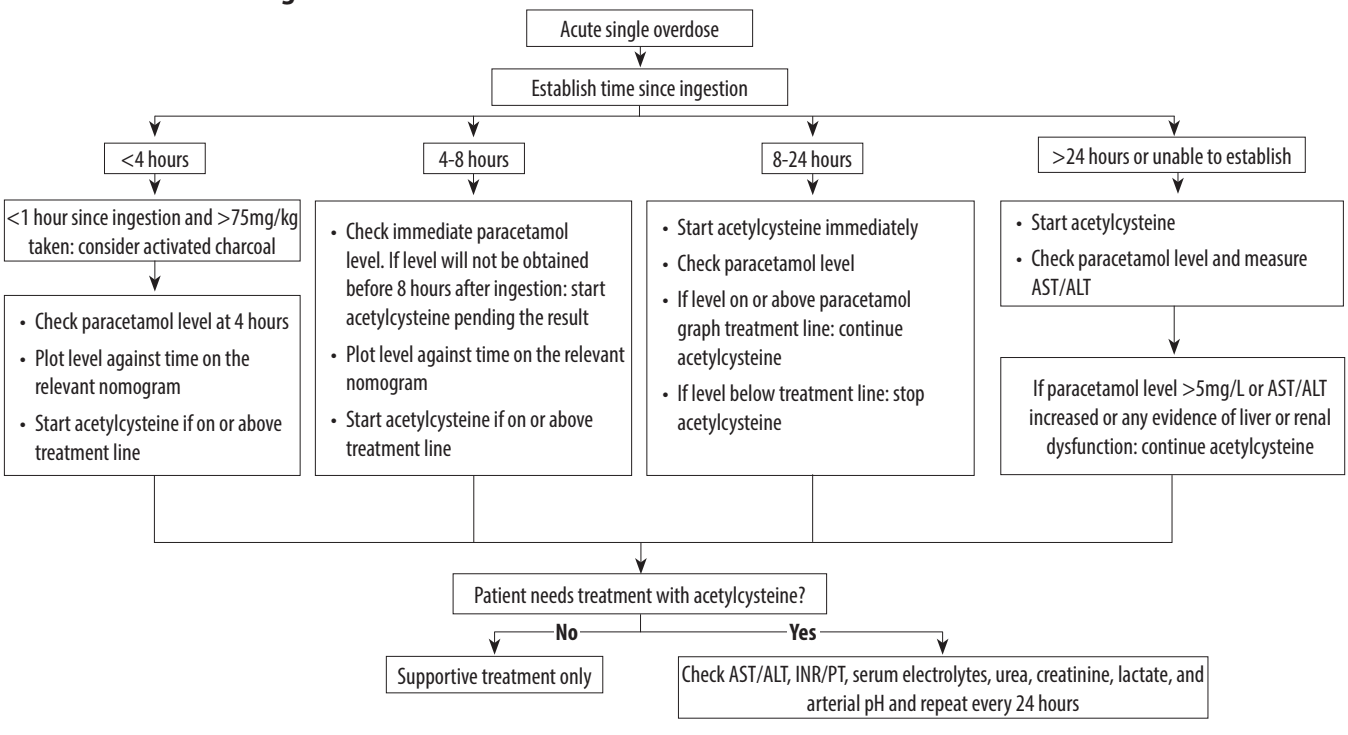
- Anticoagulants – the effect of warfarin and other coumarins may be enhanced by prolonged regular use of paracetamol with increased risk of bleeding. Occasional doses have no significant effect.
- Metoclopramide – may increase speed of absorption of paracetamol.
- Domperidone – may increase speed of absorption of paracetamol.
- Colestyramine – may reduce absorption if given within one hour of paracetamol.
- Imatinib – restriction or avoidance of concomitant regular paracetamol use should be taken with imatinib.

A total of 169 drugs (1042 brand and generic names) are known to interact with paracetamol.
14 major drug interactions (e.g. amyl nitrite)
62 moderate drug interactions
93 minor drug interactions

A total of 118 brand names are known to have paracetamol in their formulation, e.g. Lemsip.

Text B

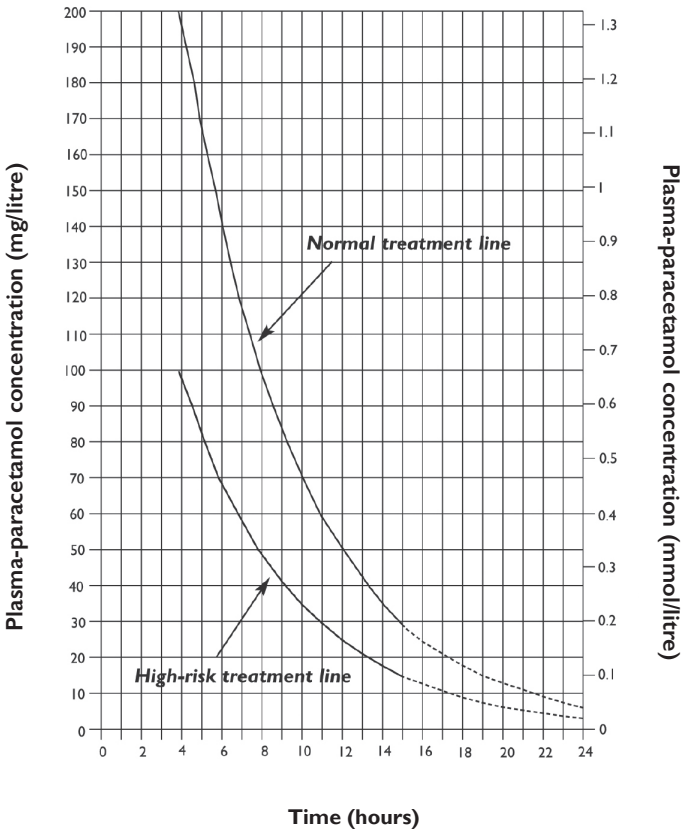
Procedure for acute single overdose



SAMPLE

Text C

Paracetamol poisoning – Emergency treatment of poisoning



Patients whose plasma-paracetamol concentrations are above the **normal treatment line** should be treated with acetylcysteine by intravenous infusion (or, if acetylcysteine cannot be used, with methionine by mouth, provided the overdose has been taken **within 10-12 hours** and the patient is not vomiting).

Patients on enzyme-inducing drugs (e.g. carbamazepine, phenobarbital, phenytoin, primidone, rifampicin and St John's wort) or who are malnourished (e.g. in anorexia, in alcoholism, or those who are HIV positive) should be treated with acetylcysteine if their plasma-paracetamol concentration is above the **high-risk treatment line.**

Text D

Clinical Assessment

- Commonly, patients who have taken a paracetamol overdose are asymptomatic for the first 24 hours or just have nausea and vomiting
- Hepatic necrosis (elevated transaminases, right upper quadrant pain and jaundice) begins to develop after 24 hours and can progress to acute liver failure (ALF)
- Patients may also develop:
  - Encephalopathy
  - Oliguria
  - Hypoglycaemia
  - Renal failure – usually occurs around day three
  - Lactic acidosis

History

- Number of tablets, formulation, any concomitant tablets
- Time of overdose
- Suicide risk – was a note left?
- Any alcohol taken (acute alcohol ingestion will inhibit liver enzymes and may reduce the production of the toxin NAPQI, whereas chronic alcoholism may increase it)

END OF PART A
THIS TEXT BOOKLET WILL BE COLLECTED

SAMPLE

OCCUPATIONAL ENGLISH TEST

READING SUB-TEST – QUESTION PAPER: PART A

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

OTHER NAMES:

PROFESSION:

VENUE:

TEST DATE:

Your details and photo will be printed here.

Passport Photo

CANDIDATE SIGNATURE: _____

TIME: 15 MINUTES

INSTRUCTIONS TO CANDIDATES:

DO NOT open this **Question Paper** or the **Text Booklet** until you are told to do so.

Write your answers on the spaces provided on this **Question Paper.**

You must answer the questions within the 15-minute time limit.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the 15 minutes, hand in this **Question Paper** and the **Text Booklet.**

DO NOT remove OET material from the test room.

SAMPLE

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Part A

TIME: 15 minutes

• Look at the four texts, **A-D**, in the separate **Text Booklet.**

• For each question, **1**-**20**, look through the texts, **A**-**D**, to find the relevant information.

• Write your answers on the spaces provided in this **Question Paper**.

• Answer all the questions within the 15-minute time limit.

• Your answers should be correctly spelt.

Paracetamol overdose: Questions

Questions 1-7

For each question, **1-7**, decide which text (**A**, **B**, **C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about

1 the various symptoms of patients who have taken too much paracetamol? _____

2 the precise levels of paracetamol in the blood which require urgent intervention? _____

3 the steps to be taken when treating a paracetamol overdose patient? _____

4 whether paracetamol overdose was intentional? _____

5 the number of products containing paracetamol? _____

6 what to do if there are no details available about the time of the overdose? _____

7 dealing with paracetamol overdose patients who have not received adequate nutrition? _____

Questions 8-13

Answer each of the questions, **8**-**13**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

8 If paracetamol is used as a long-term painkiller, what symptom may get worse?

_____

9 It may be dangerous to administer paracetamol to a patient with which viral condition?

_____

10 What condition may develop in an overdose patient who presents with jaundice?

_____

SAMPLE

11 What condition may develop on the third day after an overdose?

_____

12 What drug can be administered orally within 10 - 12 hours as an alternative to acetylcysteine?

_____

13 What treatment can be used if a single overdose has occurred less than an hour ago?

_____

Questions 14-20

Complete each of the sentences, **14**-**20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

14 If a patient has taken metoclopramide alongside paracetamol, this may affect the _____ of the paracetamol.

15 After 24 hours, an overdose patient may present with pain in the _____.

16 For the first 24 hours after overdosing, patients may only have such symptoms as _____.

17 Acetylcysteine should be administered to patients with a paracetamol level above the high-risk treatment line who are taking any type of _____ medication.

18 A non-high-risk patient should be treated for paracetamol poisoning if their paracetamol level is above _____ mg/litre 8 hours after overdosing.

19 A high-risk patient who overdosed _____ hours ago should be given acetylcysteine if their paracetamol level is 25 mg/litre or higher.

20 If a patient does not require further acetylcysteine, they should be given treatment categorised as _____ only.

END OF PART A
THIS QUESTION PAPER WILL BE COLLECTED

SAMPLE

PART A: QUESTIONS: 1 – 20

1 D

2 C

3 B

4 D

5 A

6 B

7 C

8 headache(s)

9 hepatitis C **OR** hep C

10 ALF **OR** acute liver failure

11 renal failure (**NOT**: renal dysfunction)

12 methionine

13 (activated) charcoal

14 speed of absorption

15 right upper quadrant

16 nausea **OR** vomiting **OR** nausea and vomiting **OR** vomiting and nausea

17 enzyme-inducing

18 100 **OR** a hundred **OR** one hundred

19 12 **OR** twelve

20 supportive (treatment)

SAMPLE

OCCUPATIONAL ENGLISH TEST

Sample Test 2

READING SUB-TEST – QUESTION PAPER: PARTS B & C

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

OTHER NAMES:

PROFESSION:

VENUE:

TEST DATE:

Your details and photo will be printed here.

Passport Photo

CANDIDATE SIGNATURE: _____

TIME: 45 MINUTES

INSTRUCTIONS TO CANDIDATES:

DO NOT open this **Question Paper** until you are told to do so.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the test, hand in this **Question Paper**.

DO NOT remove OET material from the test room.

HOW TO ANSWER THE QUESTIONS:

Mark your answers on this **Question Paper** by filling in the circle using a 2B pencil.

Example:

(A)

(B)

(C)

SAMPLE

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Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For **questions 1-6**, choose the answer (**A**, **B** or **C**) which you think fits best according to the text.

---

1. This guideline extract says that the nurse in charge

(A) must supervise the opening of the controlled drug cupboard.

(B) should make sure that all ward cupboard keys are kept together.

(C) can delegate responsibility for the cupboard keys to another ward.

Medicine Cupboard Keys

The keys for the controlled drug cupboard are the responsibility of the nurse in charge. They may be passed to a registered nurse in order for them to carry out their duties and returned to the nurse in charge. If the keys for the controlled drug cupboard go missing, the locks must be changed and pharmacy informed and an incident form completed. The controlled drug cupboard keys should be kept separately from the main body of keys. Apart from in exceptional circumstances, the keys should not leave the ward or department. If necessary, the nurse in charge should arrange for the keys to be held in a neighbouring ward or department by the nurse in charge there.

SAMPLE

2. When seeking consent for a post-mortem examination, it is necessary to

(A) give a valid reason for conducting it.

(B) allow all relatives the opportunity to decline it.

(C) only raise the subject after death has occurred.

Post-Mortem Consent

A senior member of the clinical team, preferably the Consultant in charge of the care, should raise the possibility of a post-mortem examination with the most appropriate person to give consent. The person consenting will need an explanation of the reasons for the post-mortem examination and what it hopes to achieve. The first approach should be made as soon as it is apparent that a post-mortem examination may be desirable, as there is no need to wait until the patient has died. Many relatives are more prepared for the consenting procedure if they have had time to think about it beforehand.

SAMPLE

3. The purpose of these notes about an incinerator is to

(A) help maximise its efficiency.

(B) give guidance on certain safety procedures.

(C) recommend a procedure for waste separation.

Low-cost incinerator: General operating notes

3.2.1 Hospital waste management

Materials with high fuel values such as plastics, paper, card and dry textile will help maintain high incineration temperature. If possible, a good mix of waste materials should be added with each batch. This can best be achieved by having the various types of waste material loaded into separate bags at source, i.e. wards and laboratories, and clearly labelled. It is not recommended that the operator sorts and mixes waste prior to incineration as this is potentially hazardous. If possible, some plastic materials should be added with each batch of waste as this burns at high temperatures. However, care and judgement will be needed, as too much plastic will create dense dark smoke.

SAMPLE

4. What does this manual tell us about spacer devices?

(A) Patients should try out a number of devices with their inhaler.

(B) They enable a patient to receive more of the prescribed medicine.

(C) Children should be given spacers which are smaller than those for adults.

Manual extract: Spacer devices for asthma patients

Spacer devices remove the need for co-ordination between actuation of a pressurized metered-dose inhaler and inhalation. In addition, the device allows more time for evaporation of the propellant so that a larger proportion of the particles can be inhaled and deposited in the lungs. Spacer devices are particularly useful for patients with poor inhalation technique, for children, for patients requiring higher doses, for nocturnal asthma, and for patients prone to candidiasis with inhaled corticosteroids. The size of the spacer is important, the larger spacers with a one-way valve being most effective. It is important to prescribe a spacer device that is compatible with the metered-dose inhaler. Spacer devices should not be regarded as interchangeable; patients should be advised not to switch between spacer devices.

SAMPLE

5. The email is reminding staff that the

(A) benefits to patients of using bedrails can outweigh the dangers.

(B) number of bedrail-related accidents has reached unacceptable levels.

(C) patient's condition should be central to any decision about the use of bedrails.

To: All Staff

Subject: Use of bed rails

Please note the following.

Patients in hospital may be at risk of falling from bed for many reasons including poor mobility, dementia or delirium, visual impairment, and the effects of treatment or medication. Bedrails can be used as safety devices intended to reduce risk.

However, bedrails aren't appropriate for all patients, and their use involves risks. National data suggests around 1,250 patients injure themselves on bedrails annually, usually scrapes and bruises to their lower legs. Statistics show 44,000 reports of patient falls from bed annually resulting in 11 deaths, while deaths due to bedrail entrapment occur less than one every two years, and are avoidable if the relevant advice is followed.

Staff should continue to take great care to avoid bedrail entrapment, but be aware that in hospital settings there may be a greater risk of harm to patients who fall out of bed.

SAMPLE

6. What does this extract from a handbook tell us about analeptic drugs?

(A) They may be useful for patients who are not fully responsive.

(B) Injections of these drugs will limit the need for physiotherapy.

(C) Care should be taken if they are used over an extended period.

Analeptic drugs

Respiratory stimulants (analeptic drugs) have a limited place in the treatment of ventilatory failure in patients with chronic obstructive pulmonary disease. They are effective only when given by intravenous injection or infusion and have a short duration of action. Their use has largely been replaced by ventilatory support. However, occasionally when ventilatory support is contra-indicated and in patients with hypercapnic respiratory failure who are becoming drowsy or comatose, respiratory stimulants in the short term may arouse patients sufficiently to co-operate and clear their secretions.

Respiratory stimulants can also be harmful in respiratory failure since they stimulate non-respiratory as well as respiratory muscles. They should only be given under expert supervision in hospital and must be combined with active physiotherapy. At present, there is no oral respiratory stimulant available for long-term use in chronic respiratory failure.

SAMPLE

Part C

In this part of the test, there are two texts about different aspects of healthcare. For **questions 7-22**, choose the answer (**A**, **B**, **C** or **D**) which you think fits best according to the text.

Text 1: Patient Safety

Highlighting a collaborative initiative to improve patient safety

In a well-documented case in November 2004, a female patient called Mary was admitted to a hospital in Seattle, USA, to receive treatment for a brain aneurysm. What followed was a tragedy, made worse by the fact that it needn't have occurred at all. The patient was mistakenly injected with the antiseptic chlorhexidine. It happened, the hospital says, because of 'confusion over the three identical stainless steel bowls in the procedure room containing clear liquids — chlorhexidine, contrast dye and saline solution'. Doctors tried amputating one of Mary's legs to save her life, but the damage to her organs was too great: she died 19 days later.

This and similar incidents are what inspired Professor Dixon-Woods of the University of Cambridge, UK, to set out on a mission: to improve patient safety. It is, she admits, going to be a challenge. Many different policies and approaches have been tried to date, but few with widespread success, and often with unintended consequences. Financial incentives are widely used, but recent evidence suggests that they have little effect. 'There's a danger that they tend to encourage **effort substitution**,' explains Dixon-Woods. In other words, people concentrate on the areas that are being incentivised, but neglect other areas. 'It's not even necessarily conscious neglect. People have only a limited amount of time, so it's inevitable they focus on areas that are measured and rewarded.'

In 2013, Dixon-Woods and colleagues published a study evaluating the use of surgical checklists introduced in hospitals to reduce complications and deaths during surgery. Her research found that that checklists may have little impact, and in some situations might even make things worse. 'The checklists sometimes introduced new risks. Nurses would use the lists as box-ticking exercises – they would tick the box to say the patient had had their antibiotics when there were no antibiotics in the hospital, for example.' They also reinforced the hierarchies – nurses had to try to get surgeons to do certain tasks, but the surgeons used the situation as an opportunity to display their power and refuse.

Dixon-Woods and her team spend time in hospitals to try to understand which systems are in place and how they are used. Not only does she find differences in approaches between hospitals, but also between units and even between shifts. 'Standardisation and harmonisation are two of the most urgent issues we have to tackle. Imagine if you have to learn each new system wherever you go or even whenever a new senior doctor is on the ward. This introduces massive risk.'

SAMPLE

Dixon-Woods compares the issue of patient safety to that of climate change, in the sense that it is a 'problem of many hands', with many actors, each making a contribution towards the outcome, and there is difficulty in identifying where the responsibility for solving the problem lies. 'Many patient safety issues arise at the level of the system as a whole, but policies treat patient safety as an issue for each individual organisation.'

Nowhere is this more apparent than the issue of 'alarm fatigue', according to Dixon-Woods. Each bed in an intensive care unit typically generates 160 alarms per day, caused by machinery that is not integrated. 'You have to assemble all the kit around an intensive care bed manually,' she explains. 'It doesn't come built as one like an aircraft cockpit. This is not something a hospital can solve alone. It needs to be solved at the sector level.'

Dixon-Woods has turned to Professor Clarkson in Cambridge's Engineering Design Centre to help. 'Fundamentally, my work is about asking how we can make it better and what could possibly go wrong,' explains Clarkson. 'We need to look through the eyes of the healthcare providers to see the challenges and to understand where tools and techniques we use in engineering may be of value.' There is a difficulty, he concedes: 'There's no formal language of design in healthcare. Do we understand what the need is? Do we understand what the requirements are? Can we think of a range of concepts we might use and then design a solution and test it before we put it in place? We seldom see this in healthcare, and that's partly driven by culture and lack of training, but partly by lack of time.' Dixon-Woods agrees that healthcare can learn much from engineers. 'There has to be a way of getting our two sides talking,' she says. 'Only then will we be able to prevent tragedies like the death of Mary.'

SAMPLE

Text 1: Questions 7-14

7. What point is made about the death of a female patient called Mary?

(A) It was entirely preventable.

(B) Nobody was willing to accept the blame.

(C) Surgeons should have tried harder to save her life.

(D) It is the type of incident which is becoming increasingly common.

8. What is meant by the phrase **'effort substitution'** in the second paragraph?

(A) Monetary resources are diverted unnecessarily.

(B) Time and energy is wasted on irrelevant matters.

(C) Staff focus their attention on a limited number of issues.

(D) People have to take on tasks which they are unfamiliar with.

9. By quoting Dixon-Woods in the second paragraph, the writer shows that the professor

(A) understands why healthcare employees have to make certain choices.

(B) doubts whether reward schemes are likely to put patients at risk.

(C) believes staff should be paid a bonus for achieving goals.

(D) feels the people in question have made poor choices.

10. What point is made about checklists in the third paragraph?

(A) Hospital staff sometimes forget to complete them.

(B) Nurses and surgeons are both reluctant to deal with them.

(C) They are an additional burden for over-worked nursing staff.

(D) The information recorded on them does not always reflect reality.

SAMPLE

11. What problem is mentioned in the fourth paragraph?

(A) failure to act promptly

(B) outdated procedures

(C) poor communication

(D) lack of consistency

12. What point about patient safety is the writer making by quoting Dixon-Woods' comparison with climate change?

(A) The problem will worsen if it isn't dealt with soon.

(B) It isn't clear who ought to be tackling the situation.

(C) It is hard to know what the best course of action is.

(D) Many people refuse to acknowledge there is a problem.

13. The writer quotes Dixon-Woods' reference to intensive care beds in order to

(A) present an alternative viewpoint.

(B) illustrate a fundamental obstacle.

(C) show the drawbacks of seemingly simple solutions.

(D) give a detailed example of how to deal with an issue.

14. What difference between healthcare and engineering is mentioned in the final paragraph?

(A) the types of systems they use

(B) the way they exploit technology

(C) the nature of the difficulties they face

(D) the approach they take to deal with challenges

SAMPLE

Text 2: Migraine – more than just a headache

When a news reporter in the US gave an unintelligible live TV commentary of an awards ceremony, she became an overnight internet sensation. As the paramedics attended, the worry was that she'd suffered a stroke live on air. Others wondered if she was drunk or on drugs. However, in interviews shortly after, she revealed, to general astonishment, that she'd simply been starting a migraine. The bizarre speech difficulties she experienced are an uncommon symptom of aura, the collective name for a range of neurological symptoms that may occur just before a migraine headache. Generally aura are visual – for example blind spots which increase in size, or have a flashing, zig-zagging or sparkling margin, but they can include other odd disturbances such as pins and needles, memory changes and even partial paralysis.

Migraine is often thought of as an occasional severe headache, but surely symptoms such as these should tell us there's more to it than meets the eye. In fact many scientists now consider it a serious neurological disorder. One area of research into migraine aura has looked at the phenomenon known as Cortical Spreading Depression (CSD) – a storm of neural activity that passes in a wave across the brain's surface. First seen in 1944 in the brain of a rabbit, it's now known that CSD can be triggered when the normal flow of electric currents within and around brain cells is somehow reversed. Nouchine Hadjikhani and her team at Harvard Medical School managed to record an episode of CSD in a brain scanner during migraine aura (in a visual region that responds to flickering motion), having found a patient who had the rare ability to be able to predict when an aura would occur. **This** confirmed a long-suspected link between CSD and the aura that often precedes migraine pain. Hadjikhani admits, however, that other work she has done suggests that CSD may occur all over the brain, often unnoticed, and may even happen in healthy brains. If so, aura may be the result of a person's brain being more sensitive to CSD than it should be.

Hadjikhani has also been looking at the structural and functional differences in the brains of migraine sufferers. She and her team found thickening of a region known as the somatosensory cortex, which maps our sense of touch in different parts of the body. They found the most significant changes in the region that relates to the head and face. 'Because sufferers return to normal following an attack, migraine has always been considered an episodic problem,' says Hadjikhani. 'But we found that if you have successive strikes of pain in the face area, it actually increases cortical thickness.'

Work with children is also providing some startling insights. A study by migraine expert Peter Goadsby, who splits his time between King's College London and the University of California, San Francisco, looked at the prevalence of migraine in mothers of babies with colic - the uncontrolled crying and fussiness often blamed on sensitive stomachs or reflux. He found that of 154 mothers whose babies were having a routine two-month check-up, the migraine sufferers were 2.6 times as likely to have a baby with colic. Goadsby believes it is possible that a baby with a tendency to migraine may not cope well with the barrage of sensory information they experience as their nervous system starts to mature, and the distress response could be what we call colic.

SAMPLE

Linked to this idea, researchers are finding differences in the brain function of migraine sufferers, even between attacks. Marla Mickleborough, a vision specialist at the University of Saskatchewan in Saskatoon, Canada, found heightened sensitivity to visual stimuli in the supposedly 'normal' period between attacks. Usually the brain comes to recognise something repeating over and over again as unimportant and stops noticing it, but in people with migraine, the response doesn't diminish over time. 'They seem to be attending to things they should be ignoring,' she says.

Taken together this research is worrying and suggests that it's time for doctors to treat the condition more aggressively, and to find out more about each individual's triggers so as to stop attacks from happening. But there is a **silver lining**. The structural changes should not be likened to dementia, Alzheimer's disease or ageing, where brain tissue is lost or damaged irreparably. In migraine, the brain is compensating. Even if there's a genetic predisposition, research suggests it is the disease itself that is driving networks to an altered state. That would suggest that treatments that reduce the frequency or severity of migraine will probably be able to reverse some of the structural changes too. Treatments used to be all about reducing the immediate pain, but now it seems they might be able to achieve a great deal more.

SAMPLE

Text 2: Questions 15-22

15. Why does the writer tell the story of the news reporter?

(A) to explain the causes of migraine aura

(B) to address the fear surrounding migraine aura

(C) to illustrate the strange nature of migraine aura

(D) to clarify a misunderstanding about migraine aura

16. The research by Nouchine Hadjikhani into CSD

(A) has less relevance than many believe.

(B) did not result in a definitive conclusion.

(C) was complicated by technical difficulties.

(D) overturned years of accepted knowledge.

17. What does the word **'This'** in the second paragraph refer to?

(A) the theory that connects CSD and aura

(B) the part of the brain where auras take place

(C) the simultaneous occurrence of CSD and aura

(D) the ability to predict when an aura would happen

18. The implication of Hadjikhani's research into the somatosensory cortex is that

(A) migraine could cause a structural change.

(B) a lasting treatment for migraine is possible.

(C) some diagnoses of migraine may be wrong.

(D) having one migraine is likely to lead to more.

SAMPLE

19. What does the writer find surprising about Goadsby's research?

(A) the idea that migraine may not run in families

(B) the fact that migraine is evident in infanthood

(C) the link between childbirth and onset of migraine

(D) the suggestion that infant colic may be linked to migraine

20. According to Marla Mickleborough, what is unusual about the brain of migraine sufferers?

(A) It fails to filter out irrelevant details.

(B) It struggles to interpret visual input.

(C) It is slow to respond to sudden changes.

(D) It does not pick up on important information.

21. The writer uses the phrase **'a silver lining'** in the final paragraph to emphasise

(A) the privileged position of some sufferers.

(B) a more positive aspect of the research.

(C) the way migraine affects older people.

(D) the value of publicising the research.

22. What does the writer suggest about the brain changes seen in migraine sufferers?

(A) Some of them may be beneficial.

(B) They are unlikely to be permanent.

(C) Some of them make treatment unnecessary.

(D) They should still be seen as a cause for concern.

END OF READING TEST
THIS BOOKLET WILL BE COLLECTED

SAMPLE

READING SUB-TEST – ANSWER KEY

PART B: QUESTIONS 1-6

1	C	can delegate responsibility for the cupboard keys to another ward.
2	A	give a valid reason for conducting it.
3	A	help maximise its efficiency.
4	B	They enable a patient to receive more of the prescribed medicine.
5	A	benefits to patients of using bedrails can outweigh the dangers.
6	A	They may be useful for patients who are not fully responsive.

PART C: QUESTIONS 7-14

7	A	It was entirely preventable.
8	C	Staff focus their attention on a limited number of issues.
9	A	understands why healthcare employees have to make certain choices.
10	D	The information recorded on them does not always reflect reality.
11	D	lack of consistency
12	B	It isn't clear who ought to be tackling the situation.
13	B	illustrate a fundamental obstacle.
14	D	the approach they take to deal with challenges

PART C: QUESTIONS 15-22

15	C	to illustrate the strange nature of migraine aura
16	B	did not result in a definitive conclusion.
17	C	the simultaneous occurrence of CSD and aura
18	A	migraine could cause a structural change.
19	D	the suggestion that infant colic may be linked to migraine
20	A	It fails to filter out irrelevant details.
21	B	a more positive aspect of the research.
22	B	They are unlikely to be permanent.

E2 LANGUAGE

Practice test

READING SUB-TEST – TEXT BOOKLET: PART A

CANDIDATE NUMBER:
LAST NAME:
FIRST NAME:
OTHER NAMES:
PROFESSION:
VENUE:
TEST DATE:

Passport Photo

CANDIDATE SIGNATURE _____

Sedation: Iron deficiencies

Text A

Iron deficiency and iron deficiency anaemia are common. The serum ferritin level is the most useful indicator of iron deficiency, but interpretation can be complex. Identifying the cause of iron deficiency is crucial. Oral iron supplements are effective first-line treatment. Intravenous iron infusions, if required, are safe, effective and practical.

Key Points

- Measurement of the serum ferritin level is the most useful diagnostic assay for detecting iron deficiency, but interpretation may be difficult in patients with comorbidities.
- Identifying the cause of iron deficiency is crucial; referral to a gastroenterologist is often required.
- Faecal occult blood testing is not recommended in the evaluation of iron deficiency; a negative result does not impact on the diagnostic evaluation.
- Oral iron is an effective first-line treatment, and simple strategies can facilitate patient tolerance.
- For patients who cannot tolerate oral therapy or require more rapid correction of iron deficiency, intravenous iron infusions are safe, effective and practical, given the short infusion times of available formulations.
- Intramuscular iron is no longer recommended for patients of any age.

Text B

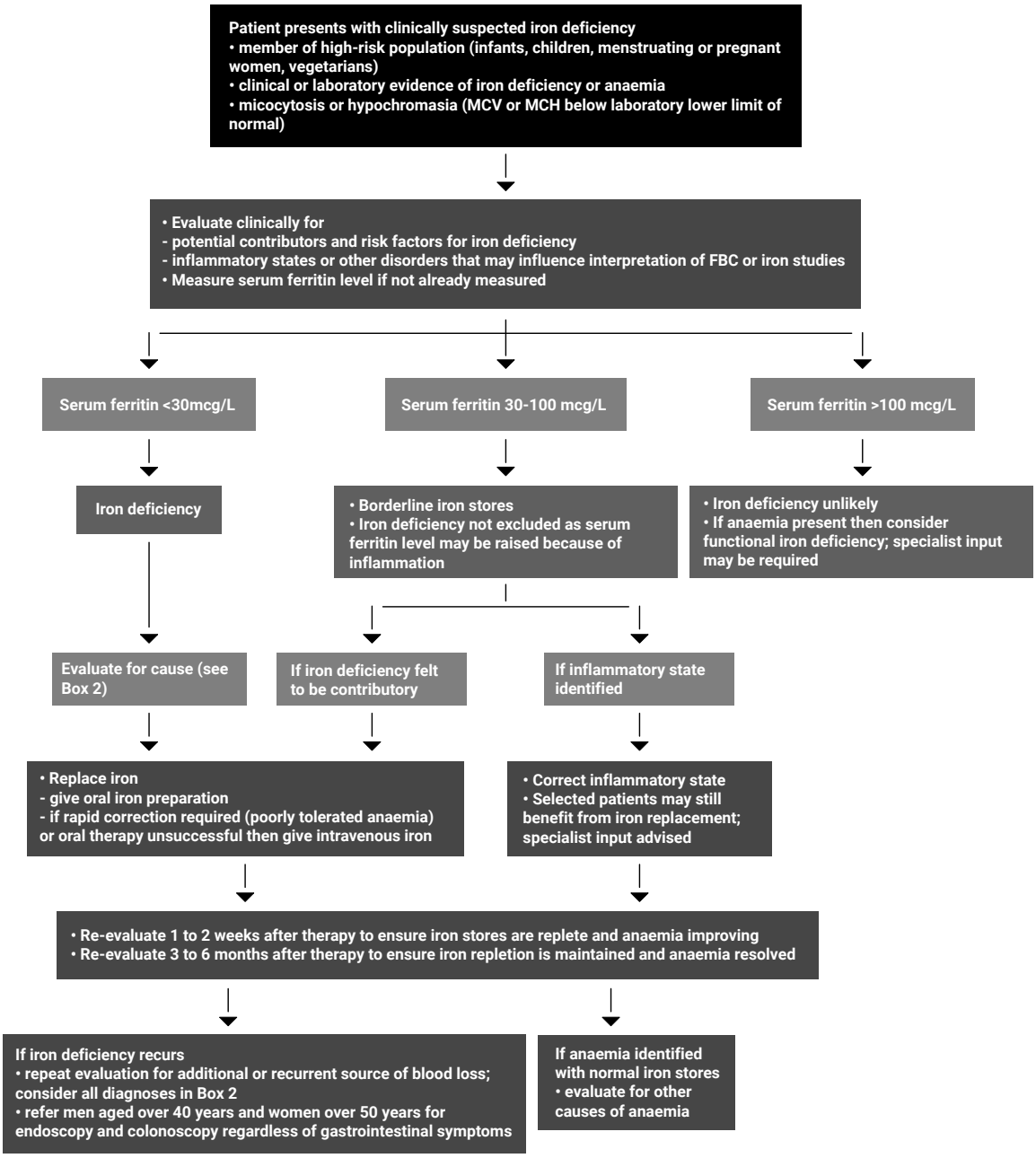
Treatment of infants and children

Although iron deficiency in children cannot be corrected solely by dietary change, dietary advice should be given to parents and carers. Cows' milk is low in iron compared with breast milk and infant formula, and enteropathy caused by hypersensitivity to cows' milk protein can lead to occult gastrointestinal blood loss. Excess cows' milk intake (in lieu of iron-rich solid foods) is the most common cause of iron deficiency in young children. Other risk factors for dietary iron deficiency include late introduction of or insufficient iron-rich foods, prolonged exclusive breastfeeding and early introduction of cows' milk.

Adult doses of iron can be toxic to children, and paediatric-specific protocols on iron supplementation should be followed. The usual paediatric oral iron dosage is 3 to 6mg/kg elemental iron daily. If oral iron is ineffective or not tolerated then consider other causes of anaemia, referral to a specialist paediatrician and use of IV iron.

Text C

AN ALGORITHM FOR THE IDENTIFICATION AND MANAGEMENT OF ADULTS WITH IRON DEFICIENCY



Text D

INTRAVENOUS PREPARATIONS FOR IRON REPLACEMENT				
Form of Iron	**Presentation**	**Maximum dose per administration**	**Dosing frequency**	**Rate of administration**
Ferric carboxymaltose	500 mg/10 mL vial or 100 mg/2 mL vial	1000 mg (or 20 mg/kg)	Maximum dose once per week, or 200 mg three times per week	IV injection or infusion 100-200mg: 3 minutes 200-500 mg: 6 minutes 500-1000 mg: 15 minutes
Iron polymaltose	100 mg/2 mL ampoule	2500 mg	Not applicable as entire dose can be delivered in single administration	IV infusion: first 50 mL infused slowly (20 to 40 mL/h); if tolerated then rate can be increased to 120 mL/h*
Iron sucrose	100 mg/5 mL ampoule	100 mg	Maximum three times per week	IV infusion 100 mg over 15 minutes
*Iron polymaltose can also be administered by the intramuscular route. Different maximum doses and dosing frequencies apply.				

END OF PART A
THIS TEXT BOOKLET WILL BE COLLECTED

- Look at the four texts, **A-D**, in the (printable) **Text Booklet**.
- For each question, **1-20**, look through the texts, **A-D**, to find the relevant information.
- Write your answers on the spaces provided in the **ANSWER SHEET.**
- Answer all the questions within the 15-minute time limit.

Iron Deficiency: Questions

Questions 1-7

For each of the questions, **1-7**, decide which text (**A, B, C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about

1 considerations when treating children with iron deficiency?

2 essential steps for identifying iron deficiency?

3 evaluating iron deficiency by testing for blood in stool?

4 risk factors associated with dietary iron deficiency?

5 different types of iron solutions?

6 a treatment for iron deficiency that is no longer supported?

7 appropriate dosage when administering IV iron infusions?

Questions 8-14

Answer each of the questions, **8-14**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.
Your answers should be correctly spelled.

8 What level of serum ferritin leads to a diagnosis of iron deficiency?

9 What is the most likely cause of iron deficiency in children?

10 Which form of iron can also be injected into the muscle?

11 What should a clinician do if iron stores are normal and anaemia is still present?

12 How long after iron replacement therapy should a patient be re-tested?

13 Which form of iron is presented in a vial?

14 What is the first type of treatment iron deficient patients are typically given?

Questions 15-20

Complete each of the sentences, **15-20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.
Your answers should be correctly spelled.

In comparison to breast milk and infant formula, cows' milk is **(15)**.....

Special procedures should be used because **(16)**..... may be poisonous for children
.

Men over 40 and women over 50 with a recurring iron deficiency should have an **(17)** .....

Iron sucrose can be given to a patient no more than **(18)**.....

Although serum ferritin level is a good indication of deficiency, interpreting the results is sometimes difficult **(19)**.....

IV iron infusions are a safe alternative when patients are unable to **(20)**.....

Answer Sheet

1) ***correct answer:*** *b*

2) ***correct answer:*** *c*

3) ***correct answer:*** *a*

4) ***correct answer:*** *b*

5) ***correct answer:*** *d*

6) ***correct answer:*** *a*

7) ***correct answer:*** *d*

8) ***correct answer:*** *<30 mcg/L / less than 30 mcg/L / < 30 mcg / L / <30mcg/L*

9) ***correct answer:*** *excess cow's milk / excess cow milk / excess cows' milk / excessive cow's milk / excessive cow milk / excessive cows' milk / excess cow's milk intake /*

excess cow milk intake / excess cows' milk intake / excessive cow's milk intake / excessive cow milk intake / excessive cows' milk intake

10) ***correct answer:*** *iron polymaltose*

11) ***correct answer:*** *consider other cases / evaluate other causes / evaluate for other causes*

12) ***correct answer:*** *1 to 2 weeks / one to two weeks / 1-2 weeks / 1 - 2 weeks*

13) ***correct answer:*** *ferric carboxymaltose*

14) ***correct answer:*** *oral iron / oral iron supplements*

15) ***correct answer:*** *low in iron*

16) ***correct answer:*** *adult doses of iron / adult iron doses*

17) ***correct answer:*** *endoscopy and colonoscopy / colonoscopy and endoscopy*

18) ***correct answer:*** *3 times per week / three times per week / 3 times a week / three times a week / 3 times weekly / three times weekly*

19) ***correct answer:*** *in patients with comorbidities*

20) ***correct answer:*** *tolerate oral iron / tolerate oral iron therapies / tolerate oral iron therapy*

E2 LANGUAGE

READING SUB-TEST – QUESTION PAPER: PART B

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

OTHER NAMES:

PROFESSION:

VENUE:

TEST DATE:

CANDIDATE SIGNATURE _____

Passport Photo

INSTRUCTIONS TO CANDIDATES:

DO NOT open this **Question Paper** until you are told to do so.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of this test, hand in this **Question Paper**.

DO NOT remove OET material from the test room.

HOW TO ANSWER THE QUESTIONS:

Mark your answers on this **Question Paper** by filling in the circle using a 2B pencil.

Example:

Ⓐ
Ⓑ
Ⓒ

www.e2language.com

Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For **questions 1-6**, choose answer (**A**, **B** or **C**) which you think fits best according to the text.

---

1. The code of conduct applies to

Ⓐ doctors friending patients on Facebook.

Ⓑ privacy settings when using social media.

Ⓒ electronic and face to face communication.

Professional obligations
The Code of conduct contains guidance about the required standards of professional behaviour, which apply to registered health practitioners whether they are interacting in person or online. The Code of conduct also articulates standards of professional conduct in relation to privacy and confidentiality of patient information, including when using social media. For example, posting unauthorised photographs of patients in any medium is a breach of the patient's privacy and confidentiality, including on a personal Facebook site or group, even if the privacy settings are set at the highest setting (such as for a closed, 'invisible' group).

2. Why does dysphagia often require complex management?

Ⓐ Because it negatively influences the cardiac system.

Ⓑ Because it is difficult contrast complex and non-complex cases.

Ⓒ Because it seldom occurs without other symptoms.

6.1 General principles

Dysphagia management may be complex and is often multi-factorial in nature. The speech pathologist's understanding of human physiology is critical. The swallowing system works with the respiratory system. The respiratory system is in turn influenced by the cardiac system, and the cardiac system is affected by the renal system. Due to the physiological complexities of the human body, few clients present with dysphagia in isolation.

6.2 Complex vs. non-complex cases

Broadly the differentiation between complex and non-complex cases relates to an appreciation of client safety and reduction in risk of harm. All clinicians, including new graduates, should have sufficient skills to appropriately assess and manage non-complex cases. Where a complex client presents, the skills of an advanced clinician are required. Supervision and mentoring should be sought for newly graduated clinicians or those with insufficient experience to manage complex cases.

3. The main point of the extract is

Ⓐ how to find documents about infection control in Australia.

Ⓑ that dental practices must have a guide for infection control.

Ⓒ that dental infection control protocols must be updated.

1 Documentation
1.1 Every place where dental care is provided must have the following documents in either hard copy or electronic form (the latter includes guaranteed Internet access). Every working dental practitioner and all staff must have access to: a). a manual setting out the infection control protocols and procedures used in that practice, which is based on the documents listed at sections 1.1(b), (c) and (d) of these guidelines and with reference to the concepts in current practice noted in the documents listed under References in these guidelines b). The current Australian Dental Association Guidelines for Infection Control (available at: http://www.ada.org.au)

4. Negative effects from prescription drugs are often

Ⓐ avoidable in young people.

Ⓑ unpredictable in the elderly.

Ⓒ caused by miscommunication.

Reasons for Drug-Related Problems: Manual for Geriatrics Specialists

Adverse drug effects can occur in any patient, but certain characteristics of the elderly make them more susceptible. For example, the elderly often take many drugs (polypharmacy) and have age-related changes in pharmacodynamics and pharmacokinetics; both increase the risk of adverse effects.

At any age, adverse drug effects may occur when drugs are prescribed and taken appropriately; e.g., new-onset allergic reactions are not predictable or preventable. However, adverse effects are thought to be preventable in almost 90% of cases in the elderly (compared with only 24% in younger patients). Certain drug classes are commonly involved: antipsychotics, antidepressants, and sedative-hypnotics.

In the elderly, a number of common reasons for adverse drug effects, ineffectiveness, or both are preventable. Many of these reasons involve inadequate communication with patients or between health care practitioners (particularly during health care transitions).

5. The guideline tries to use terminology that

Ⓐ presents value-free information about different social groups.

Ⓑ distinguishes disadvantaged groups from the traditional majority.

Ⓒ clarifies the proportion of each race, gender and culture.

Terminology

Terminology in this guideline is a difficult issue since the choice of terminology used to distinguish groups of persons can be personal and contentious, especially when the groups represent differences in race, gender, sexual orientation, culture or other characteristics. Throughout the development of this guideline the panel endeavoured to maintain neutral and non-judgmental terminology wherever possible. Terms such as “minority”, “visible minority”, “non-visible minority” and “language minority” are used in some areas; when doing so the panel refers solely to their proportionate numbers within the larger population and infers no value on the term to imply less importance or less power. In some of the recommendations the term “under-represented groups” is used, again, to refer solely to the disproportionate representation of some citizens in those settings in comparison to the traditional majority.

6. What is the purpose of this extract?

Ⓐ To illustrate situations where patients may find it difficult to give negative feedback.

Ⓑ To argue that hospital brochures should be provided in many languages.

Ⓒ To provide guidance to people who are victims of discrimination.

Special needs

Special measures may be needed to ensure everyone in your client base is aware of your consumer feedback policy and is comfortable with raising their concerns. For example, should you provide brochures in a language other than English?

Some people are less likely to complain for cultural reasons. For example, some Aboriginal people may be culturally less inclined to complain, particularly to non-Aboriginal people. People with certain conditions such as hepatitis C or a mental illness, may have concerns about discrimination that will make them less likely to speak up if they are not satisfied or if something is wrong.

1. C

2. C

3. B

4. C

5. A

6. A

E2 LANGUAGE

READING SUB-TEST – QUESTION PAPER: PART C

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

OTHER NAMES:

PROFESSION:

VENUE:

TEST DATE:

CANDIDATE SIGNATURE _____

Passport Photo

INSTRUCTIONS TO CANDIDATES:

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HOW TO ANSWER THE QUESTIONS:

Mark your answers on this **Question Paper** by filling in the circle using a 2B pencil.

Example:

Ⓐ
Ⓑ
Ⓒ

Part C

In this part of the test, there are two texts about different aspects of healthcare. For **questions 7-22**, choose the answer (**A**, **B**, **C** or **D**) which you think fits best according to the text.

Text 1: Difficult-to-treat depression

Depression remains a leading cause of distress and disability worldwide. In one country's survey of health and wellbeing in 2007, 7.2% of people surveyed had experienced a mood (affective) disorder in the previous 12 months. Those affected reported a mean of 11.7 disability days when they were "completely unable to carry out or had to cut down on their usual activities owing to their health" in the previous 4 weeks. There was also evidence of substantial under-treatment: amazingly only 35% of people with a mental health problem had a mental health consultation during the previous 12 months. Three-quarters of those seeking help saw a general practitioner (GP). In the 2015–16 follow-up survey, not much had changed. Again, there was evidence of substantial unmet need, and again GPs were the health professionals most likely to be providing care.

While GPs have many skills in the assessment and treatment of depression, they are often faced with people with depression who simply do not get better, despite the use of proven psychological or pharmacological therapies. GPs are well placed in one regard, as they often have a longitudinal knowledge of the patient, understand his or her circumstances, stressors and supports, and can marshal this knowledge into a coherent and comprehensive management plan. Of course, GPs should not soldier on alone if they feel the patient is not getting better.

In trying to understand what happens when GPs feel "stuck" while treating someone with depression, a qualitative study was undertaken that aimed to gauge the response of GPs to the term "difficult-to-treat depression". It was found that, while there was confusion around the exact meaning of the term, GPs could relate to it as broadly encompassing a range of individuals and presentations. More specific terms such as "treatment-resistant depression" are generally reserved for a subgroup of people with difficult-to-treat depression that has failed to respond to treatment, with particular management implications.

One scenario in which depression can be difficult to treat is in the context of physical illness. Depression is often expressed via physical symptoms, however it is also true is that people with chronic physical ailments are at high risk of depression. Functional pain syndromes where the origin and cause of the pain are unclear, are particularly tricky, as complaints of pain require the clinician to accept them as "legitimate", even if there is no obvious physical

cause. The use of analgesics can create its own problems, including dependence. Patients with comorbid chronic pain and depression require careful and sensitive management and a long-term commitment from the GP to ensure consistency of care and support.

It is often difficult to tackle the topic of depression co-occurring with borderline personality disorder (BPD). People with BPD have, as part of the core disorder, a perturbation of affect associated with marked variability of mood. This can be very difficult for the patient to deal with and can feed self-injurious and other harmful behaviour. Use of mentalisation-based techniques is gaining support, and psychological treatments such as dialectical behaviour therapy **form the cornerstone** of care. Use of medications tends to be secondary, and prescription needs to be judicious and carefully targeted at particular symptoms. GPs can play a very important role in helping people with BPD, but should not “go it alone”, instead ensuring sufficient support for themselves as well as the patient.

Another particularly problematic and well-known form of depression is that which occurs in the context of bipolar disorder. Firm data on how best to manage bipolar depression is surprisingly lacking. It is clear that treatments such as unopposed antidepressants can make matters a lot worse, with the potential for induction of mania and mood cycle acceleration. However, certain medications (notably, some mood stabilisers and atypical antipsychotics) can alleviate much of the suffering associated with bipolar depression. Specialist psychiatric input is often required to achieve the best pharmacological approach. For people with bipolar disorder, psychological techniques and long-term planning can help prevent relapse. Family education and support is also an important consideration.

Text 1: Questions 7-14

7. In the first paragraph, what point does the writer make about the treatment of depression?

Ⓐ 75% of depression sufferers visit their GP for treatment.

Ⓑ GPs struggle to meet the needs of patients with depression.

Ⓒ Treatment for depression takes an average of 11.7 days a month.

Ⓓ Most people with depression symptoms never receive help.

8. In the second paragraph, the writer suggests that GPs

Ⓐ are in a good position to conduct long term studies on their patients.

Ⓑ lack training in the treatment and assessment of depression.

Ⓒ should seek help when treatment plans are ineffective.

Ⓓ sometimes struggle to create coherent management plans.

9. What do the results of the study described in the third paragraph suggest?

Ⓐ GPs prefer the term “treatment resistant depression” to “difficult-to-treat depression”.

Ⓑ Patients with “difficult-to-treat depression” sometimes get “stuck” in treatment.

Ⓒ The term “difficult-to-treat depression” lacks a precise definition.

Ⓓ There is an identifiable sub-group of patients with “difficult-to-treat depression”.

10. Paragraph 4 suggests that

Ⓐ prescribing analgesics is unadvisable when treating patients with depression.

Ⓑ the co-occurrence of depression with chronic conditions makes it harder to treat.

Ⓒ patients with depression may have undiagnosed chronic physical ailments.

Ⓓ doctors should be more careful when accepting pain complaints as legitimate.

11. According to paragraph 5, people with BPD have

Ⓐ depression occurring as a result of the disorder

Ⓑ noticeable mood changes which are central to their disorder.

Ⓒ a tendency to have accidents and injure themselves.

Ⓓ problems tackling the topic of their depression.

12. In paragraph 5, what does the phrase ***'form the cornerstone'*** mean regarding BPD treatment?

Ⓐ Psychological therapies are generally the basis of treatment.

Ⓑ There is more evidence for using mentalisation than dialectical behaviour therapy.

Ⓒ Dialectical behaviour therapy is the optimum treatment for depression.

Ⓓ In some unusual cases prescribing medication is the preferred therapy.

13. In paragraph 6, what does the writer suggest about research into bipolar depression management?

Ⓐ There is enough data to establish the best way to manage bipolar depression.

Ⓑ Research hasn't provided the evidence for an ideal management plan yet.

Ⓒ A lack of patients with the condition makes it difficult to collect data on its management.

Ⓓ Too few studies have investigated the most effective ways to manage this condition.

14. In paragraph 6, what does the writer suggest about the use of medications when treating bipolar depression?

Ⓐ There is evidence for the positive and negative results of different medications.

Ⓑ Medications typically make matters worse rather than better.

Ⓒ Medication can help prevent long term relapse when combined with family education.

Ⓓ Specialist psychiatrists should prescribe medication for bipolar disorder rather than GPs.

Text 2: Are the best hospitals managed by doctors?

Doctors were once viewed as ill-prepared for leadership roles because their selection and training led them to become "heroic lone healers." However, the emphasis on patient-centered care and efficiency in the delivery of clinical outcomes means that physicians are now being prepared for leadership. The Mayo Clinic is America's best hospital, according to the 2016 US News and World Report (USNWR) ranking. Cleveland Clinic comes in second. The CEOs of both — John Noseworthy and Delos "Toby" Cosgrove — are highly skilled physicians. In fact, both institutions have been physician-led since their inception around a century ago. Might there be a general message here?

A study published in 2011 examined CEOs in the top-100 hospitals in USNWR in three key medical specialties: cancer, digestive disorders, and cardiovascular care. A simple question was asked: are hospitals ranked more highly when they are led by medically trained doctors or non-MD professional managers? The analysis showed that hospital quality scores are approximately 25% higher in physician-run hospitals than in manager-run hospitals. Of course, this does not prove that doctors make better leaders, though the results are surely consistent with that claim.

Other studies find a similar correlation. Research by Bloom, Sadun, and Van Reenen revealed how important good management practices are to hospital performance. However, they also found that it is the proportion of managers with a clinical degree that had the largest positive effect; in other words, the separation of clinical and managerial knowledge inside hospitals was associated with more negative management outcomes. Finally, support for the idea that physician-leaders are advantaged in healthcare is consistent with observations from many other sectors. Domain experts – "expert leaders" (like physicians in hospitals) — have been linked with better organizational performance in settings as diverse as universities, where scholar-leaders enhance the research output of their organizations, to basketball teams, where former All-Star players turned coaches are ***disproportionately linked*** to NBA success.

What are the attributes of physician-leaders that might account for this association with enhanced organizational performance? When asked this question, Dr. Toby Cosgrove, CEO of Cleveland Clinic, responded without hesitation, "credibility … peer-to-peer credibility." In other words, when an outstanding physician heads a major hospital, it signals that they have *"**walked the walk**".* The Mayo website notes that it is physician-led because, "This helps ensure a continued focus on our primary value, the needs of the patient come first." Having spent their careers looking through a patient-focused lens, physicians moving into executive positions might be expected to bring a patient-focused strategy.

In a recent study that matched random samples of U.S. and UK employees with employers, we found that having a boss who is an expert in the core business is associated with high levels of employee job satisfaction and low intentions of quitting. Similarly, physician-leaders may know how to raise the job satisfaction of other clinicians, thereby contributing to enhanced organizational performance. If a manager understands, through their own experience, what is needed to complete a job to the highest standard, then they may be more likely to create the right work environment, set appropriate goals and accurately evaluate others' contributions.

Finally, we might expect a highly talented physician to know what "good" looks like when hiring other physicians. Cosgrove suggests that physician-leaders are also more likely to tolerate innovative ideas like the first coronary artery bypass, performed by René Favaloro at the Cleveland Clinic in the late '60s. Cosgrove believes that the Cleveland Clinic unlocks talent by giving safe space to people with extraordinary ideas and importantly, that leadership tolerates appropriate failure, which is a natural part of scientific endeavour and progress.

The Cleveland Clinic has also been training physicians to lead for many years. For example, a cohort-based annual course, "Leading in Health Care," began in the early 1990s and has invited nominated, high-potential physicians (and more recently nurses and administrators) to engage in 10 days of offsite training in leadership competencies which fall outside the domain of traditional medical training. Core to the curriculum is emotional intelligence (with 360-degree feedback and executive coaching), teambuilding, conflict resolution, and situational leadership. The course culminates in a team-based innovation project presented to hospital leadership. 61% of the proposed innovation projects have had a positive institutional impact. Moreover, in ten years of follow-up after the initial course, 48% of the physician participants have been promoted to leadership positions at Cleveland Clinic.

Text 2: Questions 15-22

15. In paragraph 1, why does the writer mention the Mayo and Cleveland Clinics?

Ⓐ To highlight that they are the two highest ranked hospitals on the USNWR

Ⓑ To introduce research into hospital management based in these clinics

Ⓒ To provide examples to support the idea that doctors make good leaders

Ⓓ To reinforce the idea that doctors should become hospital CEOs

16. What is the writer's opinion about the findings of the study mentioned in paragraph 2?

Ⓐ They show quite clearly that doctors make better hospital managers.

Ⓑ They show a loose connection between doctor-leaders and better management.

Ⓒ They confirm that the top-100 hospitals on the USNWR ought to be physician-run.

Ⓓ They are inconclusive because the data is insufficient.

17. Why does the writer mention the research study in paragraph 3?

Ⓐ To contrast the findings with the study mentioned in paragraph 2

Ⓑ To provide the opposite point of view to his own position

Ⓒ To support his main argument with further evidence

Ⓓ To show that other researchers support him

18. In paragraph 3, the phrase ***'disproportionately linked'*** suggests

Ⓐ all-star coaches have a superior understanding of the game.

Ⓑ former star players become comparatively better coaches.

Ⓒ teams coached by former all-stars consistently outperform other teams.

Ⓓ to be a successful basketball coach you need to have played at a high level.

19. In the fourth paragraph, what does the phrase ***"walked the walk,"*** imply about physician-leaders?

Ⓐ They have earned credibility through experience.

Ⓑ They have ascended the ranks of their workplace.

Ⓒ They appropriately incentivise employees.

Ⓓ They share the same concerns as other doctors.

20. In paragraph 6, the writer suggests that leaders promote employee satisfaction because

Ⓐ they are often cooperative.

Ⓑ they tend to give employees positive evaluations.

Ⓒ they encourage their employees not to leave their jobs.

Ⓓ they understand their employees' jobs deeply.

21. In the seventh paragraph, why is the first coronary artery bypass operation mentioned?

Ⓐ To demonstrate the achievements of the Cleveland clinic

Ⓑ To present René Favaloro as an exemplar of a 'good' doctor

Ⓒ To provide an example of an encouraging medical innovation

Ⓓ To show how failure naturally contributes to scientific progress

22. In paragraph 8, what was the outcome of the course "Leading in Health Care"?

Ⓐ The Cleveland Clinic promoted almost half of the participants.

Ⓑ 61% of innovation projects lead to participants being promoted.

Ⓒ Some participants took up leadership roles outside the medical domain.

Ⓓ A culmination of more team-based innovations.

Reading Part C: Answer Key

7 d
8 c
9 c
10 b
11 b
12 a
13 b
14 a
15 c
16 a
17 c
18 b
19 a
20 d
21 c
22 a

E2 LANGUAGE

Practice test 2

READING SUB-TEST – TEXT BOOKLET: PART A

FOR THE QUESTIONS AND ANSWERS VISIT:
WWW.E2LANGUAGE.COM

CANDIDATE NUMBER:
LAST NAME:
FIRST NAME:
OTHER NAMES:
PROFESSION:
VENUE:
TEST DATE:

CANDIDATE SIGNATURE

Opioid dependence

Text A

Identifying opioid dependence

The International Classification of Disease, Tenth Edition [*ICD-10*] is a coding system created by the World Health Organization (WHO) to catalogue and name diseases, conditions, signs and symptoms.

The *ICD-10* includes criteria to identify dependence. According to the *ICD-10*, opioid dependence is defined by the presence of three or more of the following features at any one time in the preceding year:

- a strong desire or sense of compulsion to take opioids
- difficulties in controlling opioid use
- a physiological withdrawal state
- tolerance of opioids
- progressive neglect of alternative interests or pleasures because of opioid use
- persisting with opioid use despite clear evidence of overtly harmful consequences.

There are other definitions of opioid dependence or 'use disorder' (e.g. the *Diagnostic and Statistical Manual of Mental Disorders*, 5th edition, [*DSM-5*]), but the central features are the same. Loss of control over use, continuing use despite harm, craving, compulsive use, physical tolerance and dependence remain key in identifying problems.

Text B

WHY NOT JUST PRESCRIBE CODEINE OR ANOTHER OPIOID?

Now that analgesics containing codeine are no longer available OTC (over the counter), patients may request a prescription for codeine. It is important for GPs to explain that there is a lack of evidence demonstrating the long-term analgesic efficacy of codeine in treating chronic non-cancer pain. Long-term use of opioids has not been associated with sustained improvement in function or quality of life, and there are increasing concerns about the risk of harm.

GPs should explain that the risks associated with opioids include tolerance leading to dose escalation, overdose, falls, accidents and death. It should be emphasised that OTC codeine-containing analgesics were only intended for short-term use (one to three days) and that longer-term pain management requires a more detailed assessment of the patient's medical condition as well as clinical management.

New trials have shown that for acute pain, nonopioid combinations can be as effective as combination analgesics containing opioids such as codeine and oxycodone. If pain isn’t managed with nonopioid medications then consider referring the patient to a pain specialist or pain clinic.

Patient resources for pain management are freely available online to all clinicians at websites such as:

- Pain Management Network in NSW - www.aci.health.nsw.gov.au/networks/pain-management
- Australian and New Zealand College of Anaesthetists Faculty of Pain Medicine - www.fpm.anzca.edu.au

Text C

TABLE 2. COMPARISON OF MEDICATION APPROACHES FOR SHORT-TERM MANAGEMENT OF OPIOID WITHDRAWAL (OVER SEVEN TO 10 DAYS)[8-16]

Medication	Regulatory requirements	Requires off-label use	Strength of evidence	Advantages	Disadvantages	Notes on dosage
Buprenorphine–naloxone (sublingual)	Permit before treatment; patient registered as drug dependent in some jurisdictions	Indicated for opioid dependence (off-label for pain)	Multiple well-conducted RCTs demonstrate efficacy and safety[8]	• A large evidence base (e.g. Cochrane reviews) shows it is the most effective option for opioid taper[16]	• More restrictive than other options (regulatory requirements and supervised dosing) • Indicated only for opioid dependence, not for chronic pain in the absence of opioid dependence	• See sample withdrawal regimen (Box 8) or refer to state and national guidelines
Tramadol (oral)	Only those that apply to S4 medications	Yes (indicated for pain not opioid dependence)	Low – a small number of RCTs demonstrate efficacy and safety[9-12]	• A small number of studies examined a one-week tramadol taper, with outcomes comparable with those of other opioid tapers and superior to clonidine	• May produce serotonergic side effects, known drug interactions, use with caution in the elderly • Use is off label • Risk of seizures, even at usual doses • Variable metabolism through CYP P450, similar to codeine	• 100 to 300 mg sustained-release formulation twice a day for one week • Supervised dosing and daily dispensing may be indicated
Buprenorphine (transdermal patch)	Permit before treatment if patient is drug dependent	Yes (indicated for pain not opioid dependence)	None – no RCTs or published cases	• Good safety profile	• Dose likely to be insufficient for patients with clear evidence of opioid dependence	• 5, 10 or 20 mcg weekly patches are available • A single patch should be sufficient for taper from oral codeine
Symptomatic medications	Only those relating to S4/OTC medications	No (if use is consistent with product indication)	Moderate to high – well-conducted RCTs demonstrate efficacy; however, poorer outcomes than buprenorphine or tramadol[15]	• Fewer prescribing restrictions • Relatively safe in outpatient setting	• Shown to be less effective than buprenorphine and tramadol • Multiple medications can be confusing • Caution using sedative medications in outpatient setting • Clonidine can cause severe hypotension	• See Table 1

Abbreviations: OTC = over the counter; RCT = randomised controlled trial.

Text D

Preparation for tapering

As soon as a valid indication for tapering of opioid analgesics is established, it is important to have a conversation with the patient to explain the process and develop a treatment agreement. This agreement could include:

- time frame for the agreement
- objectives of the taper
- frequency of dose reduction
- requirement for obtaining the prescriptions from a designated clinician
- scheduled appointments for regular review
- anticipated effects of the taper
- consent for urine drug screening
- possible consequences of failure to comply.

Before starting tapering, it needs to be clearly emphasised to the patient that reducing the dose of opioid analgesia will not necessarily equate to increased pain and that it will, in effect, lead to improved mood and functioning as well as a reduction in pain intensity. The prescriber should establish a therapeutic alliance with the patient and develop a shared and specific goal.

END OF PART A
THIS TEXT BOOKLET WILL BE COLLECTED

E2 Language Part A.2

- Look at the four texts, **A-D**, in the (printable) **Text Booklet**.
- For each question, **1-20**, look through the texts, **A-D**, to find the relevant information.
- Write your answers on the spaces provided in the **ANSWER SHEET**.
- Answer all the questions within the 15-minute time limit.

Managing Opioid Dependence

Questions 1-7
For each of the questions, **1-7**, decide which text (**A, B, C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about...

1 what GPs should say to patients requesting codeine?

2 basic indications of an opioid problem?

3 different medications used for weaning patients off opioids?

4 decisions to make before beginning treatment of dependence?

5 defining features of a use disorder?

6 the development of a common goal for both prescriber and patient?

7 sources of further information on pain management?

Questions 8-14
Answer each of the questions, **8-14**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both. Your answers should be correctly spelled.

8 What will reduced doses of opioids lead to a reduction of?

9 What is the most effective medication for tapering opioid dependence?

10 How long should over the counter codeine analgesics be used for?

11 When should doctors consider referring a patient to a pain expert or clinic?

12 What might a patient give permission to before starting treatment?

13 What might be increasingly neglected as a result of opioid use?

14 How many Buprenorphine patches are needed to taper from codeine tablets?

Questions 15-20
Complete each of the sentences, **15-20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both. Your answers should be correctly spelled.

The use of Buprenorphine-naxolone requires a **(15)**..... before treatment.

The use of symptomatic medications for the treatment of opioid dependence has been found to have **(16)**..... than tramadol.

Different definitions of opioid dependence share the same **(17)**.....

Once it is decided that opioid taper is a suitable treatment the doctor and patient should create a (**18)**.....

Recent research indicates that **(19)**..... can work as well as combination analgesics including codeine and oxycodone.

The ICD-10 defines a patient as dependent if they have **(20)**..... key symptoms simultaneously.

Answer Sheet

1) ***correct answer:*** *b*

2) ***correct answer:*** *a*

3) ***correct answer:*** *c*

4) ***correct answer:*** *d*

5) ***correct answer:*** *a*

6) ***correct answer:*** *d*

7) ***correct answer:*** *b*

8) ***correct answer:*** *pain intensity*

9) ***correct answer:*** *buprenorphine-naloxone / buprenorphine - naloxone / buprenorphine-naloxone (sublingual) / buprenorphine - naloxone (sublingual)*

10) ***correct answer:*** *one to three days / 1 to 3 days / 1-3 days / 1 - 3 days*

11) ***correct answer:*** *if pain isn't managed with nonopioid medications / if pain isn't managed / if pain isn't managed with non-opioid medications*

12) ***correct answer:*** *urine drug screening*

13) ***correct answer:*** *alternative interests or pleasures / alternative interests and pleasures / interests or pleasures / interests and pleasures*

14) ***correct answer:*** *a single patch / one patch / 1 patch*

15) ***correct answer:*** *permit*

16) ***correct answer:*** *poorer outcomes*

17) ***correct answer:*** *central features / features*

18) ***correct answer:*** *treatment agreement*

19) ***correct answer:*** *nonopioid combinations / non-opioid combinations*

20) ***correct answer:*** *three or more / at least three / 3 or more / at least 3*

E2 Language Reading Part B.2

1. According to the guidelines nurses must

    advise the practice as soon as they get to the next home visit.

    call the patient to confirm a time before they make a home visit.

    inform fellow staff members when they return from a home visit.

Home Visit Guidelines

The nurse will complete all consultation notes in the patient's home (unless not appropriate), prior to beginning the next consultation. With a focus on nurse safety, the nurse will call the practice at the end of each visit before progressing to the next home visit and will also communicate any unexpected circumstances that may delay arrival back at the practice (more than one hour).

Calling from the patient's home to make a review appointment with the GP is sufficient and can help minimise time making phone calls. On return to the practice the nurse will immediately advise staff members of their return. This time will be documented on the patient visit list, scanned and filed by administration staff.

2. In progressive horizontal evacuation

    patients are evacuated through fire proof barriers one floor at a time.

    patients who can't walk should not be moved until the fire is under control.

    patients are moved to fire proof areas on the same level to safely wait for help.

Progressive horizontal evacuation

The principle of progressive horizontal evacuation is that of moving occupants from an area affected by fire through a fire-resisting barrier to an adjoining area on the same level, designed to protect the occupants from the immediate dangers of fire and smoke (a refuge). The occupants may remain there until the fire is dealt with or await further assisted onward evacuation by staff to a similar adjoining area or to the nearest stairway. Should it become necessary to evacuate an entire storey, this procedure should give sufficient time for non-ambulant and partially ambulant patients to be evacuated vertically to a place of safety.

3. The main purpose of the extract is to

    provide information of the legal requirements for disposing of animal waste.

    describe rules for proper selling and export of animal products.

    define the meaning of animal by-products for healthcare researchers.

Proper disposal of animal waste

Animal by-products from healthcare (for example research facilities) have specific legislative requirements for disposal and treatment. They are defined as "entire bodies or parts of animals or products of animal origin not intended for human consumption, including ova, embryos and semen." The Animal By-Products Regulations are designed to prevent animal by-products from presenting a risk to animal or public health through the transmission of disease. This aim is achieved by rules for the collection, transport, storage, handling, processing and use or disposal of animal byproducts, and the placing on the market, export and transit of animal by-products and certain products derived from them.

4. According to the extract, what is the outcome of reusing medical equipment meant to be used once?

    The maker will take no legal responsibility for safety.

    Endoscopy units will save on equipment costs.

    There is a higher incidence of cross infection.

Cleaning and disinfection of endoscopes should be undertaken by trained staff in a dedicated room. Thorough cleaning with detergent remains the most important and first step in the process. Automated washer/disinfectors have become an essential part of the endoscopy unit. Machines must be reliable, effective, easy to use and should prevent atmospheric pollution by the disinfectant if an irritating agent is used. Troughs of disinfectant should not be used unless containment or exhaust ventilated facilities are provided.

Whenever possible, "single use" or autoclavable accessories should be used. The risk of transfer of infection from inadequately decontaminated reusable items must be weighed against the cost. Reusing accessories labelled for single use will transfer legal liability for the safe performance of the product from the manufacturer to the user or his/her employers and should be avoided unless Department of Health criteria are met.

5. According to the extract what is the purpose of the guidelines?

    To present statistics on the incidence of melanoma in Australia and New Zealand.

    To support the early detection of melanoma and select the best treatments.

    To explain the causes of melanoma in populations of Celtic origin.

Foreword

Australia and New Zealand have the highest incidence of melanoma in the world. Comprehensive, up-to-date, evidence-based national guidelines for its management are therefore of great importance. Both countries have populations of predominantly Celtic origin , and in the course of day-to-day life their citizens are inevitably subjected to high levels of

solar UV exposure. These two factors are considered predominantly responsible for the very high incidence of melanoma (and other skin cancers) in the two nations. In Australia, melanoma is the third most common cancer in men and the fourth most common in women, with over 13, 000 new cases and over 1, 750 deaths each year.

The purpose of evidence-based clinical guidelines for the management of any medical condition is to achieve early diagnosis whenever possible, make doctors and patients aware of the most effective treatment options, and minimise the financial burden on the health system by documenting investigations and therapies that are inappropriate.

6. What should employees declare?

    Every item received from one donor.

    Each item from one donor valued at over $50.

    Every item from one donor if the combined value is more than $50.

Reporting of Gifts and Benefits

Employees must declare all non-token gifts which they are offered, regardless of whether or not those gifts are accepted. If multiple gifts, benefits or hospitality are received from the same donor by an employee and the cumulative value of these is more than $50 then each individual gift, benefit or hospitality event must be declared.

The Executive Director of Finance will be responsible for ensuring the gifts and benefits register is subject to annual review by the Audit Committee. The review should include analysis for repetitive trends or patterns which may cause concern and require corrective and preventive action. The Audit Committee will receive a report at least annually on the administration and quality control of the gifts, benefits and hospitality policy, processes and register.

Answers

1. **C**
2. **C**
3. **A**
4. **A**
5. **B**
6. **C**

E2 Language Reading Part C.2

Extract 1

Text 1: The case for and against e-cigarettes

Electronic cigarettes first hit European and American markets in 2006 and 2007, and their popularity has been propelled by international trends favouring smoke-free environments. Sales reportedly have reached $650 million a year in Europe and were estimated to reach $3. 6 billion in the US in 2018.

Although research on e-cigarettes is not extensive, a picture is beginning to emerge. Surveys suggest that the vast majority of those who use e-cigarettes treat them as smoking-cessation aides and self-report that they have been key to quitting. Data also indicate that e-cigarettes help to reduce tobacco cigarette consumption. A 2011 survey, based on a cohort of first-time e-cigarette purchasers, found that 66. 8 percent reported reducing the number of cigarettes they smoked per day and after six months, 31 percent reported not smoking. These results compare favorably with nicotine replacement therapies (NRTs) like the patch and nicotine gum. Interestingly, a randomized controlled trial found that even e-cigarettes not containing nicotine were effective both in achieving a reduction of tobacco cigarette consumption and longer term abstinence, suggesting that "factors such as the rituals associated with cigarette handling and manipulation may also play an important role. " Some tobacco control advocates worry that they simply deliver an insufficient amount of nicotine to ultimately prove effective for cessation.

Nevertheless, the tobacco control community has embraced FDA approved treatments—NRTs, as well as the drugs bupropion and varenicline —that have relatively low success rates . In a commentary published in the Journal of the American Medical Association, smoking cessation experts Andrea Smith and Simon Chapman of the University of Sydney said that smoking cessation drugs fail most of those who try them. "Sadly, it remains the case that by far the most common outcome at 6 to 12 months after using such medication in real world settings is continuing smoking. Few, if any, other drugs with such records would ever be prescribed, " they wrote.

Amongst smokers not intending to quit, e-cigarettes—both with and without nicotine—substantially reduced consumption in a randomized controlled trial, not only resulting in decreased cigarette consumption but also in "enduring tobacco abstinence. " In a second study from 2013, the authors reported that after 24 months, 12. 5 percent of smokers remained abstinent while another 27. 5 percent reduced their tobacco cigarette consumption by 50 percent. Finally, a third study commissioned in Australia has come to the same conclusion, though a high dropout rate (42 percent) makes these findings questionable.

Users widely perceive e-cigarettes to be less toxic. While the FDA has found trace elements of carcinogens, levels are comparable to those found in nicotine replacement therapies. Results from a laboratory study released in 2013 found that that while e-cigarettes do contain contaminants, the levels range from 9 to 450 times lower than in tobacco cigarette smoke. **These** are comparable with the trace amounts of toxic or carcinogenic substances found in medicinal nicotine inhalers. A prominent anti-tobacco advocate, Stanton Glantz, has warned of the need to protect people from secondhand emissions. While one laboratory study indicates that passive "vaping, " as smoking an e-cigarette is commonly known, releases volatile organic compounds and ultrafine particles into the indoor environment, it noted that the actual health impact is unknown and should remain a chief concern. A 2014 study

concluded that e-cigarettes are a source of second hand exposure to nicotine but not to toxins. Nevertheless, bystanders are exposed to 10 times less nicotine exposure from e-cigarettes compared to tobacco cigarettes.

There are a number of interesting points of agreement among proponents and skeptics of e-cigarettes. First, all agree that regulation to ensure the quality of e-cigarettes should be **uniform**. Laboratory analyses have found sometimes wide variation across brands, in the level of carcinogens, the presence of contaminants, and the quality of nicotine. Second, proponents and detractors of e-cigarettes tend to agree that — considered only at the individual level—e-cigarettes are a safer alternative to tobacco cigarette consumption. The main concern is how e-cigarettes might shape tobacco use patterns at the population level. Proponents stress the evidence base that we have reviewed. Skeptics remain worried that e-cigarettes will become “dual use” products. That is, smokers will use e-cigarettes, but will not reduce their smoking or quit.

Perhaps most troubling to public health officials is that e-cigarettes will "renormalize" smoking, subverting the cultural shift that has occurred over the past 50 years and transforming what has become a perverse habit into a pervasive social behaviour. In other words, the fear is that e-cigarettes will allow for re-entry of the tobacco cigarette into public view. This would unravel the gains created by smoke-free indoor (and, in some scientifically-unwarranted instances) outdoor environments. Careful epidemiological studies will be needed to determine whether the individual gains from e-cigarettes will be counteracted by population-level harms. For policy makers, the challenge is how to act in the face of uncertainty.

1. What does the writer suggest about the research into e-cigarettes?

   Not enough research is being carried out.

   Early conclusions are appearing from the evidence.

   Too much of the available data is self-reported.

   An extensive picture of e-cigarette use has emerged.

2. What explanation does the writer offer for the effect of non-nicotine e-cigarettes?

   They deliver an insufficient volume of nicotine to help smoking cessation.

   They compare well with patches, nicotine gum and other NRT's.

   First time e-cigarette buyers tend to use them

   Behavioural elements are significant in quitting smoking.

3. What is the attitude of Andrea Smith and Simon Chapman to the use of smoking cessation drugs?

   They approve of and embrace these treatments.

   They consider them largely unsuccessful as treatments.

They think they should be replaced with other treatments.

They believe they should never be prescribed as treatment.

4. What problem with one of the studies is mentioned in paragraph 4?

   The research questions the study asked.

   The number of participants who left the study.

   The similarity of the conclusion to other studies.

   The study used e-cigarettes without nicotine.

5. What is **"these"** in paragraph 5 referring to?

   Laboratory study results

   Nicotine inhalers

   Contamination levels

   Tobacco cigarettes

6. Research mentioned in paragraph 5 suggests that

   E-cigarettes release dangerous toxins into the air.

   E-cigarettes should be banned from indoor environments.

   E-cigarettes are more toxic than nicotine replacement therapies

   cigarettes present a far greater risk of secondhand exposure to toxins

7. The word **uniform** in paragraph 7 suggests that e-cigarettes should

   Be clearly regulated against.

   Only come in one brand.

   Be of a standard quality.

   Contain no contaminants.

8. What do both critics and supporters of e-cigarettes agree?

   Available research evidence must be reviewed.

   E-cigarette use may not result in quitting.

   Smoking tobacco is more dangerous than vaping.

E-cigarettes are shaping the public's tobacco use.

Answers

1. **B**
2. **D**
3. **B**
4. **B**
5. **C**
6. **D**
7. **C**
8. **C**

E2 Language Reading Part C.2

Extract 2

Text 2: Vivisection

In 1875, Charles Dodgson, under his pseudonym Lewis Carroll, wrote a blistering attack on vivisection. He sent this to the governing body of Oxford University in an attempt to prevent the establishment of a physiology department. Today, despite the subsequent evolution of one of the most rigorous governmental regulatory systems in the world, little has changed. A report sponsored by the UK Royal Society, "The use of non-human primates in research", attempts to establish a sounder basis for the debate on animal research through an in-depth analysis of the scientific arguments for research on monkeys.

In the UK, no great apes have been used for research since 1986. Of the 3000 monkeys used in animal research every year, 75% are for toxicology studies by the pharmaceutical industry. Although expenditure on biomedical research has almost doubled over the past 10 years, the number of monkeys used for this purpose (about 300) has tended to fall. The report, which mainly discusses the use of monkeys in biomedical research, pays particular attention to the development of vaccines for AIDS, malaria, and tuberculosis, and to the nervous system and its disorders. The report assesses the impact of these issues on global health, together with potential approaches that might avoid the use of animals in research. Other research areas are also discussed, together with ethics, animal welfare, drug discovery, and toxicology.

The report concludes that in some cases there is a valid scientific argument for the use of monkeys in medical research. However, no blanket decisions can be made because of the speed of progress in biomedical science (particularly in molecular and cell biology) and because of the available non-invasive methods for study of the brain. Every case must be considered individually and supported by a fully informed assessment of the importance of the work and of alternatives to the use of animals.

Furthermore, the report asks for greater openness from medical and scientific journals about the amount of animal suffering that occurred in studies and for regular publication of the outcomes of animal research and toxicology studies. It calls for the development of a national strategic plan for animal research, including the dissemination of information about

alternative research methods to the use of animals, and the creation of centres of excellence for better care of animals and for training of scientists. Finally, it suggests some approaches towards a better-informed public debate on the future of animal research.

Although the report was received favourably by the mass media, animal-rights groups thought that it did not go far enough in setting priorities for development of alternatives to the use of animals. In fact, it investigates many of these approaches, including cell and molecular biology, use of transgenic mice (an alternative to use of primates), computer modelling, in-silico technology, stem cells, microdosing, and pharmacometabonomic phenotyping. However, the report concludes that although many of these techniques have great promise, they are at a stage of development that is too early for assessment of their true potential.
The controversy of animal research continues unabated. Shortly after publication of the report , two highly charged stories were published in the media. A study that used systematic reviews to compare treatment outcome from clinical trials of animals with those of human beings suggested that discordance in the results might have been due to bias, poor design, or inadequacies of animals for modelling of human disease. Although the study made some helpful suggestions for the future, its findings are not surprising. The imperfections of animals for study of human disease and of drug trials are documented widely.

The current furore about the UK Government's ban on human nuclear-transfer experiments involving animals should not surprise us either. This area of research had a bad start when this method of production of stem cells was labelled as therapeutic cloning, thus confusing it with reproductive cloning - a problem that, surely, licensing bodies and the scientific community should have anticipated. The possibilities that insufficient human eggs will be available, and that insertion of human nuclei into animal eggs might be necessary, have been discussed by the scientific community for several years, but have been aired rarely in public, leaving much room for confusion

Biomedical science is progressing so quickly that maintenance of an adequate level of public debate on ethical issues is difficult. Hopefully the sponsors of the recent report will now activate its recommendations, not least how better mechanisms can be developed to broaden and sustain interactions between science and the public. Although any form of debate will probably not satisfy the extremists of the antivivisection movement, the rest of society deserves to receive the information it needs to deal with these extremely difficult issues.

1. How does the writer characterise Lewis Carroll's attitude to vivisection?

   He was in favour of clear regulations to control it.

   He felt the Royal Society should not support it.

   He was strongly opposed to it.

   He supported its use in physiology.

2. The word **rigorous** in paragraph 1 implies that the writer thinks UK vivisection laws are

   Strict and severe

   Careful and thorough

Ambiguous and unhelpful

Accurate and effective

3. What is the major focus of the report mentioned in paragraph 2?

Animal experimentation in the pharmaceutical industry

Recent increases in spending on Biomedical research

Testing new treatments for serious disease on monkeys

Possible alternatives to testing new drugs on animals

4. What is the main conclusion of the report?

Scientific experimentation on monkeys is justified.

Rapid development in biomedicine makes it hard to draw conclusions.

Non-invasive techniques should be preferred in most cases.

Research that requires monkeys should be evaluated independently.

5. What conclusion is drawn about alternative techniques to vivisection?

Developing alternatives should be prioritised.

Transgenic mice are a viable alternative to monkeys.

Many alternative techniques are more promising than animal testing.

They aren't well enough understood yet to adopt for research.

6. What does the writer claim about the use of animals in medical research?

The limitations of using animals in research are well understood.

Results from too many animal trials are biased.

Human studies are known to be more reliable.

Strong media reaction has kept up the controversy.

7. The phrase **a problem** in paragraph 6 refers to the

Government licensing of animal experiments.

Confusion between the names of two different methods.

Chortage of human embryos available for experiments.

Prohibition against human nuclear transfer in the UK.

8. The author thinks it is hard to keep the public adequately informed about this research because

    The report sponsors have not activated the recommendations.

    Of the rapid evolution of biomedical technologies.

    Scientists don't interact with the public enough.

    Extreme views from opponents cloud the debate.

Answers

1. **C**
2. **B**
3. **C**
4. **D**
5. **D**
6. **A**
7. **B**
8. **B**

Test 8

Practice test 3

READING SUB-TEST – TEXT BOOKLET: PART A

FOR THE QUESTIONS AND ANSWERS VISIT:
WWW.E2LANGUAGE.COM

CANDIDATE NUMBER:
LAST NAME:
FIRST NAME:
OTHER NAMES:
PROFESSION:
VENUE:
TEST DATE:

Passport Photo

CANDIDATE SIGNATURE _____

ADHD

Text A

The GP's role in the management of ADHD

It helps to remind patients that ADHD is not all bad. ADHD is associated with positive attributes such as being more spontaneous and adventurous. Some studies have indicated that people with ADHD may be better equipped for lateral thinking. It has been suggested that explorers or entrepreneurs are more likely to have ADHD.

In addition, GPs can reinforce the importance of developing healthy sleep–wake behaviours, obtaining adequate exercise and good nutrition. These are the building blocks on which other treatment is based. For patients who are taking stimulant medication, it is helpful if the GP continues to monitor their blood pressure, given that stimulant medication may cause elevation. Once a patient has been stabilised on medication for ADHD, the psychiatrist may refer the patient back to the GP for ongoing prescribing in line with state-based guidelines. However, in most states and territories, the GP is not granted permission to alter the dose.

Text B

ADHD: Overview

Contrary to common belief, ADHD is not just a disorder of childhood. At least 40 to 50% of children with ADHD will continue to meet criteria in adulthood, with ADHD affecting about one in 20 adults. ADHD can be masked by many comorbid disorders that GPs are typically good at recognising such as depression, anxiety and substance use. In patients with underlying ADHD, the attentional, hyperactive or organisational problems pre-date the comorbid disorders and are not episodic as the comorbid disorders may be. GPs are encouraged to ask whether the complaints are of recent onset or longstanding. Collateral history can be helpful for developing a timeline of symptoms (e.g. parent or partner interview). Diagnosis of underlying ADHD in these patients will significantly improve their treatment outcomes, general health and quality of life.

Text C

TABLE 2. MEDICATIONS FOR ATTENTION DEFICIT HYPERACTIVITY DISORDER AND TYPICAL DOSING

Medication	Initiation	Dose
Immediate-release methylphenidate	5 to 10 mg in the morning the first day; add a second dose of 5 to 10 mg at lunch time for a week; then add further increments weekly	Total dose typically varies between 10 mg/day and 60 mg/day Doses of more than 80 mg/day are uncommon (maximum recommended dose in the NICE guidelines is 100 mg/day)[11] Transition to longer-acting formulations can occur after a month
Extended-release methylphenidate	18 or 36 mg/day taken once daily in the morning	Increase in 18 mg increments to a maximum of 72 mg/day Adjust dosage at weekly intervals
Long-acting methylphenidate	20 mg/day taken once daily in the morning	Adjust dose weekly in 10 mg increments Dose usually would not exceed 60 mg/day
Dexamfetamine	2.5 to 5.0 mg in the morning the first day; add a second dose of 2.5 to 5.0 mg at lunch time for a week; then add further increments weekly	Total dose typically varies between 5 mg/day and 30 mg/day Doses over 40 mg/day are uncommon (maximum recommended dose in the NICE guidelines is 60 mg/day)[11]
Lisdexamfetamine	30 mg in the morning the first day; increase up to 70 mg according to response	Dose range typically 30 to 70 mg/day
Atomoxetine	For those weighing less than 70 kg, start at 0.5 mg/kg taken once daily for three days then increase to 1.2 mg/kg once daily in the morning or as evenly divided doses in the morning and late afternoon/early evening. For those weighing more than 70 kg, start at 40 mg/day taken once daily for three days then increase to target dose of 80 mg[3]	Target dose 80 mg/day Maximum dose 100 mg[3]

Text D

Treatment of ADHD

It is very important that the dosage of medication is individually optimised. An analogy may be made with getting the right pair of glasses – you need the right prescription for your particular presentation with not too much correction and not too little. The optimal dose typically requires careful titration by a psychiatrist with ADHD expertise. Multiple follow-up appointments are usually required to maximise the treatment outcome. It is essential that the benefits of treatment outweigh any negative effects. Common side effects of stimulant medication may include:

- appetite suppression
- insomnia
- palpitations and increased heart rate
- feelings of anxiety
- dry mouth and sweating

END OF PART A
THIS TEXT BOOKLET WILL BE COLLECTED

E2 Language Reading Part A.3

- Look at the four texts, **A-D**, in the (printable) **Text Booklet**.
- For each question, **1-20**, look through the texts, **A-D**, to find the relevant information.
- Write your answers on the spaces provided in the **ANSWER SHEET**.
- Answer all the questions within the 15-minute time limit.

ADHD

Questions 1-7
For each of the questions, **1-7**, decide which text (**A, B, C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about...

1 different types of ADHD medication?

2 possible side effects of medication?

3 conditions which may be present alongside ADHD?

4 a doctor's control over a patient's medication?

5 positive perspectives on having ADHD?

6 when patients should take their ADHD medicine?

7 figuring out a patient's optimal dosage of medication?

Questions 8-14
Answer each of the questions, **8-14**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both. Your answers should be correctly spelled.

8 What is the maximum recommended dose of Dexamfetamine?

9 What is typically needed to get the best results from ADHD treatment?

10 How can GP's collect information about their patient's collateral history?

11 What causes symptoms such as palpitations and anxiety in some patients?

12 What proportion of children with ADHD will carry symptoms into adulthood?

13 What positive personality traits are sometimes associated with ADHD?

14 Which medication has dose recommendations related to patient weight?

Questions 15-20
Complete each of the sentences, **15-20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both. Your answers should be correctly spelled.

Sleep, exercise and nutrition comprise the **(15)**..... of further ADHD treatment.

When diagnosing ADHD, it is important to ask if the issues arose recently or are **(16)**.....

It is possible to move to **(17)**..... after one month of immediate-release methylphenidate.

Signs of ADHD can be disguised by **(18)**..... which GPs are more likely to recognise.

GPs should regularly check the **(19)**..... of patients prescribed stimulant medication. Establishing the ideal dose of ADHD medication needs **(20)**..... by an expert psychiatrist.

Answer Sheet

1) ***correct answer:*** *c*

2) ***correct answer:*** *d*

3) ***correct answer:*** *b*

4) ***correct answer:*** *a*

5) ***correct answer:*** *a*

6) ***correct answer:*** *c*

7) ***correct answer:*** *d*

8) ***correct answer:*** *60 mg/day*

9) ***correct answer:*** *multiple follow-up appointments / multiple follow up appointments / follow up appointments*

10) ***correct answer:*** *parent or partner interview / partner or parent interview*

11) ***correct answer:*** *side effects of stimulant medication / stimulant medication*

12) ***correct answer:*** *at least 40-50% / at least 40 - 50% / at least 40 to 50 percent / 40-50% / 40 to 50% / 40 - 50%*

13) ***correct answer:*** *being more spontaneous and adventurous / spontaneous and adventurous*

14) ***correct answer:*** *atomoxetine*

15) ***correct answer:*** *building blocks*

16) ***correct answer:*** *longstanding / underlying*

17) ***correct answer:*** *longer-acting formulations of methylphenidate / longer acting formulations of methylphenidate / longer-acting formulations / longer acting formulations*

18) ***correct answer:*** *comorbid disorders*

19) ***correct answer:*** *blood pressure*

20) ***correct answer:*** *careful titration / titration*

E2 Language Reading Part B.3

1. According to the extract, to prevent the spread of infection, emergency department isolation rooms

   should be placed away from the main entry doors.

   are more numerous than those of other departments.

   ought to be situated near where people enter the unit.

DESIGN PRINCIPLES FOR ISOLATION ROOMS

The aim of environmental control in an isolation room is to control the airflow, thereby reducing the number of airborne infectious particles that may infect others within the environment.
This is achieved by:

- controlling the quality and quantity of intake and exhaust air;
- diluting infectious particles in large volumes of air;
- maintaining differential air pressures between adjacent areas; and
- designing patterns of airflow for particular clinical purposes.

The location and design of isolation rooms within a particular department or inpatient unit should ideally enable their separation from the rest of the unit. Multiple isolation rooms should be clustered and located away from the main entrance of the unit. An exception is an emergency department where it is recommended that designated isolation rooms be located near the entry to prevent spread of possible airborne infection throughout the unit.

2. What do staff need to be conscious of when working in Anterooms?

   Keeping used and unused medical clothing apart.

   Ensuring the ambient pressure in the room is a minimum 15 Pascal.

   Keeping the door closed at all times.

ANTEROOMS

Anterooms allow staff and visitors to change into, and dispose of, personal protective equipment used on entering and leaving rooms when caring for infectious patients. Clean and dirty workflows within this space should be considered so that separation is possible. Anterooms increase the effectiveness of isolation rooms by minimising the potential escape of airborne nuclei into a corridor area when the door is opened.

For Class N isolation rooms the pressure in the anteroom is lower than the adjacent ambient (corridor) pressure, and positive with respect to the isolation room. The pressure differential

between rooms should be not less than 15 Pascal.

Anterooms are provided for Class N isolation rooms in intensive care units, emergency departments, birthing units, infectious diseases units, and for an agreed number of patient bedrooms within inpatient units accommodating patients with respiratory conditions.

3. What is the basic principle of flexible design?

    Creating systems which match current policy and can adjust to other possible guidelines.

    Designing healthcare facilities which strictly adhere to current policy.

    Changing healthcare policies regularly to match changes in the marketplace.

FLEXIBLE DESIGN

In healthcare, operational policies change frequently. The average cycle may be as little as five years. This may be the result of management change, government policy, and turnover of key staff or change in the marketplace. By contrast, major healthcare facilities are typically designed for 30 years, but may remain in use for more than 50 years. If a major hospital is designed very tightly around the operational policies of the day, or the opinion of a few individuals, who may leave at any time, then a significant investment may be at risk of early obsolescence. Flexible design refers to planning models that can not only adequately respond to contemporary operational policies but also have the inherent flexibility to adapt to a range of alternative, proven and forward-looking policies.

4. When prescribing antibiotics for a human bite, what should the medical professional remember?

    Not all patients should be given antibiotics given the nominal infection risk.

    The bacterium Streptococcus spp. is the most common in bite patients.

    Eikenella corrodens is not susceptible to several antibiotics often used for skin infections.

Human bites

Human bite injuries comprise clenched-fist injuries, sustained when a closed fist strikes the teeth of another person, and occlusive bites, resulting from direct closure of teeth on tissue. Clenched-fist injuries are more common than occlusive bites, particularly in men, with most human bites occurring on the hands. Human bites result in a greater infection and complication rate than animal bites. Cultures of human bites are typically polymicrobial. Mixed aerobic and anaerobic organisms are common, with the most common isolates including Streptococcus spp. and Eikenella corrodens, which occurs in up to one-third of isolates.

Some authors suggest that all patients with human bites should be commenced on antibiotic prophylaxis, given the high risk of infection. The choice of antibiotic therapy should cover E. corrodens, which is resistant to first-generation cephalosporins (such as cefalexin), flucloxacillin and clindamycin, antibiotics that are often used for skin and soft tissue infections.

5. The extract informs us that a model of care

    is only implemented at certain times and places.

    should include its own application and assessment.

    involves the development of a project management tool.

What is a MoC?

A "Model of Care" broadly defines the way health services are delivered. It outlines best practice care and services for a person, population group or patient cohort as they progress through the stages of a condition, injury or event. It aims to ensure people get the right care, at the right time, by the right team and in the right place.

When designing a new MoC, the aim is to bring about improvements in service delivery through effecting change. As such creating a MoC must be considered as a change management process. Development of a new MoC does not finish when the model is defined, it must also encompass implementation and evaluation of the model and the change management needed to make that happen. Developing a MoC is a project and as such should follow a project management methodology.

6. What is the basic difference between delegation, referral, and handover?

    How many practitioners are involved in each part of the process.

    How much authority is attributed to each practitioner.

    How long each of the processes take a practitioner to complete.

4.3 Delegation, referral and handover

Delegation involves one practitioner asking another person or member of staff to provide care on behalf of the delegating practitioner while that practitioner retains overall responsibility for the care of the patient or client.
Referral involves one practitioner sending a patient or client to obtain an opinion or treatment from another practitioner. Referral usually involves the transfer in part of responsibility for the care of the patient or client, usually for a defined time and a particular purpose, such as care that is outside the referring practitioner's expertise or scope of practice.
Handover is the process of transferring all responsibility to another practitioner.

Answers

1. **C**
2. **A**
3. **A**

4. **C**
5. **B**
6. **B**

Text 1: Personal devices and hearing loss

Most of us have experienced walking past someone and being able to hear every sound coming from their headphones. If you've ever wondered whether this could be damaging their hearing, the answer is yes. In the past, noise-induced hearing loss typically affected industrial workers, due to prolonged exposure to excessive levels of noise with limited or non-existent protective equipment. There are now strict limits on occupational noise exposure and many medico-legal claims have been filed as a result of regulation. The **ubiquitous** use of personal music players has, however, radically increased our recreational noise exposure, and research suggests there may be some cause for concern.

The problem is not just limited to children and teenagers either; adults listen to loud music too. According to the World Health Organization, hearing loss is already one of the leading causes of disability in adults globally, and noise-induced hearing loss is its second-largest cause. In Australia, hearing loss is a big public health issue, affecting one in six people and costing taxpayers over A$12 billion annually for diagnosis, treatment, and rehabilitation.

When sounds enter our ear, they set in motion tiny frequency-specific hair cells within the cochlea, our hearing organ, which initiate the neural impulses which are perceived by us as sounds. Exposure to high levels of noise causes excessive wear and tear, leading to **their** damage or destruction. The process is usually gradual and progressive; as our cochlea struggles to pick up sounds from the damaged frequencies we begin to notice poorer hearing. Unfortunately, once the hair cells are gone, they don't grow back.

A number of US studies have shown the prevalence of noise-induced hearing loss in teenagers is increasing, and reports from Australia have suggested there's an increased prevalence of noise-induced hearing loss in young adults who use personal music players. This is a worrying trend considering the widespread usage of these devices. Even a slight hearing loss can negatively affect a child's language development and academic achievement. This is of significant concern considering some studies have reported a 70% increased risk of hearing loss associated with use of personal music players in primary school-aged children.

Some smartphones and personal music players can reach up to 115 decibels, which is roughly equivalent to the sound of a chainsaw. Generally, 85 decibels and above is considered the level where noise exposure can cause permanent damage. Listening at this level for approximately eight hours is likely to result in permanent hearing loss. What's more, as the volume increases, the amount of time needed to cause permanent damage decreases. At 115 decibels, it can take less than a minute before permanent damage is done to your hearing.

In Australia a number of hearing education campaigns, such as Cheers for Ears, are teaching children and young adults about the damaging effects of excessive noise exposure from their personal music players with some encouraging results. Hopefully, this will lead to more responsible behaviour and prevent future cases of noise-induced hearing loss in young adults. Currently, there are no maximum volume limits for the manufacturers of personal music players in Australia. This is in stark contrast to Europe, where action has been taken after it was estimated that 50 and 100 million Europeans were at risk of noise-induced hearing loss due to personal music players. Since 2009, the European Union has provided guidance to limit both the output and usage time of these devices. Considering the impact of hearing loss on individuals and its cost to society, it's unclear why Australia has not adopted similar guidelines. Some smartphones and music players allow you to set your own maximum

volume limits. Limiting the output to 85 decibels is a great idea if you're a regular user and value preserving your hearing. Taking breaks to avoid continued noise exposure will also help reduce your risk of damaging your hearing.

Losing your hearing at any age will have a huge impact on your life, so you should do what you can to preserve it. Hearing loss has often been referred to as a "silent epidemic", but in this case it is definitely avoidable.

1. The writer suggests that the risks from exposure to excessive industrial noise

   Have become better regulated over time.

   Have increased with the spread of new media devices.

   Were limited or non-existent in the past.

   Are something most people have experienced.

2. The word **'ubiquitous'** in paragraph 1 suggests that use of personal media players is

   Getting out of control.

   Radically increasing.

   Extremely common.

   A serious health risk.

3. In the second paragraph, the writer aims to emphasise the

   Impacts of hearing loss on young people.

   Significant global effect of noise related hearing loss.

   WHO's statistical information on hearing loss.

   Huge cost of hearing loss treatment in Australia.

4. What does the word **'their'** in paragraph 3 refer to?

   Smart phones and music players

   People with hearing loss

   Neural impulses entering our ear

   Tiny hair cells in the ear

5. What does the research mentioned in paragraph four show?

   A higher prevalence of personal music devices in primary schools.

The negative impact of device related hearing loss on academic and linguistic skills.

An increasing number of teens and young adults suffering noise related hearing loss.

The widespread trend for increased use of personal music devices.

6. In paragraph 5, the writer suggests that

Chainsaws and smartphones are negatively impacting the public's hearing

Listening to music on a smartphone will damage your hearing.

Smartphones are designed to play music at dangerously high volumes.

More rules should be in place to control how loud smartphones can go.

7. Why does the writer mention the Australian education programs in paragraph 6?

To encourage schools to adopt the Cheers for Ears program.

To suggest that education could lead to safer behaviour in young people

To criticise governments for not educating youths on the danger of excessive noise .

To highlight a successful solution to the issue of hearing loss in young people.

8. What is the writer's attitude to the lack of manufacturing guidelines for music devices in Australia?

There is no clear reason why Australia has not created guidelines.

The implementation of guidelines in Australia is unnecessary.

Guidelines probably won't be created in Australia.

It will be difficult to create guidelines in Australia.

Answers

1. **A**
2. **C**
3. **D**
4. **D**
5. **C**
6. **C**
7. **B**

8. A

E2 Language Reading Part C.3

Text 2: What is herd immunity?

A recent outbreak of chickenpox is a reminder that even in countries where immunisation rates are high, children and adults are still at risk of vaccine-preventable diseases. Outbreaks occur from time to time for two main reasons. The first is that vaccines don't always provide complete protection against disease and, over time, vaccine protection tends to diminish. The second is that not everyone in the population is vaccinated. This can be for medical reasons, by choice, or because of difficulty accessing medical services. When enough unprotected people come together, infections can spread rapidly. This is particularly the case in settings such as schools where large numbers of children spend long periods of time together.

When a high proportion of a community is immune it becomes hard for diseases to spread from person to person. This phenomenon is known as herd immunity. Herd immunity protects people indirectly by reducing their chances of coming into contact with an infection. By decreasing the number of people who are susceptible to infection, vaccination can starve an infectious disease outbreak in the same way that firebreaks can starve a bushfire: by reducing the fuel it needs to keep spreading. If the immune proportion is high enough, outbreaks can be prevented and a disease can even be eliminated from the local environment. Protection of "the herd" is achieved when immunity reaches a value known as the "critical vaccination threshold". This value varies from disease to disease and takes into account how contagious a disease is and how effective the vaccine against it is.

For a disease outbreak to "grow", each infected person needs to pass their disease on to more than one other person, in the same way that we think about population growth more generally. If individuals manage only to "reproduce" themselves once in the infectious process, a full-blown outbreak won't occur. For example, on average someone with influenza infects up to two of the people they come into contact with. If one of those individuals was already fully protected by vaccination, then only one of them could catch the flu. By immunising half of the population, we could ***stop flu in its tracks***.

On the other hand, a person with chickenpox might infect five to ten people if everyone were susceptible. This effectively means that we need to vaccinate around nine out of every ten

people (90% of the population) to prevent outbreaks from occurring. As mentioned earlier, vaccines vary in their ability to prevent infection completely, particularly with the passing of time. Many vaccines require several "booster" doses for this reason. When vaccine protection is not guaranteed, the number of people who need to be vaccinated to achieve herd immunity and prevent an outbreak is higher. Chickenpox vaccine is one such example: infections can occur in people who have been vaccinated. However, such cases are typically less severe than in unimmunised children, with fewer spots and a milder symptom course.

In Australia, overall vaccine coverage rates are high enough to control the spread of many infectious diseases. Coverage shows considerable geographic variation, though, with some communities recording vaccination levels of less than 85%. In these communities, the conditions necessary for herd immunity may not be met. That means localised outbreaks are possible among the unvaccinated and those for whom vaccination did not provide full protection. In the Netherlands, for example, high national measles vaccine uptake was not enough to prevent a very large measles outbreak (more than 2, 600 cases) in orthodox Protestant communities opposed to vaccination.

Australia's National Immunisation Strategy specifically focuses on achieving high vaccine uptake within small geographic areas, rather than just focusing on a national average. Although uptake of chickenpox vaccine in Australia was lower than other infant vaccines, coverage is now comparable.

Media attention has emphasised those who choose not to vaccinate their children due to perceived risks associated with vaccination. However, while the number of registered conscientious objectors to vaccination has increased slightly over time, these account for only a small fraction of children. A recent study found only 16% of incompletely immunised children had a mother who disagreed with vaccination. Other factors associated with under vaccination included low levels of social contact, large family size and not using formal childcare.

Tailoring services to meet the needs of all parents requires a better understanding of how families use health services, and of the barriers that prevent them from immunising. To ensure herd immunity can help protect all children from preventable disease, it's vital to maintain community confidence in vaccination. It's equally important the other barriers that prevent children from being vaccinated are identified, understood and addressed.

1. According to the writer what causes occasional outbreaks of preventable diseases?

   A high prevalence of disease.

   Limited access to vaccination.

   A low prevalence of vaccination.

   Attitudes towards vaccination.

2. Why does the writer mention bushfires in paragraph 2?

   To emphasise the effectiveness of herd immunity.

   To describe a method for eliminating disease.

   To warn of the risks of of vaccination.

To highlight the severity of the flu.

3. The phrase **"stop flu in its tracks"** in paragraph 3 refers to the

   Prevention of flu spreading.

   Eradication of the flu virus.

   Minimisation of flu victims.

   Reduction in severity of flu symptoms.

4. Information in paragraph 4 implies that

   The chickenpox vaccine is highly unreliable.

   Chickenpox is more contagious than the flu.

   Booster vaccines should be given in schools.

   Outbreaks of chickenpox are on the rise.

5. In paragraph 5, the writer emphasises the importance of

   How geographical variation contributes to outbreaks.

   Differences in global vaccination guidelines.

   The influence of religious beliefs on vaccination.

   Enforcing high vaccine coverage rates.

6. Why does the writer mention Australia's National Immunisation Strategy?

   To serve as a counter argument.

   To engage Australian readers.

   To reinforce a previous point.

   To introduce a new topic.

7. The research quoted in paragraph 7 reinforces that

   The media presents vaccination negatively.

   Many factors contribute to under vaccination.

   Parental objections account for most unvaccinated children.

   The number of conscientious objectors has increased over time.

8. In the final paragraph, the writer focuses on

   The importance of widespread faith in vaccination.

   The difficulty of tailoring health services to all parents.

   The identification of barriers to overcoming under vaccination.

   The different kinds of preventable disease that need to be overcome.

Answers

1. **C**
2. **B**
3. **A**
4. **B**
5. **A**
6. **C**
7. **B**
8. **C**

Dengue Fever: Texts

Text A

Dengue: virus, fever and mosquitoes

Dengue fever is a viral disease spread only by certain mosquitoes – mostly Aedes aegypti or "dengue mosquitoes" which are common in tropical areas around the world.

There are four types of the dengue virus that cause dengue fever – Dengue Type 1, 2, 3 and 4. People become immune to a particular type of dengue virus once they've had it, but can still get sick from the other types of dengue if exposed. Catching different types of dengue, even years apart, increases the risk of developing severe dengue. Severe dengue causes bleeding and shock, and can be life threatening.

Dengue mosquitoes only live and breed around humans and buildings, and not in bush or rural areas. They bite during the day – mainly mornings and evenings. Dengue mosquitoes are not born with dengue virus in them, but if one bites a sick person having the virus in their blood, that mosquito can pass it on to another human after about a week. This time gap for the virus to multiply in the mosquito means that only elderly female mosquitoes transmit dengue fever. The mosquitoes remain infectious for life, and can infect several people. Dengue does not spread directly from person to person.

Text B

Signs and Symptoms

Classic dengue fever, or "break bone fever," is characterised by acute onset of high fever 3–14 days after the bite of an infected mosquito. Symptoms include frontal headache, retro-orbital pain, myalgias, arthralgias, hemorrhagic manifestations, rash, and low white blood cell count. The patient also may complain of weight loss and nausea. Acute symptoms, when present, usually last about 1 week, but weakness, malaise, and weight loss may persist for several weeks. A high proportion of dengue infections produce no symptoms or minimal symptoms, especially in children and those with no previous history of having a dengue infection.

Text C

Steps to take when seeing a suspected case of dengue fever

Step 1: Notify your nearest Public Health Unit immediately upon clinical suspicion.

Step 2: Take a comprehensive travel history and determine whether the case was acquired overseas or locally.

Step 3: Note the date of onset of symptoms to identify the correct diagnostic test, as suitable laboratory tests depend on when the blood sample is collected during the illness.

• Another useful test is full blood count. Cases often have leukopenia and/or thrombocytopenia.

The table below shows which test to order at which stage of illness:

Test Type	PCR	NS1 ELISA	IgM	IgG
Days after onset of symptoms	0-5 days	0-9 days	From day 5 onwards	From day 8 onwards

Step 4: Provide personal protection advice.

• The patient should stay in screened accommodation and have someone stay home to look after them.

• The patient should use personal insect repellent particularly during daylight hours to avoid mosquito bites.

• All household members should use personal insect repellent during daylight hours.

• Advise family members or associates of the case who develop a fever to present immediately for diagnosis.

Text D

Prior to discharge:

• Tell patients to drink plenty of fluids and get plenty of rest.

• Tell patients to take antipyretics to control their temperature. Children with dengue are at risk for febrile seizures during the febrile phase of illness.

• Warn patients to avoid aspirin and anti-inflammatory medications because they increase the risk of haemorrhage.

• Monitor your patients' hydration status during the febrile phase of illness. Educate patients and parents about the signs of dehydration and have them monitor their urine output.

• Assess hemodynamic status frequently by checking the patient's heart rate, capillary refill, pulse pressure, blood pressure, and urine output. If patients cannot tolerate fluids orally, they may need IV fluids.

• Perform hemodynamic assessments, baseline hematocrit testing, and platelet counts.

• Continue to monitor your patients closely during defervescence. The critical phase of dengue begins with defervescence and lasts 24–48 hours.

END OF PART A

Part A:Questions

[redacted]

Type all your answers in the **Answer box** provided.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

Part A

TIME: 15 minutes

- Look at the four texts, **A-D**, in the separate **Reading Part A: Text Booklet**.
- For each question, **1-20**, look through the texts **A-D**, to find the relevant information.
- Type your answers in the **Answer box** provided.
- Answer all the questions within the 15-minute time limit.
- Your answers should be spelled correctly.

- **Questions 1 - 7**
- For each of the questions**, 1 - 7,** decide which text **(A, B, C, or D)** the information comes from. You may use any letter more than once**.**
-
- In which text can you find information about

1. In which text can you find information about the different types of dengue virus?
2. In which text can you find information about how fever presents in patients?
3. In which text can you find information about how dengue fever is transmitted?
4. In which text can you find information about the stages at which to conduct tests for dengue fever?
5. In which text can you find information about monitoring and assessing a patient's condition?
6. In which text can you find information about what advice to give patients to avoid mosquito bites?
7. In which text can you find information about advice for patients regarding medication?

Questions 8 - 14

Complete each of the sentences, **8 - 14**, with a word or short phrase from one of the texts.

Each answer may include words, numbers, or both.

8. How long after being bitten by an infected mosquito does high fever occur?
9. What might patients with dengue fever complain of?
10. Which test should only be ordered 5 days after symptoms appear?
11. What other test is also useful when checking for dengue fever?
12. Who is at risk of seizures during the febrile stage of dengue?
13. What takes places in the most lethal cases of dengue?
14. How long does the most serious stage of dengue last?

Questions 15 - 20

Answer each of the questions, **15 - 20**, with a word or short phrase from one of the texts.

Each answer may include words, numbers or both.

15. Dengue fever does not spread _____

16. In many _____ dengue infections cause almost no symptoms.

17. Within three days of symptoms beginning a PCR or _____ can be ordered.

18. To avoid haemorrhage patients mustn't take anti-inflammatory medications or _____.

19. Advise patients be cared for by someone at home in _____ accommodation.

20. Patients must be made aware of the need to check their _____.

Part B

[redacted]

1. Which type of hazard does the workplace extract relate to?

Select one:

- A. Chemical agents.
- B. Biological agents.
- C. Physical agents.

Extract from Workplace Policy Document: Hazard Assessment
Hazards to Look for When Inspecting Hospitals Examples of typical hazards include bacteria, viruses, fungi, and other living organisms that can cause acute and chronic infections by entering the body through ingestion, inhalation or breaks in the skin. They also include exposure to blood or other body fluids or to clients or patients with infectious diseases (e.g., MRSA, staph, HIV, HBV, HCV, influenza, tuberculosis). Hospital workers can be exposed to blood borne pathogens from blood and other potentially infectious materials if not following universal precautions.

2. The policy extract is explaining

Select one:

- A. why to make a LAM submission.
- B. how to make a LAM submission.
- C. where to make a LAM submission.

Requesting a change to the list of approved medicines (LAM)
Any hospital staff member can request a change to the list of approved medicines (LAM). It is expected that applications for changes will include input from a senior prescriber. Changes should be requested by completing either the standard or, in limited circumstances, a minor submission form. A standard submission form is available online or from your local pharmacy department. A minor submission form can be obtained through contacting the relevant Secretariat. Staff are also encouraged to flag potential issues regarding the use of medicines or pharmaceuticals in writing, with evidence attached. Requests from pharmaceutical manufacturers or their agents will not be accepted.

3.

3.What point do the guidelines make about leadership for doctors?

Select one:

- A. The role of a doctor should go beyond practising medicine.
- B. Doctors are the most important clinician in a health care setting.
- C. There could be harsh penalties for doctors who don't improve their skills.

Guidelines: Leadership and Management for all Doctors

This guidance sets out the wider management and leadership responsibilities of doctors in the workplace. The principles in this guidance apply to all doctors, whether they work directly with patients or have a formal management role.

Being a good doctor means more than simply being a good clinician. In their day-to-day role doctors can provide leadership to their colleagues and vision for the organisations in which they work and for the profession as a whole. However, unless doctors are willing to contribute to improving the quality of services and to speak up when things are wrong, patient care is likely to suffer.

You must be prepared to explain and justify your decisions and actions. Serious or persistent failure to follow this guidance will put your registration, and so your right to practise medicine, at risk.

4. The purpose of this memo to staff is to

Select one:

- A. state the potential risks to patients who smoke electronic cigarettes.
- B. provide information about the substances used in electronic cigarettes.
- C. advise that no position has yet been reached about electronic cigarettes.

Memo to staff: Electronic Cigarettes

Electronic cigarettes (e-cigarettes) are battery operated devices that heat a liquid (called 'e-liquid') to produce a vapour that users inhale. Although the composition of this liquid varies, it typically contains a range of chemicals, including solvents and flavouring agents, and may or may not contain nicotine.

Electronic cigarettes are a topic of contention among public health and tobacco-control advocates, some of whom argue they don't pose the same dangers to smokers as traditional cigarettes. Others, however, argue that electronic cigarettes should not be promoted as a lower threat option for smokers when their long-term safety is unknown.

There is currently insufficient evidence to support claims that electronic cigarettes are safe, and further research is needed to determine their long-term safety. As a result, there will be no change to hospital policy at this time.

5. As a result of an update in favour of patient-centeredness what is going to happen?

Select one:

- A. There will be a greater focus on hospital wait times.
- B. More staff will be required to undertake training.
- C. New standards of practice will be developed.

Patient-centred Interdisciplinary Goal Setting in Rehabilitation Services

Although goal setting is fundamental to rehabilitation practice and optimal patient outcomes, it typically varies in the practices taught across different health professions, and the preparedness of rehabilitation clinicians to undertake it. Patient-centeredness has been shown to improve patient care experiences and create value for public services through increasing the quality and safety of health care.

Hospital pressures to facilitate discharge and decrease length of stay have been identified by staff as barriers to implementing patient-centred goal setting practice. This has resulted in goal setting often being hospital driven rather than patient driven. Furthermore, staff have recently expressed a lack of strategies or tools to implement patient-centred principles in care processes such as goal setting. There is therefore a need to enable rehabilitation services to improve goal setting models and patient engagement in health care related goals and decisions.

6. According to the procedure, when inserting a catheter clinicians should

Select one:

- A. only use a catheter once.
- B. carefully follow all guidelines.
- C. ensure the patient isn't left alone.

Catheter Insertion Procedure

Where possible, use a two clinician buddy system to carry out the procedure. The patient's ethical, religious and cultural beliefs and personal history should be considered when appointing clinicians to perform a catheterisation. A chaperone may also be required to observe the procedure.

It is recommended that the patient's genital area be washed with soap and water prior to catheterisation. If unable to insert a catheter after two attempts (includes changing to a different catheter size), seek further assistance from a senior clinician. A new catheter should be used for each attempt.

Testing of the catheter balloon prior to insertion may compromise the integrity of some catheters. Refer to manufacturer's instructions to see if pre-insertion testing is required.

Part C

Text ■ Restless Leg Syndrome

For questions **1** to **8**, choose the answer (**A, B, C** or **D**) which you think fits best according to the text.

Paragraph 1

The relatively common neurologic movement disorder known as restless legs syndrome (RLS) is poorly understood. Patients describe uncomfortable sensations in their legs that often are worse at night, having a huge impact on their ability to sleep and overall quality of life. Experts now speculate that patients with RLS, like those with unrelieved chronic pain, might feel hopeless, leading to suicidal thoughts and actions. The risk correlates with history of depression, and is independent of the severity of restless leg symptoms and demographic factors.

1. The writer suggests that restless legs syndrome (RLS)

Select one:

- A. is impossible to cure.
- B. could lead to depression.
- C. doesn’t occur during the day.
- D. may relate to pain management.

Paragraph 2

Although evaluating restless legs syndrome and finding effective treatments is challenging, a recent study suggests that it's important to assess not only the impact of RLS on the patient's life, but also the presence of suicidal thoughts. People with severe RLS are more likely to plan and attempt suicide than people without it, even after controlling for depression, according to new research. "Lifetime suicidal ideation and attempts are very prevalent among people with restless legs syndrome and seem to be independent of demographic factors and depression and seem to be associated with severity of restless legs," said Brian Koo, MD, director of the Yale Center for Restless Legs Syndrome, Yale University, New Haven, Connecticut.

2. Dr Brian Koo suggests it’s important for clinicians to treat any suicidal thoughts because

Select one:

- A. older people are more likely to suffer from RLS.
- B. the effects of RLS can be better identified.
- C. it makes managing RLS much easier.
- D. RLS is a mental health condition.

Paragraph 3

Until now, RLS and its potential relationship with suicide had not been studied in any depth. The Yale team investigated the frequency of lifetime suicidal behaviour in 198 patients with severe RLS and 164 controls. All participants completed the Suicidal Behaviour Questionnaire-revised (SBQ-R) and the Brief Lifetime Depression Scale. RLS and controls were similar in age (mean age, 51), income, and gender. Compared with controls, patients with RLS were more often white (96% vs 88%), less often had higher education (84% vs 96%), were more often married (72% vs 60%), and were less often employed or retired (80% vs 90%). Significantly more patients with RLS than controls were at high suicide risk (SBQ-R score ≥ 7) and had lifetime suicidal thoughts or behaviour, independent of depression history.

3. What did the Yale team learn from their investigations?

Select one:

- A. Some people in the control group had previously suffered from RLS.
- B. The likelihood of someone developing RLS depends on various factors.
- C. Answers to the questionnaires didn’t provide a lot of useful data about RLS.

D. A person with RLS is more likely to attempt suicide than someone without it.

Paragraph 4

"Mood and anxiety disorders are highly comorbid in RLS patients," noted John W. Winkelman, MD, PhD, from Harvard Medical School and Massachusetts General Hospital, Boston. "My feeling is that the suicidal ideation, or even plan or intent, and even some who have followed through, is the same thing you see in patients with chronic pain. In many respects, RLS is a chronic pain disorder. And if you have chronic pain, for which you feel there is no appropriate treatment and your physician may not understand what you have, or may not know how to treat it appropriately, it can lead you to feeling hopeless, and I think pain and hopelessness can lead to those kinds of thoughts," Winkelman said.

4. The expression '**followed through**' refers to

Select one:

A. RLS patients who have attempted suicide.

B. the relationship between RLS and pain.

C. a time when RLS has been resolved.

D. management of RLS by the doctor.

5. John Winkelman's comments in the fourth paragraph show his

Select one:

A. concern that a lot of doctors have never heard of RLS.

B. belief that RLS relates to many other health conditions.

C. frustration that too many people with RLS commit suicide.

D. understanding of the situation facing a lot of RLS sufferers

Paragraph 5

One such case is Lisa, a 45-year-old married woman who came to see a psychiatrist initially for depressive symptoms. During the initial evaluation, she complained of difficulty in falling asleep and other depressive symptoms such as low mood, difficulty with concentration, poor appetite, and low energy along with daytime fatigue. Depression was diagnosed. A selective serotonin reuptake inhibitor (SSRI) was prescribed on an as-needed basis, and the patient was advised to take a nightly dose of diphenhydramine to help her sleep. Three days later—after staying up nearly all night—Lisa called her doctor in despair and complained of worsening insomnia. On more detailed questioning about the insomnia, Lisa revealed that for the past 2 years, she has experienced leg discomfort when she gets into bed. She is so uncomfortable that she needs to walk or ride on her exercise bike past 2 or 3 am until the discomfort subsides. While not painful, this leg discomfort sometimes prevents her from relaxing and watching television because she just "has to move" her legs.

6. The case involving Lisa highlights that

Select one:

- A. some patients don't follow the recommended advice for RLS.
- B. regular exercise is recommended for people with RLS.
- C. sleep problems and exhaustion could indicate RLS.
- D. medication is important in the treatment of RLS.

Paragraph 6

Lisa describes a deep uncomfortable sensation that feels like "bugs crawling in her legs." She also reveals that her mother used to suffer from similar night-time leg restlessness. Lisa's leg discomfort became more intense and was lasting most of the night. After secondary causes of RLS, such as iron deficiency anaemia, pregnancy, uraemia, and neuropathy were ruled out, SSRI and diphenhydramine therapy were stopped. Low-dose dopamine agonist therapy was started, after which the symptoms subsided. However, despite resolution of the RLS symptoms, her depressive symptoms continued. This only serves to further reinforce the need to investigate and treat any associated mood or anxiety disorders in conjunction with RLS symptoms.

7. In the final paragraph, the writer suggests Lisa's treatment was changed because

Select one:

- A. a new diagnosis was made.
- B. she no longer had depression.
- C. SSRI medication wasn't working for her.
- D. she developed a range of new symptoms.

8.What does the word '**this**' in the final paragraph refer to?

Select one:

- A. Low-dose dopamine agonist therapy.
- B. The differences between therapies.
- C. The end of her RLS symptoms.
- D. Lisa's unresolved depression.

Text: Statins - How safe are they?

For questions **1** to **8**, choose the answer (**A, B, C** or **D**) which you think fits best according to the text.

Paragraph 1

Heart disease is the leading cause of death in the U.S and statins are a commonly prescribed medicine that helps to lower harmful levels of LDL cholesterol in the blood and mitigate the risks of cardiovascular disease, including heart attack and stroke. Trials have consistently demonstrated a clear correlation between reducing LDL cholesterol with statins and a decrease in cardiovascular risk. So it may appear puzzling that uncertainty over statins still remains.

1. The writer suggests that uncertainty over the use of statins is puzzling because

Select one:

- A. no other medication is used as often to treat cardiovascular disease.
- B. heart disease kills large numbers of people in the United States.
- C. extensive studies have been conducted about their use.
- D. they are so effective in lowering LDL cholesterol.

Paragraph 2

As the body of evidence evaluating statins has expanded, so too have the indications for the drug. Guidelines released in 2013 by the American College of Cardiology (ACC) and the American Heart Association (AHA) recommended that statin therapy might be beneficial for people with cardiovascular disease, people who have high LDL cholesterol levels, people aged 40 to 75 years with diabetes and high LDL levels and people aged 40 to 75 years without diabetes, but with high LDL cholesterol levels and a predicted 10-year risk of cardiovascular disease of 7.5 percent or higher. However, experts questioned the 2013 guidelines, arguing that a 7.5 percent threshold seemed too low.

2. In the second paragraph, what do we learn about the guidelines released in 2013?

Select one:

- A. They were seen as worse than the previous guidelines.
- B. They recommended the use of statins for anyone with high LDL levels.
- C. They contained a lot of advice that health professionals didn't agree with.
- D. They suggested a connection between heart disease and other conditions.

Paragraph 3

In 2015, two research teams examined the 7.5 percent threshold and published their findings. The first paper, led by Dr. Udo Hoffmann at Massachusetts General Hospital and Harvard Medical School - both in Boston - found that compared with guidelines published in 2004, the 2013 guidelines were more accurate at identifying individuals at a greater risk of cardiovascular disease. They estimated that by adopting the 2013 guidelines, between 41,000 and 63,000 cardiovascular events would be prevented over 10 years compared with previous guidelines. The second paper, led by Drs. Ankur Pandya and Thomas A. Gaziano at the Harvard T.H. Chan School of Public Health - also in Boston - assessed the cost-effectiveness of the 10-year cardiovascular disease threshold. The researchers concluded that the risk threshold of 7.5 percent or higher had an acceptable cost-effectiveness profile.

3. The research papers written in 2015 concluded that the 7.5 percent threshold would

Select one:

- A. focus more on patient health than the previous guidelines.
- B. result in lower treatment costs for most patients.
- C. reduce the amount of cardiovascular disease.
- D. take many years to implement.

Paragraph 4

As a result of the expansion of the groups reported to benefit from statins, suspicions have been raised about the pharmaceutical industry and of the prescribing healthcare professionals.
Alarm bells started ringing that people were being overmedicated and put at risk of adverse effects. Statins are generally considered to be safe and well tolerated. However, as with any medication, statins may have negative effects in some people. "We know that statins can prevent a significant number of heart attacks and strokes. We know there is a small increase in the risk of diabetes, and at high doses there is a very small increase in myopathy, but overall the benefits greatly outweigh the harms," says Peter Sever, professor of clinical pharmacology and therapeutics at Imperial College London. "Widespread claims of high rates of statin intolerance still prevent too many people from taking an affordable, safe, and potentially life-saving medication."

4. The writer uses the phrase **'alarm bells started ringing'** to indicate

Select one:

- A. some health professionals have been overprescribing statins.
- B. the numbers of people taking statins has grown too quickly.
- C. there are too many risks associated with taking statins.
- D. research into the use of statins has cost too much.

5. What concerns does Peter Sever have about statins in the fourth paragraph?

Select one:

- A. They aren't being promoted as widely as they should be.
- B. They are linked to several other health conditions.
- C. They are too expensive for some patients.
- D. They aren't being used enough.

Paragraph 5

Some people, however, believe heart disease is better treated by other means, such as diet. A study found those who had a diet rich in vegetables, nuts, fish and oils, such as a Mediterranean-style diet were a third less likely to die early, compared with those who ate larger quantities of red meat, such as beef, and butter. Sir David Nicholson, former chief executive of the National Health Service (NHS) in the UK, entered the debate over statins when he said he had stopped taking them as part of his medication for diabetes. "If a lifestyle change works then why would you take the statin? The trouble is that they give you a statin straightaway, so you don't know what is working," he said.

Paragraph 6

While a heart-healthy diet, regular physical activity, and maintaining a healthy weight are all components that may help to reduce cholesterol and lower the risk of heart disease and stroke, certain factors are unable to be influenced - such as genetics. In some people, lifestyle changes alone are not enough to lower cholesterol. According to a study published in the Journal of the American Medical Association, from 1969 to 2013, deaths from heart disease fell by 68 percent, and there were 77 percent fewer deaths from stroke. There may be a link between the rise in statin use and the fall of deaths connected to cardiovascular disease. However, the progress made could be attributed to the "cumulative effect of better prevention, diagnosis, and treatment," says Wayne D. Rosamond, Ph.D., professor of epidemiology at the University of North Carolina in Chapel Hill.

6. Sir David Nicholson's comments show that he believes statins

Select one:

- A. should only be prescribed after other options have been tried.
- B. aren't as effective as diet in improving a person's health.
- C. only work after you have been taking them for a while.

D. don’t work as an effective treatment for diabetes.

7. In the sixth paragraph, Wayne D. Rosamond attributes a reduction in deaths from heart attack and stroke to

Select one:

A. a combination of different factors that work together.

B. the rise in medications that treat heart disease.

C. a person’s family history and background.

D. improved diet and regular exercise.

Paragraph 7

The mounting research appears to overturn debate around statins and aims to reassure doctors and patients that the risks of not taking statins - heart attack or stroke - far outweigh concerns about side effects associated with the drug. Serious side effects are rare, and study authors seem to agree that the substantial proven benefits of statins have been compromised by "serious misrepresentations of the evidence for its safety."

8. The benefits of statins are described as having been ‘**compromised**’ because

Select one:

A. their benefits are too few in number.

B. a lot more research needs to be done.

C. there is still a lot of debate around their use.

D. too many lies have been told about their effects.

Answer key Part A

1. A
2. B
3. A
4. C
5. D
6. C
7. D
8. 3-14 days
9. weight loss and nausea
10. the IgM test
11. full blood count
12. children
13. bleeding and shock
14. 24 - 48 hours
15. directly
16. Children
17. NS1 ELISA test
18. Aspirin
19. Screened
20. urine ouput

part B

1. Biological agents.
2. how to make a LAM submission.
3. The role of a doctor should go beyond practising medicine.
4. advise that no position has yet been reached about electronic cigarettes.
5. New standards of practice will be developed.
6. only use a catheter once.

Part C text 2

1. could lead to depression.
2. it makes managing RLS much easier.
3. A person with RLS is more likely to attempt suicide than someone without it.
4. RLS patients who have attempted suicide.
5. understanding of the situation facing a lot of RLS sufferers.
6. sleep problems and exhaustion could indicate RLS.
7. a new diagnosis was made.
8. Lisa's unresolved depression.

Part C statins

1. they are so effective in lowering LDL cholesterol.
2. They recommended the use of statins for anyone with high LDL levels.
3. reduce the amount of cardiovascular disease.

4. some health professionals have been overprescribing statins.
5. They aren't being used enough.
6. should only be prescribed after other options have been tried.
7. a combination of different factors that work together.
8. too many lies have been told about their effects.

KAPLAN READING TEST

Practice set 1

Part A

TIME: 15 minutes

Anaemia Texts

Text A

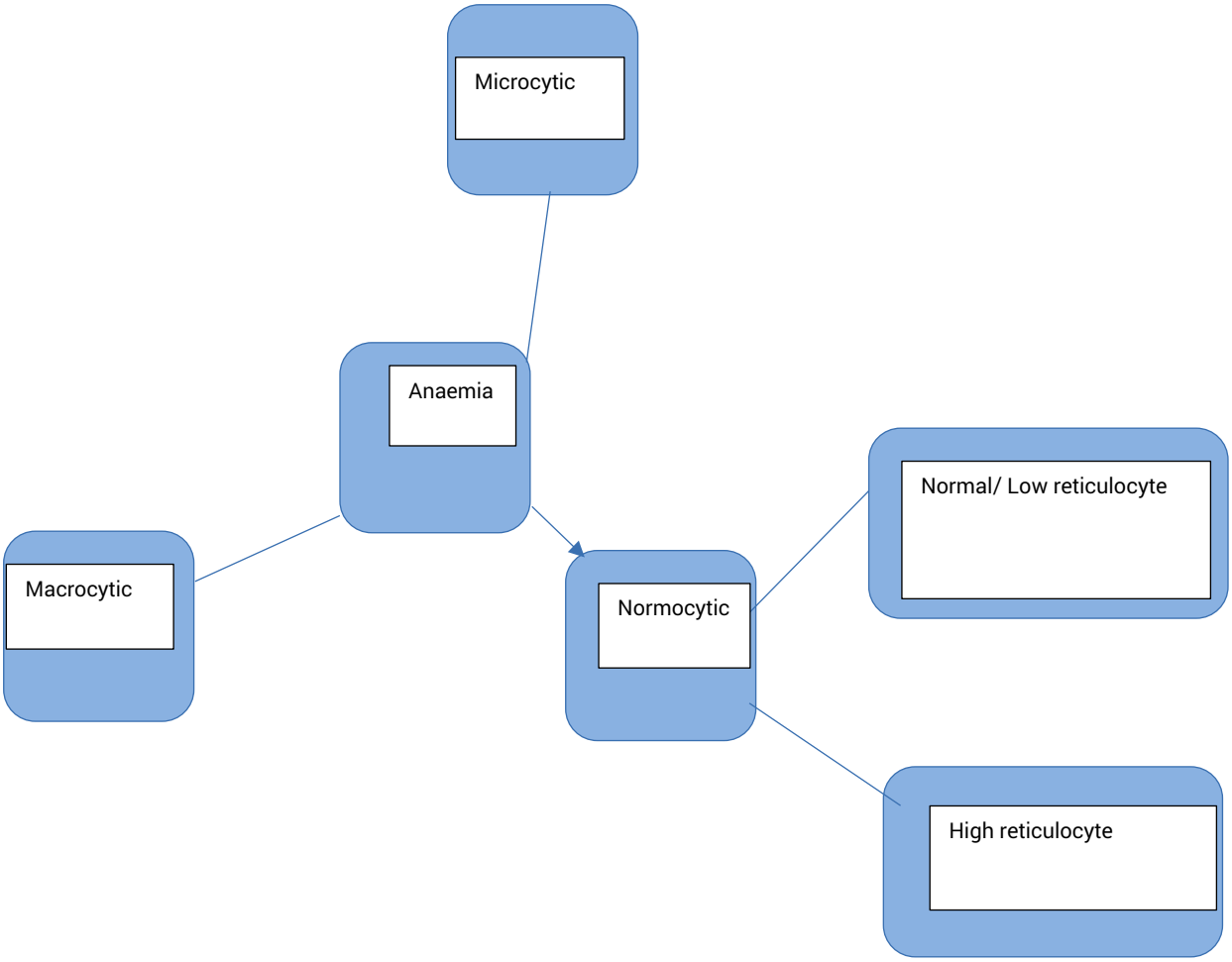
Anaemia is defined as an overall decrease in red blood cell mass. There are many varying causes of anaemia, which all present with some general symptoms. Anaemia results in a lack of red blood cells in the blood. Because it is the haemoglobin in red blood cells that carries oxygen from the lungs to the rest of the body, a decrease in red blood cells results in less oxygen going into the tissues. This causes a state known as hypoxia, or reduced oxygen in body tissues.

The common symptoms of all anaemias are those of hypoxia:

- Weakness, fatigue, difficult or laboured breathing
- Pale skin
- Headache and light-headedness
- Chest pain (if the patient already has a disease of the arteries supplying the heart)

Text B

There are many classification systems to differentiate anaemias. The most commonly used is based on the size of the red blood cell. Anaemias with red blood cells that are smaller than normal are known as microcytic anaemias. If the anaemia has normally sized red blood cells, it is referred to as a normocytic anaemia. Finally, if the red blood cells are too big, it is known as a macrocytic anaemia. Normocytic anaemias are further broken up into whether or not there is an increased number of young red blood cells (a.k.a. reticulocytes), which is an indication if the bone marrow is working properly—for example, if the red blood cells are being destroyed (haemolysis), there should be higher reticulocytes because there is no effect on the bone marrow's ability to produce new cells.


Microcytic
Anaemia
Macrocytic
Normocytic
Normal/ Low reticulocyte
High reticulocyte

Text C

While there are many different causes of anaemia, laboratory studies and unique features of the patient can be used to help differentiate between various aetiologies.

Laboratory studies used to diagnose anaemia include:

- Haemoglobin (Hb)—a measure of the protein that transports oxygen in the red blood cell
- Haematocrit (Hct)—a measure of the percentage of red blood cells in the blood
- Red blood cell amount (erythrocyte count)—a measure of the number of red blood cells in the blood

A general diagnosis of anaemia can be determined by the following values:

- Haemoglobin level

  Males: less than 13.5 g/dL

  Females: less than 12.5 g/dL (women have a generally lower haemoglobin because of blood loss during the monthly menstrual cycle)

- Haematocrit

  Males: less than 45% red blood cells

  Females: less than 37% red blood cells (women have a generally lower haematocrit because of blood loss during the monthly menstrual cycle)

- Red blood cell amount

  Male: less than 4.7 million cells/mL

  Female: less than 4.2 million cells/mL (women have a generally lower red blood cell amount because of blood loss during the monthly menstrual cycle)

While these laboratory tests are good estimates of the red blood cell mass, they are not perfect. Red blood cell mass is very difficult to measure, and therefore these laboratory tests are used together to assess whether or not someone has anaemia.

Text D

The treatment of anaemia depends heavily on the type of anaemia that the patient is experiencing. However, there are several overarching goals of treatment. If possible, treat the underlying cause of the red blood cell loss. For example, if the patient has anaemia because of blood loss, give a blood transfusion.

Identify and treat any complications that have occurred because of the anaemia. Educate the patient on how to manage their anaemia. For example, a patient with anaemia because of iron deficiency can supplement their treatment with iron rich foods, such as leafy green vegetables. Alternatively, a patient with anaemia caused by vitamin deficiency should be advised to increase their intake of folic acid and B-12. Note that patients who follow vegetarian or vegan diets may struggle to meet B-12 requirements, so eating fortified foods and using supplements should be advised.

Part A

Question paper

- Look at the four texts, A – D
- For each question, 1 – 20, look through the texts, A – D, to find the relevant information.
- Write your answers in the spaces provided in this Question Paper.
- Answer all the questions within the 15-minute time limit.

Questions 1 – 6

For each question below, decide which text (A, B, C or D) the information comes from. You may use any letter more than once. In which text can you find information about . . .

1. treating patients with anaemia?
2. the symptoms of hypoxia?
3. methods used to identify anaemic patients?
4. the different types of anaemia?
5. the levels of haemoglobin in a woman with anaemia?
6. how red blood cell size affects anaemia?

Questions 7 – 14

Answer the questions below. For each answer, use a word or short phrase from the text. Each answer may include words, numbers or both.

7. What should vegan patients with vitamin deficiency anaemia be encouraged to add to their diets?

8. If there is a decreased number of young red blood cells, what type of anaemia is being dealt with?

9. How will a patient's breathing sound when experiencing a significant reduction of oxygen in the body's tissues?

10. A male with anaemia must have less than what percentage of red blood cells?

11. What is an increase in the number of reticulocytes an indication of?

12. What reduces the amount of red blood cells in some patients?

13. What should be treated in anaemic patients, after identifying the cause?

14. How are the different types of anaemia most commonly distinguished?

Questions 15 – 20

Complete the sentences below by using a word or short phrase from the text. Each answer may include words, numbers or both.

15. Anaemia caused by (15) _____ should be treated with a blood transfusion.

16. Patients suffering from hypoxia and chest pain are likely to also have a (16)

17. If (17) is functioning properly, high reticulocyte anaemia is likely to be present.

18. A number of tests may be necessary to diagnose anaemia, due to the difficulties involved in measuring (18)

19. Patients with anaemia caused by (19) should be instructed to adjust their diet.

20. When identifying the type of aetiology, (20) _____ of the patient should be considered, in addition to laboratory studies.

Part A: Answer Keys

1.D

2. A

3. C

4. B

5. C

6. B

7. fortified foods (and supplements)

8. low reticulocytes

9. laboured

10. 45

11. reticulocytes

12. menstrual cycle

13. complications

14. size of the red blood cell

15. blood loss

16. disease of the arteries

17. bone marrow

18. iron deficiency

19. chest pain

20. unique features

Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For questions 1 to 6, choose the answer (A, B or C) which you think fits best according to the text.

Write your answers on the separate Answer Sheet.

1. Why is epinephrine added to Lidocaine injections?

(A) to numb the area

(B) to prolong the effects

(C) to reduce patient discomfort

Extract 1

Preparation of Injection

Lidocaine is a local anesthetic that is often injected subcutaneously before minor medical procedures such as laceration repair, excisional biopsy, and hormone implantation. A key step to prepare for this procedure is clearing a suitable workspace and obtaining any necessary supplies. First, be sure to check with your provider about the concentration and mixture of Lidocaine to be used. Epinephrine is often included to constrict local blood vessels for longer duration, but can increase the risk of causing ischemia in areas with poor blood supply (fingers, ears, toes). Sodium bicarbonate can also be added to avoid pain during injection due to Lidocaine's acidic pH. Be sure to obtain the proper sized needle and syringe, which will be dependent on the location of the injection and the size of the area requiring anesthesia, respectively.

2. The policy document on collateral information offers advice to staff about how to

(A) gather information from colleagues about specific patients.

(B) collect information about patients from their friends and relatives.

(C) inform patients and their carers about recent diagnoses over the phone.

Extract 2

Policy Reminder: Collecting Collateral Information

Collateral information is an important factor in determining appropriate disposition for psychiatric patients in the Emergency Department. Often, patients with psychiatric complaints are unable to accurately or thoroughly describe their medical history, baseline condition, or events leading up to their arrival at the hospital. Thus, it becomes imperative to contact those who might know the patient best or

were in the patient's company prior to their arrival. Contact information can be obtained from the patient themselves, persons accompanying the patient, or the medical record. When initiating contact, confirm the other person's identity before revealing the patient's name or the reason you are speaking with them. If you reach voicemail and the answering machine does not clearly identify the person you are looking for, do not reveal any information about the patient – simply state your name, number, position, and whom you are requesting a callback from.

3. When dealing with patients with symptoms of peripheral arterial disease, staff should

(A) look for signs of swelling in the upper body.

(B) confirm that the patient has a history of poor diet.

(C)identify the cause through physical examination and tests.

Extract 3

Assessing and Managing Peripheral Arterial Disease

Staff should assess patients who have symptoms suggestive of peripheral arterial disease or diabetes with non-healing wounds for the presence of peripheral arterial disease.

- Ask about the presence of intermittent claudication and critical limb ischaemia.
- Examine the lower limbs for evidence of critical limb ischaemia.
- Examine pulses in the lower limbs: femoral, popliteal and feet.
- Measure the ankle brachial pressure index.

Imaging is possible for patients with peripheral arterial disease: duplex ultrasound is the first-line imaging technique. If patients require additional imaging, contrast-enhanced magnetic resonance angiography is used. If this is contraindicated or not possible, use computed tomography angiography instead.

Lifestyle changes are the first-line treatment for peripheral arterial disease, this includes: smoking cessation, better control of diabetes, better management of hypertension, management of high cholesterol, in combination with antiplatelet drugs. Finally, regular exercise has shown to beneficially revascularise tissues in those with claudication.

4. The guidelines on alcohol withdrawal treatment informs healthcare professionals about

(A) determining the quantity of medication required.

(B) reducing the dosage as the symptoms improve.

(C) various types of drugs to prescribe to patients.

Extract 4

Guidelines: Alcohol Withdrawal Treatment

Alcohol withdrawal can present as a life-threatening emergency and requires treatment at a hospital. Providers use algorithms to determine when and how much medication to administer for a safe and optimal outcome. A key component of this assessment is determining the severity of alcohol withdrawal using the Clinical Institute Withdrawal Assessment for Alcohol (CIWAAr). The scale contains 10 subjective and objective items that can be queried and scored in minutes. Symptoms asked about include nausea, vomiting, tremors, sweating, anxiety, agitation, tactile/auditory/visual disturbances, headache, and cognitive dysfunction. Every hospital has different cutoffs for treatment, but as a general rule, treatment with benzodiazepines begin starting at a score 8–10, with higher scoring indicating increasing amount and frequency of medication.

5. The memo is advising staff dealing with agitated patients on how to

(A)identify the cause of the agitation.

(B)avoid adding to the feelings of agitation.

(C)deal with violent behaviour caused by the agitation.

Extract 5

For the attention of all staff:

RE: AGITATED PATIENTS

Agitated patients are a common occurrence in the Emergency Department. There are many reasons for agitation, ranging from medical conditions, substance intoxication, psychiatric illness, and distressing circumstances. While both physical and chemical

restraints are available to providers, these are items of last resort as their use creates significant risk to the patient, staff, and other persons in the area. Verbal de-escalation is a proven, effective technique that can be used to calm a patient down and promote a safe treatment environment. When de-escalating, designate one person to speak for the group. Agitated patients can be easily confused by multiple speakers and a unified message must be presented. Respect personal space to prevent the patient from feeling 'trapped' and maintain sufficient distance to avoid any resultant physical aggression. Remember to introduce yourself and your role on the treatment team to the patient. Use their name and orient them to their surroundings and why they are here in the hospital.

6. The guidelines advise that patients with heart problems

(A) may need to avoid ibuprofen.

(B) should be given lidocaine for pain relief.

(C) must receive a lower dose of acetaminophen.

Extract 6

Extract from Appropriate Treatment for Pain

Pain is one of the most common complaints that will be brought to a physician's attention. This section will cover treatment of mild to moderate pain without the use of opioids. More severe pain may require judicious use of short-acting opioid medications or a consult to pain medicine. For most patients, the first line medications for pain are acetaminophen and ibuprofen. Maximum daily dosage of acetaminophen is suggested to be 4 grams, reduced to under 2 grams for patients with liver issues such as a cirrhosis. Ibuprofen is particularly effective in patients whose pain is caused by inflammation, though caution is urged in elderly patients, patients with diagnosed bleeding issues (especially gastrointestinal bleeds), or any cardiac issues. Maximum daily dosage suggested is 2.4 grams. A combination of acetaminophen and ibuprofen can be used if either one used alone is not sufficient. For more localised pain relief, consider using lidocaine dermal patches over non-broken areas of skin.

Part B : Answer keys

1. B to prolong the effects

2. B collect information about patients from their friends and relatives.

3. C identify the cause through physical examination and tests.

4. A determining the quantity of medication required.

5. C deal with violent behaviour caused by the agitation.

6. A may need to avoid ibuprofen.

Part C

For questions 1 to 16, choose the answer (A, B, C or D) which you think fits best according to the text. Write your answers on the separate answer sheet.

QUESTIONS 1 TO 8

Text 1: Delivering Serious News

Delivering serious news to patients and relatives: it's many healthcare professionals' most dreaded task. Unfortunately, it's not something that can be avoided, and it's something that must be done right. Patients and relatives need our guidance and support, particularly when the prognosis is serious. In this article, we use the phrase 'serious news' or 'life-altering news' rather than choosing a term with negative connotations, such as 'bad news', for example, as it helps to reframe the discussion. If you discuss 'serious news' with a patient, they can decide how to respond, whereas giving a patient 'bad news', may prevent them from being able to accept the news in a more constructive light.

Studies show the vast majority of patients would prefer to be informed of a lifealtering diagnosis, rather than remain in ignorance. However, the amount of information they wish to receive can vary, with most wanting to know details concerning the different treatment options, and the effectiveness of proposed treatments, while they may want to hear less about the specific details of their prognosis. According to statistics, in western cultures, the majority of patients may not wish to know certain details, such as life expectancy. Healthcare workers may also find families asking that diagnoses be kept from the patient, or that patients prefer to have care wholly managed by their family, rather than themselves.

One model for delivering serious news is called SPIKES, developed by Walter Baile and initially used for discussions with cancer patients. The first step in SPIKES is setting up the interview. A quiet private area such as an exam room or family meeting room is an ideal

setting. The patient should be able to choose family members or friends to be present for support. For those who don't speak fluent English, a hospital-contracted medical interpreter should be used. The healthcare professional should be prepared to answer difficult queries about prognosis, treatment, and overall plan going forward, but also know when to refer to a specialist for more esoteric information. If there is a multi-disciplinary approach, every team member should be on the same page with regards to the care plan to avoid confusion.

The second item in SPIKES is the patient's perception. Last week, I asked a patient, let's call him Harry, if he understood his current condition. Of course, he said he did, but when he came to explain it to me, I saw that there were many gaps in his knowledge that needed to be addressed. A good way to assess the patient's understanding is to ask what the patient already knows about their condition and what they have been told so far. Make sure to assess the level of their understanding, as well as their awareness of the basic facts. This will allow you to assess their level of background knowledge, their current knowledge, and where to begin your own discussion.

The third item in SPIKES is the patient's invitation for discussion. Different patients desire different levels of information about their condition. Some of the more technical-minded or younger patients may want to know their diagnosis, prognosis, treatments, course of illness, etc. Others, including older patients, may simply wish to know the diagnosis and accept the recommendations of the treatment team as being in their best interests. Before beginning to discuss their condition, you might find it helpful to ask "Would you like me to discuss all the information we know about your condition or just certain parts? What would you like us to tell your family?"

The fourth item in SPIKES is giving knowledge to the patient. You should be direct, but avoid being unfeeling or blunt when you discuss their condition, and utilise non-technical terms in small chunks. Prognosis and course of illness should be realistic, but also convey hope and planning for the future. An appropriate opening for our patient would be, "I'm afraid, we have some serious news about the CT scan that was performed. It showed that the cancer in your liver has spread to your spine." Take note of how the words 'hepatocellular carcinoma' and 'metastasis' were rephrased into layman's terms.

The fifth item in SPIKES is addressing the patient's emotions. You should identify the emotion the patient is experiencing, the reasoning, and provide support during this difficult time. Don't try to change the patient's emotions, just help them to express how they feel. For example, in a patient who is dysphoric and crying, you can offer a tissue box and physical support if appropriate. You might say something like, "I know these results weren't what you wanted to hear. I wish we had better news for you." Other responses can

range from asking the patient to elaborate on their reaction, “Can you tell me what you’re worried about?” to validating their concerns, “I can understand why you felt that way. Many other patients have had similar reactions.”

The sixth item in SPIKES is strategy and summary. Patients who receive serious news will often feel that they are in over their head, so you should make sure that they leave with a clear plan for the future. This will help them to feel less anxious and more hopeful. Patients should know what options are available for them and what follow-up is planned. You should also recheck that they understand what has just been discussed and have had all their questions answered. A good opening statement could be, “I understand this is a lot to take in, but you have several options available. A decision does not need to be made now, but we would like to refer you to an oncologist and follow-up with us in a week to discuss your next steps.”

Giving serious news is one of the most difficult parts of being a healthcare professional. However, with careful planning and an effective protocol, patients can leave feeling well-informed and in control of their own outcome.

Text 1: Questions 1 to 8

1. Why does the writer prefer the term ‘serious news’?

(A) It enables doctors to avoid unnecessary conversations.

(B) It avoids influencing the patient’s emotional response.

(C)It helps patients to better understand their condition.

(D)It offers a more specific definition of the information.

2. The writer’s purpose in the second paragraph is to highlight

(A)the treatment options available to most patients.

(B) the difficulty of knowing what a patient wants to be told.

(C)the trends concerning what patients and relatives want to hear.

(D)the different topics that healthcare workers should cover with

patients.

3. What does the word ‘those’ refer to?

(A) healthcare staff

(B) treatment experts

(C) language translators

(D) patients and relatives

4. In the fourth paragraph, the writer mentions the patient, Harry, in order to explain that

(A) patients are often reluctant to ask for help.

(B) patients may not be aware of their ignorance.

(C) healthcare professionals often find it hard to relate to patients.

(D) healthcare professionals may not always explain things effectively.

5. The writer suggests that older patients may be more likely to

(A) require more information.

(B) limit their family's involvement.

(C) accept the staff's suggested plan.

(D) inquire further about their treatment plans.

6. In the sixth paragraph, the writer offers an example to emphasise that when explaining information professionals should

(A) avoid using complex medical language.

(B) prevent patients from becoming upset.

(c) discuss how the illness was identified.

(D) repeat information multiple times.

7. The seventh paragraph focuses on

(A) ensuring the patient understands how to react.

(B) helping the patient to feel more positive.

(C) comparing different patient responses.

(D) empathising with the patient's reaction.

8. The expression 'in over their head' is used to stress that patients might

(A) find the information overwhelming.

(B) struggle to remember information.

(C) make a choice about their treatment quickly.

(D) have difficulty understanding their prognosis.

Questions 9-16

TEXT 2: TREATING OPIUM ADDICTION

In the United States alone, there are around 115 deaths caused by opioid addiction every day. The addiction impacts individuals rapidly and drastically, damages families, and costs the US huge amounts of money: the total economic burden of prescription opioid abuse is estimated to be $78.5 billion a year, while the economic burden of non-prescription opioid abuse simply cannot be calculated. Measures are constantly being improved to prevent patients from developing opioid addictions to begin with, but it is also imperative that we continue to provide treatment for those already in the thrall of opioid addiction.

Jane's story is one heard over and over again in opioid addiction clinics. When she was 20, she had a bad automobile accident that required two surgeries. She was soon home from the hospital but her residual pain meant she was prescribed scheduled opiates. Jane's body soon became tolerant of the dosage; however, and she needed higher and higher doses in order to achieve the same pain relieving effect. She eventually reached a level that her physician felt uncomfortable prescribing. Unable to find another prescriber in time, Jane turned to alternative sources of narcotics. Unfortunately, when purchased on the street, these pills are exorbitantly expensive and increasingly hard to come by in an era of prescription monitoring throughout the United States. Heroin is much cheaper and, when delivered by IV, produces a much more potent high and greater pain relief.

Eventually, after destroying relationships with her loved ones, bankrupting her savings, and hitting rock-bottom, Jane turned to a local opioid addiction clinic for help. At the clinic, they put her on Methadone, a long-acting opioid agonist that is standard for addiction treatment. It binds to the mu-opioid receptors, prevents withdrawal symptoms, reduces cravings, and can also provide a level of pain relief. Of course, as an opioid agonist, methadone serves as a substitute for the primary addiction, meaning many of the issues associated with long-term opioid usage remain. Patients must often begin treatment with daily visits, which can be disruptive. Fortunately for Jane, these visits are her first steps towards putting her life back together. As Jane's road to recovery is likely to be long and fraught with difficulty, many doctors are led to wonder: does she have any other options?

One of the increasingly popular alternatives to methadone is buprenorphine, a partial mu-opioid agonist. Aside from its unique mechanism of action (MOA), there are two major differences when compared to Methadone: first, it can be administered as oral tablets,

sublingual/buccal films, and a long-acting implant, second, It can be prescribed month-to-month from a clinician's office directly to a local pharmacy. These factors make it much easier to use in the community, and are ideal for patients who cannot visit a methadone clinic every day.

To initiate buprenorphine, a patient must already be in a mild state of withdrawal due to the high affinity for the mu-opioid receptor displacing other opioids. This means that patients generally transition best from a short-acting opioid like heroin or oxycodone rather than a long-acting opioid agonist like Methadone, given the length of time needed until mild withdrawal occurs. As Jane had been using opioids for a long time prior to her admission, however, she was better suited to treatment with Methadone, as there is no ceiling effect to this drug, and Jane had developed a high tolerance to opioids. Buprenorphine, being a partial agonist, has a maximum level of effect which it cannot be increased beyond. For this reason, buprenorphine can be used as a maintenance therapy in some patients, but it can also be tapered down over time. This allows patients to resume their normal lives with minimal interruptions and avoid relapse through pharmacological blocking.

Alongside treatment with medication, patients recovering from opioid addiction must also deal with recovery at a mental level. As with many healing processes, the first stage is acceptance. Jane was not able to seek the treatment she needed until she had nowhere else to hide. Once everything was lost, she couldn't deny that she was in trouble anymore, so she came to the clinic. Many patients suffering from opioid addictions are reluctant to admit that they are addicted, and reluctant to ask for help. Patients are often worried about being judged, being treated like a criminal, and meeting with disapproval from the healthcare professionals who must treat them.

When patients do seek aid, healthcare professionals need to help them to build a support network around themselves, so that they are protected when they feel the need to relapse. Opioid addicts are likely to have burned bridges with friends and family who have not enabled their addiction, so patients beginning recovery may not have positive role models to support and influence their recovery. Talking therapies, such as cognitive behavioural therapy (CBT) can be offered to recovering patients experiencing anxiety or depression, though patients may find it more useful to join local confidential support groups, such as Narcotics Anonymous, as they can discuss recovery with those who have first-hand experience. Though Jane was hesitant to discuss her experiences with anyone when she was first admitted to the clinic for treatment, she has since gone on to attend weekly sessions at Narcotics Anonymous, where she not only listens to others share their stories of recovery, but where she also is beginning to tell her own.

Questions 9 to 16

9. In the first paragraph, the writer highlights that opioid addiction in the US

(A) has been gradually increasing for a number of years.

(B) is largely influenced by the illegal sale of drugs.

(C)causes more deaths than any other addiction.

(D)has a significant financial and social impact.

10. In the second paragraph, the writer outlines Jane's case in order to emphasise that

(A) opioid addiction is increasingly rare.

(B) it can be remarkably easy for a patient to become addicted.

(C) in some cases, heroin is less harmful to addicts than opioids.

(D)healthcare professionals must take responsibility for opioid addiction.

11. The writer uses the phrase 'hitting rock bottom' about the patient Jane in order to describe

(A) how her addiction led to the most distressing point in her life.

(B) her sudden awareness that she had to recover.

(C) the large tolerance she developed for opioids.

(D) the physical pain she felt at that time.

12. In the fourth paragraph, the writer suggests that buprenorphine may be preferable because

(A) it is less addictive than alternatives.

(B)it can be easier for patients to access.

(C) it does not interfere with other treatments.

(D) it can be picked up more often than other medications.

13. What does ‘this means that’ refer to?

(A) The effectiveness of buprenorphine when combating opioid displacement.

(B)The requirement for the medication to be reserved for heroin addicts.

(C) The need for patients to have begun to experience withdrawals.

(D)The impact of mu-opioids on recovered opioid addicts.

14. In the fifth paragraph, the writer suggests that Jane was prescribed methadone, rather than buprenorphine because

(A) buprenorphine is too similar to heroin.

(B) the effects of methadone last for longer.

(C) she was dependent on high doses of opioids.

(D) it is more readily available at addiction clinics.

15. According to the seventh paragraph, why do patients often delay seeking treatment for opioid addiction?

(A) They are unwilling to face the damage they have caused.

(B) They do not realise they are addicted until it's too late.

(C) They think that they can recover without help.

(D)They do not want to be labelled as an addict.

16. In the final paragraph, the writer suggests that recovering addicts may prefer to discuss their experiences with

(A) those who have experienced addiction.

(B) people who are not aware of their history.

(C) healthcare professionals.

(D) friends and family.

Part C: Answer keys

1. B It avoids influencing the patient's emotional response.

2. C the trends concerning what patients and relatives want to hear.

3. D patients and relatives

4. B patients may not be aware of their ignorance.

5. C accept the staff's suggested plan.

6. A avoid using complex medical language.

7. D empathising with the patient's reaction.

8. A find the information overwhelming.

9. D has a significant financial and social impact.

10. B it can be remarkably easy for a patient to become addicted.

11. A how her addiction led to the most distressing point in her life.

12. B it can be easier for patients to access.

13. C The need for patients to have begun to experience withdrawals.

14. C she was dependent on high doses of opioids.

15. D They do not want to be labelled as an addict.

16. A those who have experienced addiction.

KAPLAN READING TEST 2

Part A

Time : 15 minutes

- Look at the four texts, A – D, in the Text Booklet.
- For each question, 1 – 20, look through the texts, A – D, to find the relevant information.
- Write your answers in the spaces provided in this Question Paper.
- Answer all the questions within the 15-minute time limit.

Asthma : Texts

Text A

Establishing the severity of an acute asthma attack

	Moderate asthma	Severe asthma	Life-threatening asthma
Adults	**Measure PEF and arterial saturation**		
	PEF >50-75% predicted	PEF 33-50% predicted	PEF <33% predicted
	• SpO_2 ≥92% • PEF > 50-75% predicted	• SpO_2 ≥92% • PEF < 50% predicted	• SpO_2 ≥92% • silent chest
	• No features of acute severe asthma	• RR ≥ 25/min • HR ≥ 110/min • difficulty talking	• cyanosis • poor respiratory effort • arrhythmia • hypotension • exhaustion • altered consciousness

Asthma sufferers of any severity may also experience the following:

- shortness of breath
- coughing
- tightness or pain in the chest
- a whistling sound when exhaling

Text B

Lung Function Tests in Asthma

Asthma tests should be undertaken to diagnose and aid management of the condition. This is particularly important in asthma, because it presents slightly differently with each patient. Spirometry is the most important test, however several different types of test are available:

- Peak expiratory flow rate (PEFR): this is the maximum flow rate during exhalation, after full lung inflation. Diurnal variation in PEFR is a good measure of asthma and useful to the long-term management of patients and the response to treatment. Monitor PEFR over 2-4 weeks in adults if there is uncertainty about diagnosis. It is measured with a peak flow meter - a small, handheld device - into which the patient blows, giving a reading in l/min.

- Spirometry: measures volume and flow of air that can be exhaled or inhaled during normal breathing. Asthma can be diagnosed with a >15% improvement in FEV1 or PEFT following bronchodilator inhalation. Alternatively, consider FEV1/FVC < 70% as a positive result for obstructive airway disease. A spirometry test usually takes less than 10 minutes, but will last about 30 minutes if it includes reversibility testing.

Direct bronchial challenge test with histamine or methacholine: in this test, patients breathe in a bronchoconstrictor. The degree of narrowing can be quantified by spirometry. Asthmatics will react to lower doses, due to existing airway hyperactivity.

Exercise tests: these are often used for the diagnosis of asthma in children. The child should run 6 minutes (on a treadmill or other) at a workload sufficient to increase their heart rate > 160/min. Spirometry is used before and after the exercise - an FEV1 decrease > 10% indicates exercise-induced asthma.

- Allergy testing: can be useful if year-round allergies trigger a patient's asthma. This will be recommended if inhaled corticosteroids are not controlling symptoms. Three different tests are used to measure the patient's reaction to allergens: nitric oxide testing, sputum eosinophils and blood eosinophils.

Text C

Patients with asthma of any severity may find their attacks panic-inducing. Remember that the patient's struggle to breathe can cause stress, panic and a feeling of helplessness. There is a strong link between people who suffer from asthma and those who experience panic attacks. Staff must keep this in mind when treating patients with asthma, as some sufferers will require additional emotional support.

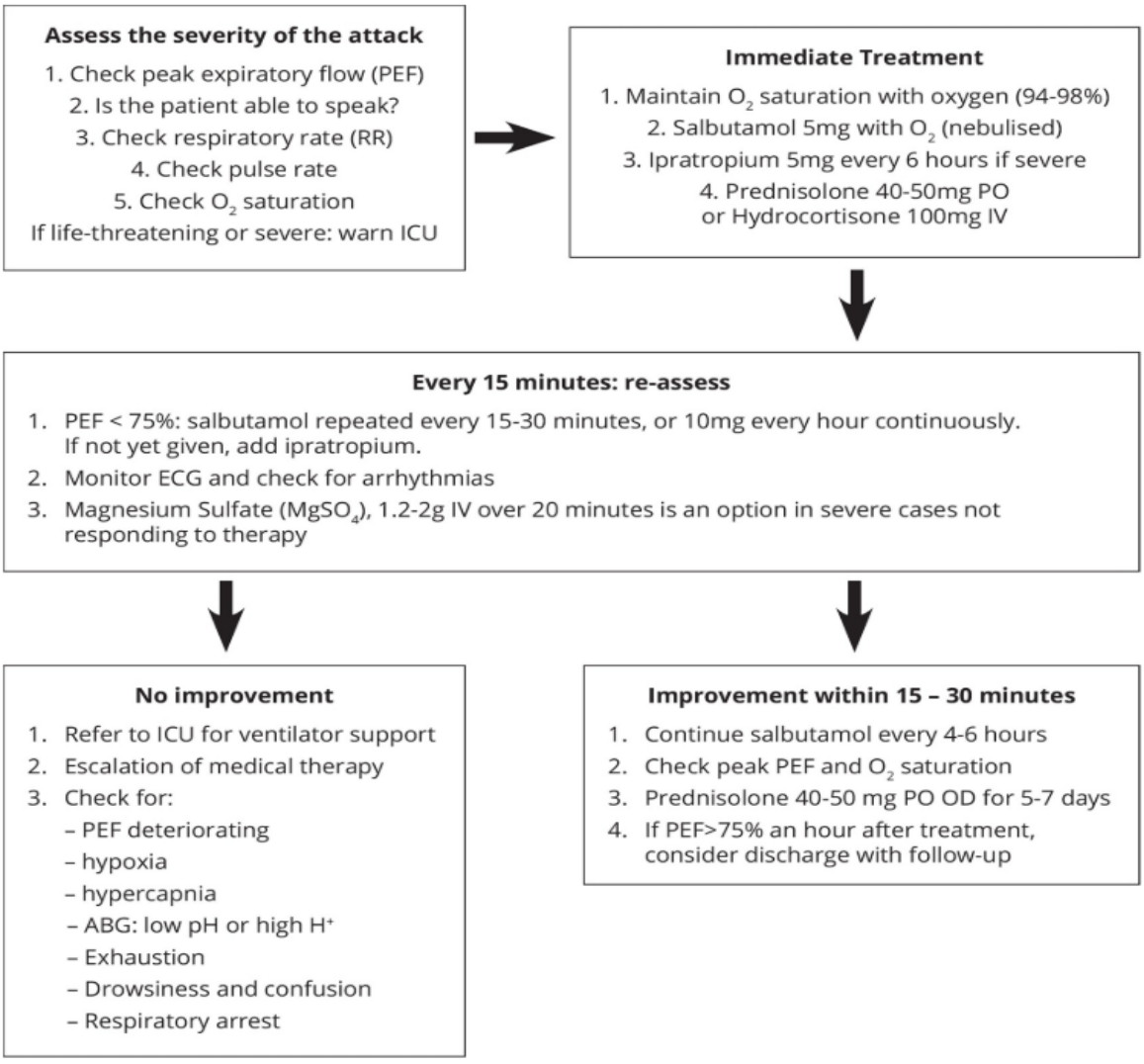
Patients may find breathing exercises beneficial. Advise patients to practice daily, to allow these exercises to become habitual. When experiencing an attack, patients should make a conscious effort to relax their muscles and maintain steady breathing. Advise patients to breathe deeply in through the nose and out through the mouth.

Smokers are at a higher risk of developing both panic attacks and asthma. In addition, smoking can irritate the airways in patients with asthma, causing neutrophilic inflammation, and exacerbating breathing problems in those with asthma. Ensure that patients who smoke are fully aware of the risks of smoking with asthma.

Text D

Management of Acute Asthma

Rapid treatment and reassessment is of paramount importance. It is sometimes difficult to assess severity. Maintaining a calm atmosphere is helpful to resolving an acute asthmatic attack.


Assess the severity of the attack
1. Check peak expiratory flow (PEF)
2. Is the patient able to speak?
3. Check respiratory rate (RR)
4. Check pulse rate
5. Check O_2 saturation
If life-threatening or severe: warn ICU
Immediate Treatment
1. Maintain O_2 saturation with oxygen (94-98%)
2. Salbutamol 5mg with O_2 (nebulised)
3. Ipratropium 5mg every 6 hours if severe
4. Prednisolone 40-50mg PO
or Hydrocortisone 100mg IV
Every 15 minutes: re-assess
1. PEF < 75%: salbutamol repeated every 15-30 minutes, or 10mg every hour continuously. If not yet given, add ipratropium.
2. Monitor ECG and check for arrhythmias
3. Magnesium Sulfate ($MgSO_4$), 1.2-2g IV over 20 minutes is an option in severe cases not responding to therapy
No improvement
1. Refer to ICU for ventilator support
2. Escalation of medical therapy
3. Check for:
– PEF deteriorating
– hypoxia
– hypercapnia
– ABG: low pH or high H^+
– Exhaustion
– Drowsiness and confusion
– Respiratory arrest
Improvement within 15 – 30 minutes
1. Continue salbutamol every 4-6 hours
2. Check peak PEF and O_2 saturation
3. Prednisolone 40-50 mg PO OD for 5-7 days
4. If PEF>75% an hour after treatment, consider discharge with follow-up

Questions 1 – 6

- For each question below, 1 – 6, decide which text (A, B, C or D) the information comes from.
- You may use any letter more than once.
- In which text can you find information about……

1. relaxation techniques for those suffering from an asthma attack?

2. measuring the respiration abilities in patients with asthma?

3. identifying the intensity of asthma attacks in patients?

4. the procedure to follow when treating an asthma attack?

5. symptoms of asthma in patients?

6. how to diagnose asthma in patients?

Questions 7 – 12

Complete each of the sentences, 7 – 12, with a word or short phrase from one of the texts. Each answer may include words, numbers or both. Your answers should be correctly spelled.

7. To understand how severe an asthma attack is, (7)) must be measured, in addition to PEF.

8. For patients who do not respond to therapy, an IV of (8) can be used to treat severe asthma attacks.

9. Nitric oxide testing can be used to determine (9) in patients.

10. A patient suffering from arrhythmia and a peak expiratory flow of greater than 33% would be diagnosed with (10) asthma attacks.

11. Spirometry tests that contain (11) typically last for half an hour.

12. (12) can cause neutrophilic inflammation in patients with asthma.

Questions 13 – 20

- Answer each of the questions, 13 – 20, with a word or short phrase from one of the texts.
- Each answer may include words, numbers or both.
- Your answers should be correctly spelled.

13. How often should patients be advised to practice breathing exercises?

14. How often should patients with a peak expiratory flow of less than 75% be given 10 mg of salbutamol?

15. When should patients be given 2mg of magnesium sulfate?

16. Which patients will typically need to run when completing spirometry tests?

17. What should staff do when assessing a patient suffering from a lifethreatening panic attack?

18. Which lung function test is helpful for understanding how the patient responds to treatment?

19. What sort of noise might patients with asthma make when breathing?

20. What is used to measure peak expiratory flow rate?

PART B: QUESTIONS 1 TO 6

In this part of the test, there are six short extracts relating to the work of health professionals. For questions 1 to 6, choose the answer (A, B or C) which you think fits best according to the text.

1. The notice reminds staff that patients who are dying

(A) will need to be prescribed anti-emetics.

(B) might not need to continue with certain medication.

(C) should be encouraged to discuss their condition with loved ones.

Extract 1

End-of-Life Decision Making

Remember the five priorities when caring for a dying patient:

1. Recognise that the end of life may be approaching.
2. Communicate with patients, families, carers and staff.
3. Involve patients and those close to them in decision making.
4. Support the needs of families and carers.
5. Develop an individualised plan of care for the patient.

An end-of-life care plan must ensure the physical, psychological, social and spiritual comfort of the patient, and should strive for the best possible quality of life for the patient's remaining time. This includes prescribing anticipatory medications which can be given as required, falling under the following categories which staff are encouraged to remember as the 'Four As': Analgesia (pain relief), Anxiolytics (anti-anxiety), Anti-emetics (for nausea and vomiting), and Anti-secretory (for respiratory and airway secretions). Any unnecessary medications, such as long-term diabetes control and blood pressure medications can be stopped. A Do-Not-Resuscitate (DNACPR) decision also needs to be made.

2. The guidelines inform us that multiple anaesthetics can be used

(A) to increase the numbing effects.

(B) to prevent bleeding throughout the procedure.

(C) to more accurately control how long it will last.

Extract 2

Anaesthesia use at Harlow Dental Centre

At this practice, preference is given to the use of local anaesthetics in combination with conscious sedation.

Many local anaesthetics may be used in order to reversibly block specific pain pathways and/or cause paralysis of muscles. The most commonly used local anaesthetic at the centre is lidocaine remember that the half-life of lidocaine in the body is about 1.5 to 2 hours. Other local anaesthetic agents include articaine, bupivacaine, prilocaine and mepivacaine. Often, a combination of local anaesthetics may be used, sometimes with adrenaline or another vasoconstrictor to modulate the metabolism of the local anaesthetic and control local bleeding.

Sedation during procedures should mostly be limited to conscious sedation. Benzodiazepines enhance the effect of neurotransmitter gamma-aminobutyric acid (GABA) at the GABAA receptor. This results in a sedative, hypnotic, anxiolytic, anticonvulsant and muscle relaxant properties.

3. The purpose of this memo is to explain

(A) how to treat multi-resistant pathogens.

(B) the causes of bacterial infections.

(C) when to prescribe antibiotics.

Extract 3

For the attention of all medical staff:

Microbial resistance to antibiotics is on the rise and infection with multi-resistant pathogens, such as Clostridium difficile and MRSA amongst others, is becoming more common.

Patients receiving antibiotics are at increased risk of such infections. As such, please be aware of our antimicrobial prescribing guidelines, which ensure that antibiotics are only prescribed with clear, clinical justification; evidence of infection; and/or guaranteed medical benefit.

It is recommended that specimens should be cultured and results obtained before commencing treatment with antibiotics, thus only prescribing the therapy to which the microbe is sensitive. Prescription of broad-spectrum antibiotics should be avoided where possible, as these not only damage the normal bacteria of the human body, but also increase microbial exposure to antimicrobial medications, increasing their potential for developing resistance. Review narrow-spectrum antibiotic prescriptions within 5 days, and broad-spectrum prescriptions within 48 hours.

4. This guidelines on autism in young people inform us that

(A) the disorder is more difficult to identify in patients with ADHD.

(B) most children with autism are diagnosed before the age of three.

(C) young people with autism are more likely to suffer from other conditions.

Extract 4

Autism in Young People

More than 1% of the UK population has an autism spectrum disorder. Signs can vary widely between individuals and at different stages of an individual's development. When children present with other conditions such as ADHD (attention deficit hyperactivity disorder) or other learning difficulties, autism spectrum disorders often go undiagnosed.

In children with autism spectrum disorders, symptoms are present before three years of age but diagnosis can be made after this age too. Individuals with autism spectrum disorder tend to have issues with social interaction and communication, including difficulty with eye contact, facial expressions, body language and gestures. Often, children with autism spectrum disorders may lack awareness or interest in other children and tend to play alone.

The causes of autism spectrum disorder are unknown but are linked to several complex genetic and environmental interactions.

5. The memo reminds all staff to avoid

(A) challenging a patient's criticisms.

(B) handling grievances of a sensitive nature.

(C) recording complaints that are not legitimate.

Extract 5

Subject: Fielding Patient Complaints

For the attention of all hospital staff:

At County Green Hospital, we endeavour to provide our patients and families with the highest quality of services. Unfortunately, there may be times where performance does not meet expectation. We routinely survey our patients on how we can do better, but members of the treatment team may also be approached with patient feedback, so all employees must be aware of the correct procedure for handling patient complaints. The first step is to listen to what patients have to say and document details appropriately. Whether or not you feel there is a legitimate grievance, it is important to keep a record for later examination. While listening to the complaint, the employee should validate the patient or family member's experience. This does not mean there needs be agreement about the nature of the complaint, but that the employee demonstrates a clear understanding of why the patient or family member might be feeling this way.

6. Patients with delirium are more likely to recover quickly

(A) if kept in a darkened environment.

(B) staff changes are kept to a minimum.

(C) treatment ensures they receive adequate rest.

Extract 6

Diagnostic Criteria for Delirium

Delirium affects up to 87% of patients in intensive care and is

particularly common among the elderly. Delirium can have serious adverse effects and even lead to mortality and must therefore be treated as a medical emergency.

All hospital staff must know how to prevent, detect, and rapidly assess and treat delirium on the hospital wards. Risk factors for developing delirium include: change of environment , loss of vision/hearing aids, inappropriate noise or lighting, sleep deprivation, severe pain, dehydration, drug withdrawal, infections of any kind, recent surgery, and old age. For patients at risk of delirium, think of the mnemonic DELIRIUM which indicates the common causes: Drugs or Dehydration, Electrolyte Imbalance, Level of pain, Infection or Inflammation (such as post-surgery), Respiratory failure, Impaction of faeces (severe constipation), Urinary retention, Metabolic disorder (such as liver or renal failure).

Management requires re-orientation of the patient to where they are and who everybody around them is, as well as re-assurance and a non-confrontational, empathetic approach towards agitated and distressed patients. Please refrain from changing the staff of the medical team responsible for a delirious patient's care, in order to ensure consistency for the patient. Avoid unfamiliar noises, equipment and staff in the immediate vicinity of the patient, and facilitate visits from family and friends as much as possible.

Part C

In this part of the test, there are two texts about different aspects of healthcare. For questions 7 to 22, choose the answer (A, B, C or D) which you think fits best according to the text.

Text 1: Work-Related Stress & Medical Errors

Stress is a term that crops up all too often in modern conversation, used to describe every unfortunate circumstance, every out-of-sequence event, and every foot out of line. What is stress? Most definitions of stress cover any internal or external stimulus which results in a negative response or disturbance in one's physical, social or mental wellbeing. Unfortunately, stress is common, and it can be devastating to people's lives and health when it is maintained over long periods of time, and when it gains the capacity to overwhelm one's coping abilities and mechanisms.

In the medical profession, daily stress is almost guaranteed. Recently, changes to many healthcare workers' contracts in the UK have resulted in longer and more antisocial working hours, as well as an increased workload, greater bed crises in hospitals and larger budget cuts, so stress levels amongst UK healthcare professionals are on the rise. A 1996 questionnaire study in the Lancet reported that 27% of doctors in the UK believed that the stress they experienced was triggered by poor management, low job satisfaction, financial concerns, and patients' suffering, amongst other factors.

Over two decades later, these problems still exist; some healthcare worker's argue that conditions have actually deteriorated. A 2013 report by the British Medical Association stated that over 50% of UK doctors had experienced an increase in work-related stress over the preceding year, in addition to an increase in the complexity of their work. 25% of junior doctors in hospitals also reported a reduced quality of care for patients due to high levels of stress and the pressures put on individual members of staff, with levels of stress exacerbated by longer working hours. In many healthcare jobs, stress is the elephant in the room, particularly with junior staff, who may feel unable to voice concerns about their workload. Unfortunately, however, these factors have the potential to lead to medical mistakes, which could be detrimental to patient lives. In such a circumstance, who is really to blame? The overworked medical staff, or the poor management of modern hospitals?

We do not need to look far to examine the effect that stress can have on doctors today. In 2015, Dr Hadiza Bawa-Garba was found guilty of manslaughter after failing to provide life-saving treatment to a patient when needed, resulting in the unfortunate death of a six-year-old child, Jack Adcock. In 2018, this experienced senior paediatrician with a previously

unblemished record was struck off the medical register, unable to ever practice again as a doctor. The case of Dr BawaGarba infuriated many in the medical profession, as fingers were pointed at an overworked doctor working under immense pressure who was blamed for gross negligence. But who is the truly negligent one in our current healthcare system? While the death of young Jack is extremely saddening, it is important to explore the circumstances around his death in order to prevent such tragedies from reoccurring. On the day of the incident, Dr Bawa-Garba was covering her own workload as well as that of two senior colleagues who were away, across six wards, spanning four floors, with malfunctioning IT software and out-of-order results systems. Did Dr Bawa-Garba make detrimental mistakes? Yes. But one must ask, are we creating a recipe for disaster when we require our medical staff to work under such immense pressures? Could this be one tragic event of many waiting to happen? Such mistakes ruin lives.

Studies have shown that the most common cause of medical errors is the use of heuristics in medical decision-making, leading to bias. Heuristics are shortcuts taken to reach decisions quickly, based on previous patterns of disease and similar cases seen by the doctor. Mistakes are more likely when such shortcuts are used by junior doctors who lack the experience necessary to make such fast decisions accurately. Tversky and Kahneman outlined seven types of heuristics in their 1974 article: Availability heuristics are based on how easy specific diagnoses are to recall, resulting in over-diagnosis of rare but memorable conditions; Representativeness heuristics are based on similarity of patient presentations to previous typical cases, leading to delayed or missed diagnoses in atypical or non-characteristic patients; Anchoring heuristics occur when a diagnosis is based on one piece of information only, leading to rapid conclusions which lack evidence and early diagnosis without consideration of all available information; Confirmation bias occurs when a diagnosis is based on a preconceived idea, where the doctor pays attention to the information that supports their theory, and evidence which challenges the diagnosis is consciously or subconsciously ignored; Commissioning bias where a doctor acts too soon rather than waiting to gather and review all the information first; Gambler's Fallacy which is where consecutive patients have the same diagnosis and so the doctor assumes a similar patient who follows must also have the same diagnosis; Fundamental Attribution Error which is the tendency to blame patients rather than their circumstances for their poor health.

Research shows that the best way to avoid medical errors in diagnosis is to consider several hypotheses, known as "differential diagnoses", and investigate them all equally until the one with the most supporting evidence is found and agreed upon. Use of heuristics and the resultant flawed decision-making could be prevented by reducing work stresses and pressures on medical professionals. One way to achieve this would be to

reduce working hours and shift durations in order to prevent sleep deprivation in medical staff, which is known to hinder focus, thus creating a safer medical environment for both staff and patients.

Text 1: Questions 7 to 14

7. The first paragraph explains that stress

(A) is usually caused by a factor than cannot be controlled.

(B) is interpreted in various ways by different people.

(C) is unusual when it lasts for an extended time.

(D) generally impacts people's behaviour.

8. In the second paragraph, doctors are said to claim that stress

(A) is often improperly managed by chronic sufferers.

(B) could be improved by increasing the welfare budget.

(C) generally resulted in their having to work longer hours.

(D) was caused by a number of issues including money worries.

9. The writer uses the phrase 'the elephant in the room' to emphasise the fact that

(A) levels of stress experienced by staff has declined.

(B) senior staff generally experience less stress than their juniors.

(C) many healthcare professionals do not discuss the stress they experience.

(D) junior doctors have reported a lower quality personal life as a result of stress.

10. Why does the writer comment on Dr Hadiza Bawa-Garba and her patient Jack?

(A) to suggest that doctors are more likely to make significant errors when stressed.

(B) to outline a scenario where a doctor's concerns about stress were ignored.

(C) to demonstrate that stress in healthcare professionals is unacceptable.

(D) to emphasise the impact the death of a patient can have on stress.

11. The writer suggests that Jack Adcock's death was partly caused by

(A) technology that was out of date and faulty.

(B) a hospital ward overcrowded with patients.

(C) an insufficient number of nursing team staff.

(D) a lack of experience among the clinical team.

12. Why might doctors who use heuristics be at a greater risk of making clinical errors?

(A) heuristics are more likely to be used by junior doctors

(B) doctors might take too long to complete their tasks

(C) doctors might skip over the relevant information

(D) the different types of heuristics are confused

13. The writer claims that confirmation bias might cause doctors to ignore relevant information if

(A) they have recently treated a patient with the same condition.

(B) they are very familiar with the evidence being presented.

(C) the patient displays extreme symptoms.

(D) it does not support their existing theory.

14. What does the word 'them' refer to in the final paragraph?

(A) the team of healthcare staff

(B) a variety of possible causes

(C) the mistakes in patient care

(D) a number of different texts

QUESTIONS 15 TO 22

Text 2: Electroconvulsive therapy (ECT)

Electrodes. Wires. Bite Blocks. For many these terms bring to mind a sinister mental asylum and the foreboding image of a patient about to suffer a tortuous electric shock. Literature written in the 20th century did much to criticise this practice, with writers frequently describing electroconvulsive therapy (ECT) as a form of torture, reserved for the most vulnerable members of society. Interestingly enough, ECT has actually been used in the healthcare field for hundreds of years. Before the advent of effective antipsychotic medications, a wide variety of therapies were trialled for serious mental illnesses. One of these involved the therapeutic use of inducing seizures in patients. As early as Benjamin Franklin's (1705 – 1790) time, an electrostatic machine could be used to cure someone of 'hysterical fits'.

Through the 19th century, British asylums began to employ electroconvulsive therapy in a widespread effort to cure diseases of the mind. In the early 20th century, a neuropsychiatrist by the name of Ladislas J. Meduna promoted the idea that schizophrenia and epilepsy were antagonistic disorders, and that precipitating seizures could serve as a potential treatment of schizophrenia. There were several methods used to induce seizures, including insulin coma, seizure-inducing medications (metrazol), and most famously, ECT.

While many of these practices are now seen as barbaric, there were very few options for psychiatric treatment before the development of antipsychotics, mood stabilisers, and anti

-depressants. With the rise of these new treatment options came an increase in the public awareness of the often inhuman conditions of electroshock. The revelations resulted in widespread backlash, and the use of ECT therapy began to swiftly decline. However, in the later part of the 20th century, after much debate and research, the National Institute of Mental Health in the US came to a consensus that ECT was both safe and effective when proper guidelines were implemented. In the US today, ECT treatment is routinely covered by insurance for severe and treatment-resistant forms of mental illness.

The exact mechanism of action for ECT is unknown, but there are several hypotheses: Firstly, increased release of monoamine neurotransmitters such as dopamine, serotonin, and norepinephrine; secondly, enhanced transmission of monoamine neurotransmitters between synapses; thirdly, release of hypothalamus or pituitary gland hormones and fourthly, anticonvulsant effect. ECT has several indications, the most notable being refractory major depression, catatonia, persistent suicidality, and bipolar disorder. It is also used in pregnancy as it is effective and does not have the teratogenic effects of some other psychiatric medications. While there are no absolute contraindications, it goes without saying that when using ECT, the risks involved will carry more weight with certain patients. Those with unstable cardiovascular conditions, those who have recently suffered a stroke, and those with increased intracranial pressure, severe pulmonary conditions, or a high risk in anaesthesia may not be suitable candidates for ECT. To further explore the appropriateness of using of ECT on specific patients, consider the following case study.

The patient, let's call her Dana, is a 35 year old female who has a history of schizophrenia. She was taken to the hospital by ambulance because her parents found her motionless in her bed, staring blankly, not responding to external stimuli, and not eating or drinking for two days. The psychiatrist caring for her is understandably concerned, because this represents symptoms of catatonia. If Dana does not eat or drink, she may develop life-threatening nutritional deficiencies and electrolyte imbalances. If she does not move, Dana may end up developing a blood clot that could result in a fatal pulmonary embolism. The first-line treatment is benzodiazepines, but in this particular case, there is no improvement in her condition. The psychiatrist decides that that ECT is the next best option. There is the issue of informed consent. Legal jurisdiction handles this differently throughout the world, but if a patient lacks capacity or is too ill to provide consent, a court must provide substitute consent to ensure adequate legal oversight. Once this happens, Dana is medically screened and prepped for treatment.

A course of ECT treatments does not have a standard regimen. Generally, most patients require between six to twelve treatments, but the actual endpoint is determined by the level of improvement. ECT is often given two to three times a week, usually on a Monday/Wednesday/Friday schedule with psychiatric symptoms and testing carried out on

a regular basis to monitor progress. Dana starts Monday by being NPO (nothing by mouth) except for any necessary medications. This reduces the chance for aspiration under anaesthesia during the seizure. She will be taken down to the ECT suite where an anaesthesiologist, psychiatrist, and nurse will greet her. She will be placed in a supine position with EEG monitoring to determine the quality of the seizure given. She will have electrodes placed on her head bitemporally, bifrontally, or unilaterally on the right. In this case, given her life-threatening catatonia, we will use the bitemporal position. The anesthesiologist will then induce anaesthesia, first preoxygenating the patient, then administering anticholinergic agent to reduce oral secretions, anaesthesic medication, muscle relaxation medication, and any cardiovascular prophylaxis as needed.

Once the patient is sufficiently sedated, a brief (0.5 to 2.0 milliseconds) electrical pulse will be introduced at a level determined to reliably cause a seizure. A therapeutic ECT seizure should last at least 15 seconds but no more than 180 seconds. Dana will be monitored for thirty to sixty minutes once this has finished, to ensure her recovery. The goal is for further treatments to reduce her symptoms and enable her to eat, drink, communicate, and move again. Of course, there are adverse effects that must be considered. Anaesthesia can cause nausea, aspiration pneumonia, dental and tongue injuries. The seizure itself can cause cardiovascular issues, and fractures in patients with osteoporosis, and can temporarily impair cognition and memory. It is advised that patients do not make any major or financial decisions during or after ECT treatment, and patients must refrain from driving until a few weeks after the last session.

For most patients, one treatment may be all that is needed. For some, continuation of ECT as a single session every couple of weeks may help to prevent relapse. Maintenance treatment for patients with chronically recurring psychiatric illness may also be appropriate. The scheduling of these sessions generally depends on the patient's needs and episodes, sometimes even going on indefinitely. In Dana's case, a few treatments are all that is needed to resolve her catatonia and soon she will be healthy enough to be discharged home with outpatient follow-up for her mental health management.

Text 2: Questions 15-22

15. In the first paragraph, the writer mentions the role of 20th century literature in

(A) informing patients of the side effects of antipsychotic medication.

(B) preventing the mistreatment of defenceless people.

(C) increasing the number of patients receiving ECT.

(D) promoting a negative image of ECT.

16. What do we learn about schizophrenia in the second paragraph?

(A) It was less prevalent in patients who experienced seizures.

(B) It had a significant impact on the treatment of epilepsy.

(C) Many asylums in the UK were not prepared to treat it.

(D) The medication metrazol could be used to induce it.

17. What did the US National Institute of Mental Health decide in the 20th century?

(A) Practitioners must follow identical treatment plans when using ECT.

(B) Patients should be given the right to refuse ECT treatment.

(C) ECT should only be used as a treatment in severe cases.

(D) ECT was accepted as a safe treatment for patients.

18. In the fourth paragraph, what idea does the writer emphasise with the phrase 'it goes without saying'?

(A) Some women find ECT treatments successful while carrying a child.

(B) It is well known that some patients will not respond well to ECT.

(C) Few patients realise that they could benefit from ECT therapy.

(D) The risks associated with ECT are rarely discussed.

19. In the case study, the psychiatrist decides to use ECT on Dana

(A) despite Dana's parents' concerns about this type of procedure.

(B) because the patient expresses a preference for this treatment.

(C) after treatment with benzodiazepines proves ineffective.

(D) as she has developed an electrolyte imbalance.

20. In the sixth paragraph, why isn't Dana given food before her ECT treatment?

(A) to lower the likelihood of anaesthesia-related aspiration.

(B) to reduce the likelihood of vomiting during treatment

(C) as medication can interfere with the treatment

(D) as the catatonic state makes eating difficult

21. In the seventh paragraph, what does the word 'this' refer to?

(A) a treatment plan

(B) a seizure caused by ECT

(C) an abnormal reaction to medication

(D) an improvement to the patient's condition.

22. In the final paragraph, the writer suggests that Dana's treatment

(A) was complete after only one ECT session.

(B) will ultimately cure her catatonia using only ECT sessions.

(C) will continue for a number of weeks before improvement can be seen.

(D) will consist of two ECT sessions each week for the foreseeable future.

Answer Keys

Part A

1. C

2. B

3. A

4. D (dirty) green

5. A

6. B

7. arterial saturation

8. magnesium sulfate

9. allergies

10. life-threatening

11. reversibility testing

12. smoking

13. daily

14. every hour

15. in severe cases

16. children

17. warn ICU

18. peak expiratory flow rate OR PEFR

19. a whistling sound

20. a peak flow meter

Part B: Questions 1 to 6

1. B might not need to continue with certain medication.

2. C to more accurately control how long it will last.

3. C when to prescribe antibiotics.

4. A the disorder is more difficult to identify in patients with ADHD.

5. A challenging a patient's criticisms.

6. B staff changes are kept to a minimum.

Part C:

Questions 7 to 14

7. B is interpreted in various ways by different people.

8. D was caused by a number of issues including money worries.

9. C many healthcare professionals do not discuss the stress they experience.

10. A to suggest that doctors are more likely to make significant errors when stressed

11. A technology that was out of date and faulty.

12. C doctors might skip over the relevant information

13. D it does not support their existing theory.

14. B a variety of possible causes

Questions 15 to 22

15. D promoting a negative image of ECT.

16. A It was less prevalent in patients who experienced seizures.

17. D ECT was accepted as a safe treatment for patients.

18. B It is well known that some patients will not respond well to ECT.

19. C after treatment with benzodiazepines proves ineffective.

20. A to lower the likelihood of anaesthesia-related aspiration

21. B a seizure caused by ECT

22. C will continue for a number of weeks before improvement can be seen

OCCUPATIONAL ENGLISH TEST

READING SUB-TEST – TEXT BOOKLET: PART A

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

OTHER NAMES:

PROFESSION:

VENUE:

TEST DATE:

CANDIDATE SIGNATURE

Passport Photo

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Sedation: Texts

Text A

Procedural sedation and analgesia for adults in the emergency department

Patients in the emergency department often need to undergo painful, distressing or unpleasant diagnostic and therapeutic procedures as part of their care. Various combinations of analgesic, sedative and anaesthetic agents are commonly used for the procedural sedation of adults in the emergency department.

Although combinations of benzodiazepines and opioids have generally been used for procedural sedation, evidence for the use of other sedatives is emerging and is supported by guidelines based on randomised trials and observational studies. Patients in pain should be provided with analgesia before proceeding to more general sedation. The intravenous route is generally the most predictable and reliable method of administration for most agents.

Local factors, including availability, familiarity, and clinical experience will affect drug choice, as will safety, effectiveness, and cost factors. There may also be cost savings associated with providing sedation in the emergency department for procedures that can be performed safely in either the emergency department or the operating theatre.

Text B

Levels of sedation as described by the American Society of Anesthesiologists

Non-dissociative sedation

- Minimal sedation and analgesia: essentially mild anxiolysis or pain control. Patients respond normally to verbal commands. Example of appropriate use: changing burns dressings
- Moderate sedation and analgesia: patients are sleepy but also aroused by voice or light touch. Example of appropriate use: direct current cardioversion
- Deep sedation and analgesia: patients require painful stimuli to evoke a purposeful response. Airway or ventilator support may be needed. Example of appropriate use: major joint reduction
- General anesthesia: patient has no purposeful response to even repeated painful stimuli. Airway and ventilator support is usually required. Cardiovascular function may also be impaired. Example of appropriate use: not appropriate for general use in the emergency department except during emergency intubation.

Dissociative sedation

Dissociative sedation is described as a trance-like cataleptic state characterised by profound analgesia and amnesia, with retention of protective airway reflexes, spontaneous respirations, and cardiopulmonary stability. Example of appropriate use: fracture reduction.

Text C

Drug administration: General principles

International consensus guidelines recommend that minimal sedation – for example, with 50% nitrous oxide-oxygen blend – can be administered by a single physician or nurse practitioner with current life support certification anywhere in the emergency department. Guidelines recommend that for moderate and dissociative sedation using intravenous agents, a physician should be present to administer the sedative, in addition to the practitioner carrying out the procedure.

For moderate sedation, resuscitation room facilities are recommended, with continuous cardiac and oxygen saturation monitoring, non-invasive blood-pressure monitoring, and consideration of capnography (monitoring of the concentration or partial pressure of carbon dioxide in the respiratory gases).

During deep sedation, capnography is recommended, and competent personnel should be present to provide cardiopulmonary rescue in terms of advanced airway management and advanced life support.

Text D

Drugs used for procedural sedation and analgesia in adults in the emergency department

Class	Drug	Dosage	Advantages	Cautions
Opioids	Fentanyl	0.5-1 µg/kg over 2 mins	Short acting analgesic; reversal agent (naloxone) available	May cause apnoea, respiratory depression, bradycardia, dysphoria, muscle rigidity, nausea and vomiting
	Morphine	50-100 µg/kg then 0.8-1 mg/h	Reversal agent (naloxone); prolonged analgesic	Slow onset and peak effect time; less reliable
	Remifentanil	0.025-0.1 µg/kg/ min	Ultra-short acting; no solid organ involved in metabolic clearance	Difficult to use without an infusion pump
Benzodiazepines	Midazolam	Small doses of 0.02-0.03 mg/kg until clinical effect achieved; repeat dosing of 0.5-1 mg with total dose ≤ 5mg	Minimal effect on respiration; reversal agent (flumazenil)	No analgesic effect; may cause hypotension
Volatile agents	Nitrous oxide	50% nitrous oxide - 50% oxygen mixture	Rapid onset and recovery; cardiovascular and respiratory stability	Acute tolerance may develop; specialised equipment needed
Propofol	Propofol	Infusion of 100 µg/kg/min for 3-5 min then reduce to~50 µg/kg/min	Rapid onset; short-acting; anticonvulsant properties	May cause rapidly deepening sedation, airway obstruction, hypotension
Phencyclidines	Ketamine	0.2-0.5 mg/kg over 2-3 min	Rapid onset; short-acting; potent analgesic even at low doses; cardiovascular stability	Avoid in patients with history of psychosis; may cause nausea and vomiting
Etomidate	Etomidate	0.1-0.15 mg/kg may re-administer	Rapid onset; short-acting; cardiovascular stability	May cause pain on injection, nausea, vomiting; caution when using in patients with seizure disorders/epilepsy – may induce seizures

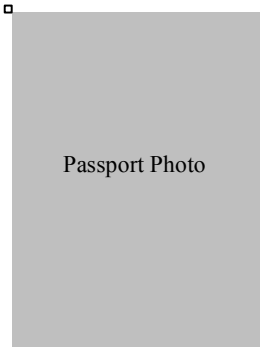
END OF PART A
THIS TEXT BOOKLET WILL BE COLLECTED

OCCUPATIONAL ENGLISH TEST

READING SUB-TEST – QUESTION PAPER: PART A

CANDIDATE NUMBER:
LAST NAME:
FIRST NAME:
OTHER NAMES:
PROFESSION:
VENUE:
TEST DATE:

CANDIDATE SIGNATURE



TIME: 15 MINUTES

INSTRUCTIONS TO CANDIDATES:

DO NOT open this **Question Paper** or the **Text Booklet** until you are told to do so.

Write your answers on the spaces provided on this **Question Paper**.

You must answer the questions within the 15-minute time limit.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of the 15 minutes, hand in this **Question Paper** and the **Text Booklet**.

DO NOT remove OET material from the test room.

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Part A

TIME: 15 minutes

- Look at the four texts, **A-D**, in the separate **Text Booklet**.
- For each question, **1-20**, look through the texts, **A-D**, to find the relevant information.
- Write your answers on the spaces provided in this **Question Paper**.
- Answer all the questions within the 15-minute time limit.
- Your answers should be correctly spelt.

Sedation: Questions

Questions 1-7

For each question, **1-7**, decide which text (**A**, **B**, **C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about

1 the point at which any necessary pain relief should be given? _____

2 the benefits and drawbacks of specific classes of drugs? _____

3 financial considerations when making decisions about sedation? _____

4 typical procedures carried out under various sedation levels? _____

5 measures to be taken to ensure a patient's stability under sedation? _____

6 reference to research into alternative sedative agents? _____

7 patients' levels of sensory awareness when sedated? _____

Questions 8-14

Answer each of the questions, **8-14**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

8 What class of drug is traditionally administered together with opioids for the purpose of procedural sedation?

_____

9 What level of sedation is appropriate for changing burns dressings?

_____

10 What is the only emergency department procedure for which it is appropriate to use general anaesthesia?

_____

11 What procedure may be carried out under dissociative sedation?

_____

12 What class of drugs is unsuitable for patients who have a history of psychosis?

_____

13 What opioid drug should be administered using specific equipment?

_____

14 What is the maximum overall dose of Midazolam which should be given?

_____

Questions 15-20

Complete each of the sentences, **15-20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

15 The majority of sedative drugs are administered via the _____.

16 General anaesthesia is the one form of sedation under which patients may have reduced _____.

17 Patients under minimal sedation will react if they are given _____.

18 Care should be taken when administering Etomidate to patients who are likely to have _____.

19 It may be helpful to use capnography to keep track of patients' _____ levels during moderate sedation.

20 Fentanyl, Morphine and Midozolam each have a _____, which is used to cancel out the effects of the drug.

END OF PART A
THIS QUESTION PAPER WILL BE COLLECTED

OCCUPATIONAL ENGLISH TEST

Sample Test 5

READING SUB-TEST – QUESTION PAPER: PARTS B & C

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

OTHER NAMES:

PROFESSION:

VENUE:

TEST DATE:

CANDIDATE SIGNATURE

Passport Photo

TIME: 45 MINUTES

INSTRUCTIONS TO CANDIDATES:

DO NOT open this **Question Paper** until you are told to do so.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of this test, hand in this **Question Paper**.

DO NOT remove OET material from the test room.

HOW TO ANSWER THE QUESTIONS:

Mark your answers on this **Question Paper** by filling in the circle using a 2B pencil.

Example:

A

B

C

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Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For **questions 1-6**, choose answer (**A**, **B** or **C**) which you think fits best according to the text.

---

1. The manual states that the wheelchair should not be used

Ⓐ inside buildings.

Ⓑ without supervision.

Ⓒ on any uneven surfaces.

Manual extract: Kuschall ultra-light wheelchair
Intended use The active wheelchair is propelled manually and should only be used for independent or assisted transport of a disabled patient with mobility difficulties. In the absence of an assistant, it should only be operated by patients who are physically and mentally able to do so safely (e.g., to propel themselves, steer, brake, etc.). Even where restricted to indoor use, the wheelchair is only suitable for use on level ground and accessible terrain. This active wheelchair needs to be prescribed and fit to the individual patient's specific health condition. Any other or incorrect use could lead hazardous situations to arise.

2. These guidelines contain instructions for staff who

Ⓐ need to screen patients for MRSA.

Ⓑ are likely to put patients at risk from MRSA.

Ⓒ intend to treat patients who are infected with MRSA.

MRSA Screening guidelines
It may be necessary to screen staff if there is an outbreak of MRSA within a ward or department. Results will normally be available within three days, although occasionally additional tests need to be done in the laboratory. Staff found to have MRSA will be given advice by the Department of Occupational Health regarding treatment. Even minor skin sepsis or skin diseases such as eczema, psoriasis or dermatitis amongst staff can result in widespread dissemination of staphylococci. If a ward has an MRSA problem, staff with any of these conditions (colonised or infected) must contact Occupational Health promptly, so that they can be screened for MRSA carriage. Small cuts and/or abrasions must always be covered with a waterproof plaster. Staff with infected lesions must not have direct contact with patients and must contact Occupational Health.

3. The main point of the notice is that hospital staff

Ⓐ need to be aware of the relative risks of various bodily fluids.

Ⓑ should regard all bodily fluids as potentially infectious.

Ⓒ must review procedures for handling bodily fluids.

Infection prevention

Infection control measures are intended to protect patients, hospital workers and others in the healthcare setting. While infection prevention is most commonly associated with preventing HIV transmission, these procedures also guard against other blood borne pathogens, such as hepatitis B and C, syphilis and Chagas disease. They should be considered standard practice since an outbreak of enteric illness can easily occur in a crowded hospital.

Infection prevention depends upon a system of practices in which all blood and bodily fluids, including cerebrospinal fluid, sputum and semen, are considered to be infectious. All such fluids from all people are treated with the same degree of caution, so no judgement is required about the potential infectivity of a particular specimen. Hand washing, the use of barrier protection such as gloves and aprons, the safe handling and disposal of 'sharps' and medical waste and proper disinfection, cleaning and sterilisation are all part of creating a safe hospital.

4. What do nursing staff have to do?

Ⓐ train the patient how to control their condition with the use of an insulin pump

Ⓑ determine whether the patient is capable of using an insulin pump appropriately

Ⓒ evaluate the effectiveness of an insulin pump as a long-term means of treatment

Extract from staff guidelines: Insulin pumps

Many patients with diabetes self-medicate using an insulin pump. If you're caring for a hospitalised patient with an insulin pump, assess their ability to manage self-care while in the hospital. Patients using pump therapy must possess good diabetes self-management skills. They must also have a willingness to monitor their blood glucose frequently and record blood glucose readings, carbohydrate intake, insulin boluses, and exercise. Besides assessing the patient's physical and mental status, review and record pump-specific information, such as the pump's make and model. Also assess the type of insulin being delivered and the date when the infusion site was changed last. Assess the patient's level of consciousness and cognitive status. If the patient doesn't seem competent to operate the device, notify the healthcare provider and document your findings.

5. The extract states that abnormalities in babies born to mothers who took salbutamol are

Ⓐ relatively infrequent.

Ⓑ clearly unrelated to its use.

Ⓒ caused by a combination of drugs.

Extract from a monograph: Salbutamol Sulphate Inhalation Aerosol

Pregnant women

Salbutamol has been in widespread use for many years in humans without apparent ill consequence. However, there are no adequate and well controlled studies in pregnant women and there is little published evidence of its safety in the early stages of human pregnancy. Administration of any drug to pregnant women should only be considered if the anticipated benefits to the expectant woman are greater than any possible risks to the foetus.

During worldwide marketing experience, rare cases of various congenital anomalies, including cleft palate and limb defects, have been reported in the offspring of patients being treated with salbutamol. Some of the mothers were taking multiple medications during their pregnancies. Because no consistent pattern of defects can be discerned, a relationship with salbutamol use cannot be established.

6. What is the purpose of this extract?

Ⓐ to present the advantages and disadvantages of particular procedures

Ⓑ to question the effectiveness of certain ways of removing non-viable tissue

Ⓒ to explain which methods are appropriate for dealing with which types of wounds

Extract from a textbook: debridement

Debridement is the removal of non-viable tissue from the wound bed to encourage wound healing. Sharp debridement is a very quick method, but should only be carried out by a competent practitioner, and may not be appropriate for all patients. Autolytic debridement is often used before other methods of debridement. Products that can be used to facilitate autolytic debridement include hydrogels, hydrocolloids, cadexomer iodine and honey. Hydrosurgery systems combine lavage with sharp debridement and provide a safe and effective technique, which can be used in the ward environment. This has been shown to precisely target damaged and necrotic tissue and is associated with a reduced procedure time. Ultrasonic assisted debridement is a relatively painless method of removing non-viable tissue and has been shown to be effective in reducing bacterial burden, with earlier transition to secondary procedures. However, these last two methods are potentially expensive and equipment may not always be available.

Part C

In this part of the test, there are two texts about different aspects of healthcare. For **questions 7-22**, choose the answer (**A**, **B**, **C** or **D**) which you think fits best according to the text.

Text 1: Cardiovascular benefits of exercise

Cardiovascular disease (CVD) is the leading cause of death for both men and women in the United States. According to the American Heart Association (AHA), by the year 2030, the prevalence of cardiovascular disease in the USA is expected to increase by 9.9%, and the prevalence of both heart failure and stroke is expected to increase by approximately 25%. Worldwide, it is projected that CVD will be responsible for over 25 million deaths per year by 2025. And yet, although several risk factors are non-modifiable (age, male gender, race, and family history), the majority of contributing factors are amenable to intervention. These include elevated blood pressure, high cholesterol, smoking, obesity, diet and excess stress. Aspirin taken in low doses among high risk groups is also recommended for its cardiovascular benefits.

One modifiable behaviour with major therapeutic implications for CVD is inactivity. Inactive or sedentary behaviour has been associated with numerous health conditions and a review of several studies has confirmed that prolonged total sedentary time (measured objectively via an accelerometer) has a particularly adverse relationship with cardiovascular risk factors, disease, and mortality outcomes. The cardiovascular effects of leisure time physical activity are compelling and well documented. Adequate physical leisure activities like walking, swimming, cycling, or stair climbing done regularly have been shown to reduce type 2 diabetes, some cancers, falls, fractures, and depression. Improvements in physical function and weight management have also been shown, along with increases in cognitive function, quality of life, and life expectancy.

Several occupational studies have shown adequate physical activity in the workplace also provides benefits. Seat-bound bus drivers in London experienced more coronary heart disease than mobile conductors working on the same buses, as do office-based postal workers compared to their colleagues delivering mail on foot. The AHA recommends that all Americans invest in at least 30 minutes a day of physical activity on most days of the week. In the face of such unambiguous evidence, however, most healthy adults, **apparently by choice** it must be assumed, remain sedentary.

The cardiovascular beneficial effects of regular exercise for patients with a high risk of coronary disease have also been well documented. Leisure time exercise reduced cardiovascular mortality during a 16-year follow-up study of men in the high risk category. In the Honolulu Heart Study, elderly men walking more than 1.5 miles per day similarly reduced their risk of coronary disease. Such people engaging in regular exercise have also demonstrated other CVD benefits including decreased rate of strokes and improvement in erectile dysfunction. There is also evidence of an up to 3-year increase in lifespan in these groups.

Among patients with experience of heart failure, regular physical activity has also been found to help improve angina-free activity, prevent heart attacks, and result in decreased death rates. It also improves physical endurance in patients with peripheral artery disease. Exercise programs carried out under supervision such as cardiac rehabilitation in patients who have undergone percutaneous coronary interventions or heart valve surgery, who are transplantation candidates or recipients, or who have peripheral arterial disease result in significant **short- and long-term CVD benefits**.

Since data indicate that cardiovascular disease begins early in life, physical interventions such as regular exercise should be started early for optimum effect. The US Department of Health and Human Services for Young People wisely recommends that high school students achieve a minimum target of 60 minutes of daily exercise. This may be best achieved via a mandated curriculum. Subsequent transition from high school to college is associated with a steep decline in physical activity. Provision of convenient and adequate exercise time as well as free or inexpensive college credits for documented workout periods could potentially enhance participation. Time spent on leisure time physical activity decreases further with entry into the workforce. Free health club memberships and paid supervised exercise time could help promote a continuing exercise regimen. Government sponsored subsidies to employers incorporating such exercise programs can help decrease the anticipated future cardiovascular disease burden in this population.

General physicians can play an important role in counselling patients and promoting exercise. Although barriers such as lack of time and patient non-compliance exist, medical reviews support the effectiveness of physician counselling, both in the short term and long term. The good news is that the percentage of adults engaging in exercise regimes on the advice of US physicians has increased from 22.6% to 32.4% in the last decade. The empowerment of physicians, with training sessions and adequate reimbursement for their services, will further increase this percentage and ensure long-term adherence to such programmes. Given that risk factors for CVD are consistent throughout the world, reducing its burden will not only improve the quality of life, but will increase the lifespan for millions of humans worldwide, not to mention saving billions of health-related dollars.

Text 1: Questions 7-14

7. In the first paragraph, what point does the writer make about CVD?

Ⓐ Measures to treat CVD have failed to contain its spread.

Ⓑ There is potential for reducing overall incidence of CVD.

Ⓒ Effective CVD treatment depends on patient co-operation.

Ⓓ Genetic factors are likely to play a greater role in controlling CVD.

8. In the second paragraph, what does the writer say about inactivity?

Ⓐ Its role in the development of CVD varies greatly from person to person.

Ⓑ Its level of risk lies mainly in the overall amount of time spent inactive.

Ⓒ Its true impact has only become known with advances in technology.

Ⓓ Its long-term effects are exacerbated by certain medical conditions.

9. The writer mentions London bus drivers in order to

Ⓐ demonstrate the value of a certain piece of medical advice.

Ⓑ stress the need for more research into health and safety issues.

Ⓒ show how important free-time activities may be to particular groups.

Ⓓ emphasise the importance of working environment to long-term health.

10. The phrase **'apparently by choice'** in the third paragraph suggests the writer

Ⓐ believes that health education has failed the public.

Ⓑ remains unsure of the motivations of certain people.

Ⓒ thinks that people resent interference with their lifestyles.

Ⓓ recognises that the rights of individuals take priority in health issues.

11. In the fourth paragraph, what does the writer suggest about taking up regular exercise?

Ⓐ Its benefits are most dramatic amongst patients with pre-existing conditions.

Ⓑ It has more significant effects when combined with other behavioural changes.

Ⓒ Its value in reducing the risks of CVD is restricted to one particular age group.

Ⓓ It is always possible for a patient to benefit from making such alterations to lifestyle.

12. The writer says '**short- and long-term CVD benefits**' derive from

Ⓐ long distance walking.

Ⓑ better cardiac procedures.

Ⓒ organised physical activity.

Ⓓ treatment of arterial diseases.

13. The writer supports official exercise guidelines for US high school students because

Ⓐ it is likely to have more than just health benefits for them.

Ⓑ they are rarely self-motivated in terms of physical activity.

Ⓒ it is improbable they will take up exercise as they get older.

Ⓓ they will gain the maximum long-term benefits from such exercise.

14. What does the writer suggest about general physicians promoting exercise?

Ⓐ Patients are more likely to adopt effective methods under their guidance.

Ⓑ They are generally seen as positive role models by patients.

Ⓒ There are insufficient incentives for further development.

Ⓓ It may not be the best use of their time.

Text 2: Power of Placebo

Ted Kaptchuk is a Professor of Medicine at Harvard Medical School. For the last 15 years, he and fellow researchers have been studying the placebo effect – something that, before the 1990s, was seen simply as **a thorn in medicine's side**. To prove a medicine is effective, pharmaceutical companies must show not only that their drug has the desired effects, but that the effects are significantly greater than those of a placebo control group. However, both groups often show healing results. Kaptchuk's innovative studies were among the first to study the placebo effect in clinical trials and tease apart its separate components. He identified such variables as patients' reporting bias (a conscious or unconscious desire to please researchers), patients simply responding to doctors' attention, the different methods of placebo delivery and symptoms subsiding without treatment – the inevitable trajectory of most chronic ailments.

Kaptchuk's first randomised clinical drug trial involved 270 participants who were hoping to alleviate severe arm pain such as carpal tunnel syndrome or tendonitis. Half the subjects were instructed to take pain-reducing pills while the other half were told they'd be receiving acupuncture treatment. But just two weeks into the trial, about a third of participants - regardless of whether they'd had pills or acupuncture - started to complain of terrible side effects. They reported things like extreme fatigue and nightmarish levels of pain. Curiously though, these side effects were exactly what the researchers had warned patients about before they started treatment. But more astounding was that the majority of participants - in other words the remaining two-thirds - reported real relief, particularly those in the acupuncture group. This seemed amazing, as no-one had ever proved the superior effect of acupuncture over standard painkillers. But Kaptchuk's team hadn't proved it either. The 'acupuncture' needles were in fact retractable shams that never pierced the skin and the painkillers were actually pills made of corn starch. This study wasn't aimed at comparing two treatments. It was deliberately designed to compare two fakes.

Kaptchuk's needle/pill experiment shows that the methods of placebo administration are as important as the administration itself. It's a valuable insight for any health professional: patients' feelings and beliefs matter, and the ways physicians present treatments to patients can significantly affect their health. This is the one finding from placebo research that doctors can apply to their practice immediately. Others such as sham acupuncture, pills or other fake interventions are nowhere near ready for clinical application. Using placebo in this way requires deceit, which falls foul of several major pillars of medical ethics, including patient autonomy and informed consent.

Years of considering this problem led Kaptchuk to his next clinical experiment: what if he simply told people they were taking placebos? This time his team compared two groups of IBS sufferers. One group received no treatment. The other patients were told they'd be taking fake, inert drugs (from bottles labelled 'placebo pills') and told also, at some length, that placebos often have healing effects. The study's results shocked the investigators themselves: even patients who knew they were taking placebos described real improvement, reporting twice as much symptom relief as the no-treatment group. It hints at a possible future in which clinicians cajole the mind into healing itself and the body – without the drugs that can be more of a problem than those they purport to solve.

But to really change minds in mainstream medicine, researchers have to show biological evidence – a feat achieved only in the last decade through imaging technology such as positron emission tomography (PET) scans and functional magnetic resonance imaging (MRI). Kaptchuk's team has shown with these technologies that placebo treatments affect the areas of the brain that modulate pain reception. 'It's those advances in "hard science"', said one of Kaptchuk's researchers, 'that have given placebo research a legitimacy it never enjoyed before'. **This new visibility** has encouraged not only research funds but also interest from healthcare organisations and pharmaceutical companies. As private hospitals in the US run by healthcare companies increasingly reward doctors for maintaining patients' health (rather than for the number of procedures they perform), research like Kaptchuk's becomes increasingly attractive and the funding follows.

Another biological study showed that patients with a certain variation of a gene linked to the release of dopamine were more likely to respond to sham acupuncture than patients with a different variation – findings that could change the way pharmaceutical companies conduct drug trials. Companies spend millions of dollars and often decades testing drugs; every drug must outperform placebos if it is to be marketed. If drug companies could preselect people who have a low predisposition for placebo response, this could seriously reduce the size, cost and duration of clinical trials, bringing cheaper drugs to the market years earlier than before.

Text 2: Questions 15-22

15. The phrase '**a thorn in medicine's side**' highlights the way that the placebo effect

- (A) varies from one trial to another.
- (B) affects certain patients more than others.
- (C) increases when researchers begin to study it.
- (D) complicates the process of testing new drugs.

16. In the first paragraph, it's suggested that part of the placebo effect in trials is due to

- (A) the way health problems often improve naturally.
- (B) researchers unintentionally amplifying small effects.
- (C) patients' responses sometimes being misinterpreted.
- (D) doctors treating patients in the control group differently.

17. The results of the trial described in the second paragraph suggest that

- (A) surprising findings are often overturned by further studies.
- (B) simulated acupuncture is just as effective as the real thing.
- (C) patients' expectations may influence their response to treatment.
- (D) it's easy to underestimate the negative effect of most treatments.

18. According to the writer, what should health professionals learn from Kaptchuk's studies?

- (A) The use of placebos is justifiable in some settings.
- (B) The more information patients are given the better.
- (C) Patients value clarity and honesty above clinical skill.
- (D) Dealing with patients' perceptions can improve outcomes.

19. What is suggested about conventional treatments in the fourth paragraph?

Ⓐ Patients would sometimes be better off without them.

Ⓑ They often relieve symptoms without curing the disease.

Ⓒ They may not work if patients do not know what they are.

Ⓓ Insufficient attention is given to developing effective ones.

20. What does the phrase '**This new visibility**' refer to?

Ⓐ improvements in the design of placebo studies

Ⓑ the increasing acceptance of placebo research

Ⓒ innovations in the technology used in placebo studies

Ⓓ the willingness of placebo researchers to admit mistakes

21. In the fifth paragraph, it is suggested that Kaptchuk's research may ultimately benefit from

Ⓐ the financial success of drug companies.

Ⓑ a change in the way that doctors are paid.

Ⓒ the increasing number of patients being treated.

Ⓓ improved monitoring of patients by healthcare providers.

22. According to the final paragraph, it would be advantageous for companies to be able to use genetic testing to

Ⓐ understand why some patients don't respond to a particular drug.

Ⓑ choose participants for trials who will benefit most from them.

Ⓒ find out which placebos induce the greatest response.

Ⓓ exclude certain individuals from their drug trials.

END OF READING TEST
THIS BOOKLET WILL BE COLLECTED

ANSWER KEY

Reading Part A

1 A

2 D

3 A

4 B

5 C

6 A

7 B

8 benzodiazepines

9 minimal sedation / minimal

10 emergency intubation / intubation

11 fracture reduction

12 Phencyclidines

13 Remifentanil

14 5mg / 5milligrams / 5 mg / 5 milligrams

15 IV / intravenous route

16 cardiovascular function

17 verbal commands

18 epileptic seizures / seizures / a seizure / an epileptic seizure / seizure disorders

19 carbon dioxide

20 reversal agent

Reading Part B: Questions 1-8

1 C

2 B

3 B

4 B

5 A

6 A

Reading Part C: Questions 7-14

7 B

8 B

9 A

10 B

11 D

12 C

13 D

14 A

Reading Part C: Questions 15-22

15 D

16 A

17 C

18 D

19 A

20 B

21 B

22 D